

PassReady Prep — NCE Question Bank · Batch 5

Generated: 2026-10-04 · Showing: In review · Total items: 200

How to review

✓ **KEY** marks the correct option. Every option shows why it is right or wrong; check both the key and each distractor.

Mark each item **Approve**, **Revise** (note what to change) or **Retire**. Approved items are published from Admin → NCE Questions.

📌 **Check** notes mark specific facts or citations the writer was less than certain of — verify those first.

Check: one clearly best answer · factual accuracy (theorists, dates, test properties, code sections) · distractors plausible but defensibly wrong · no answer given away by length or wording · references support the key.

NCE domains (NBCC 2023 content outline)

Professional Practice and Ethics 19 · Intake, Assessment, and Diagnosis 19 · Areas of Clinical Focus 47 · Treatment Planning 14 · Counseling Skills and Interventions 48 · Core Counseling Attributes 13 — of 160 scored items

In this copy

Professional Practice and Ethics **24** · Intake, Assessment, and Diagnosis **24** · Areas of Clinical Focus **59** · Treatment Planning **17** · Counseling Skills and Interventions **60** · Core Counseling Attributes **16**

Professional Practice and Ethics

24 items · NCE weight 12% (19 of 160 scored items)

Q1 of 200 · nce-s-eth-097 · Professional Counseling Orientation and Ethical Practice · duty to warn and protect · medium · In review

After a careful risk assessment, a counselor concludes that she must notify a man whom her client, a 31-year-old warehouse worker, has credibly threatened to stab. What information should she share when she makes the call?

A. The client's full diagnosis, treatment history and recent session notes for context

Diagnosis, history and notes go well beyond what is needed to protect the man.

B. Nothing that names the client, so the client's privacy is kept fully intact

A warning that does not identify the source of danger may not let the man protect himself.

C. Only the essential facts: who poses the threat and the nature of the danger ✓ **KEY**

B.2.e directs that only essential information is revealed when disclosure is required.

D. Whatever the man asks about the client, since the duty overrides all privacy

The duty to protect permits a limited disclosure, not open access to the client's information.

Rationale: When serious and foreseeable harm requires a disclosure, ACA B.2.e limits it to essential information. In a warning, that means enough for the person to protect themselves, such as the source and nature of the threat, without diagnosis or treatment details. Where possible, the client is told about the disclosure beforehand and involved in the decision.

References: ACA Code of Ethics — B.2.e Minimal Disclosure · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

The father of an adult client calls a counselor and says his son has been talking about "getting even" with his ex-partner's new boyfriend and has been driving past the boyfriend's home. The client has said nothing like this in session. How should the counselor BEST view the father's report?

A. As risk information to assess promptly, since some state laws treat it as able to trigger a duty ✓ KEY

Courts in some states (e.g., Ewing v. Goldstein) have held that a family member's report to the therapist can trigger the duty to protect.

B. As hearsay that cannot create any duty, because only the client's own words count in law

This is not true in every jurisdiction; some courts have held otherwise, and clinically the report must be assessed.

C. As grounds to warn the boyfriend at once, before speaking with the client about any of it

A warning without assessment skips the step that tells the counselor whether the risk is serious and foreseeable.

D. As a confidentiality breach by the father, which the counselor should report to the client

The father did not breach any duty, and treating the call this way ignores a possible risk to an identifiable person.

Rationale: Information about a client's dangerousness does not have to come from the client to be clinically and legally significant. In Ewing v. Goldstein (2004), a California appellate court held that a family member's report to the therapist could trigger the statutory duty to protect. Counselors assess such reports promptly, consult, and follow the law of their own state.

References: State law — Ewing v. Goldstein (Cal. Ct. App. 2004): family member communications and the duty to protect · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling student asks why many textbooks now use the phrase "duty to protect" rather than "duty to warn." Which answer is MOST accurate?

A. Duty to protect applies only to suicide risk, while duty to warn covers harm to others

Both terms concern danger to others; suicide risk is handled under separate standards.

B. Duty to protect requires that police be called first, before any other step is taken

No single required first step applies; options depend on the case and on state law.

C. Duty to protect means the counselor shares liability for any harm the client commits

The term describes the clinician's obligation to act reasonably, not automatic liability.

D. Protecting may involve steps besides a warning, such as hospitalization or police contact ✓ KEY

The 1976 Tarasoff ruling framed the duty as reasonable care to protect, which a warning alone may or may not meet.

Rationale: The 1976 Tarasoff decision held that when a therapist determines a patient presents a serious danger of violence to another, the therapist must use reasonable care to protect the intended victim. Warning the victim is one option; others include notifying police, arranging hospitalization, or intensifying treatment. State laws define which actions discharge the duty.

References: State law — Tarasoff v. Regents of the University of California (1976): duty to use reasonable care to protect

☐ Approve ☐ Revise ☐ Retire Notes:

An agency counselor tells her supervisor that a 9-year-old client described being burned with a cigarette by a parent's partner. The supervisor says, "I'll take care of the report." Two days later, the counselor cannot confirm that any report was made. What should she do?

A. Wait for the supervisor to follow up, since the agency chain of command now owns it

In most states the mandated reporter's duty is personal and is not met by telling a supervisor.

B. Make the report herself now, since telling a supervisor does not discharge her duty ✓ **KEY**

Mandated reporting duties generally rest on the individual; she must make sure a report is made promptly.

C. Ask the child about the burn in more detail first, so the report will be complete

Investigating is the role of child protective services; more questioning can harm the child and the case.

D. Call the parent's partner to hear his account before deciding whether to report

Contacting the alleged abuser can put the child at further risk and is not the counselor's role.

Rationale: In most jurisdictions a mandated reporter's obligation is individual: informing a supervisor or administrator does not relieve the counselor of the duty to make sure a report reaches child protective services or law enforcement within the required time. Agency procedures can support reporting but do not replace it. The counselor should report and document the date, time and agency contacted.

References: State law — Child abuse reporting statutes: individual duty of mandated reporters and reporting timelines

☐ Approve ☐ Revise ☐ Retire Notes:

A 40-year-old father in counseling for anger tells his counselor that last week he struck his 6-year-old with a belt, leaving welts. He says it will not happen again and that counseling is how he is "fixing it." He asks the counselor to keep it private. What should the counselor do?

A. Keep it confidential, since the client is already in treatment for the very problem

Being in treatment does not remove the legal duty to report suspected child abuse.

B. File the required report and discuss it with the client, as reviewed in informed consent ✓ **KEY**

Mandated reporting applies when the client is the alleged abuser; the counselor can stay engaged and transparent.

C. Report only if the client strikes the child again, since this was a single incident

Reporting laws do not wait for repeated abuse; one incident with injury can trigger the duty.

D. End counseling and refer him elsewhere, so a new provider can decide about reporting

Referring out evades the duty and abandons a client who is seeking help.

Rationale: Child abuse reporting laws apply regardless of whether the person disclosing is the victim, a witness, or the alleged perpetrator, and they do not exempt clients who are already in treatment. Because reporting is a limit of confidentiality, it should be covered in informed consent at the start. Counselors can report while staying engaged and supporting the client's change efforts.

References: State law — Child abuse reporting statutes: duty applies to disclosures by alleged perpetrators · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A new school counselor hesitates to report a suspicion of neglect because she fears the family will sue her if child protective services finds nothing. What should her supervisor tell her?

A. She should wait for proof, since a report that turns out unfounded can lead to liability

The standard is reasonable suspicion, not proof, and good-faith reports are protected.

B. She may report anonymously, which is the only way to avoid being named in a lawsuit

Mandated reporters usually must identify themselves; protection comes from good-faith immunity.

C. State laws give immunity for reports made in good faith, even if abuse is not found ✓ **KEY**

All states grant immunity from liability for good-faith reports, and failing to report can carry penalties.

D. She should get the parents' consent before reporting, which protects her from a suit

Parental consent is not required to report and seeking it could put the child at risk.

Rationale: Mandated reporters must report when they have reasonable cause to suspect abuse or neglect. State laws, consistent with federal CAPTA requirements, protect people who report in good faith from civil and criminal liability even when the report is not substantiated. By contrast, knowingly failing to report can bring fines or other penalties.

References: State law — Good-faith immunity for child abuse and neglect reporters; penalties for failure to report

☐ Approve ☐ Revise ☐ Retire Notes:

A billing specialist at a HIPAA-covered group practice asks a counselor to forward complete session notes for every client so claims can be submitted faster. Which HIPAA principle MOST directly applies?

A. Minimum necessary: share only the information that is needed to process the claims ✓ **KEY**

For payment and operations uses, the minimum necessary standard limits what staff may access.

B. Right of access: staff have the same rights to full records that clients themselves do

The right of access belongs to the individual client, not to workforce members.

C. Treatment exception: anyone in the practice may see full notes without any limits

Billing is a payment function, not treatment, and is subject to minimum necessary limits.

D. Breach notification: clients must be told each time a billing staff member views notes

Breach notification applies to impermissible disclosures, not routine permitted access.

Rationale: The HIPAA Privacy Rule requires covered entities to limit uses and disclosures of protected health information to the minimum necessary to accomplish the purpose. Claims usually need dates, codes and diagnoses, not full session content. The minimum necessary standard does not apply to disclosures to other providers for treatment, but billing is a payment function.

References: HIPAA — Privacy Rule minimum necessary standard (45 CFR 164.502(b), 164.514(d))

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor opening a private practice that bills health insurance electronically is setting up her intake paperwork. Under the HIPAA Privacy Rule, what must she give new clients?

- A. A full copy of her case notes from every past client so they can judge her methods
Sharing other clients' records would itself be a serious privacy violation.
- B. A signed waiver giving up their right to see their own records for the treatment term
HIPAA gives clients a right of access; providers may not require waiving it.
- C. A written guarantee that no information will be shared without their permission ever
HIPAA and law permit some disclosures without authorization, so this promise is inaccurate.
- D. A Notice of Privacy Practices, with a good-faith effort to get written acknowledgment ✓ KEY
Direct treatment providers give the notice at first service and try to obtain acknowledgment of receipt.

Rationale: A covered health care provider with a direct treatment relationship must give clients a Notice of Privacy Practices no later than the first service and make a good-faith effort to obtain written acknowledgment of receipt. The notice explains how information may be used and disclosed and describes clients' rights, such as access and amendment.

References: HIPAA — Notice of Privacy Practices and acknowledgment of receipt (45 CFR 164.520)

☐ Approve ☐ Revise ☐ Retire Notes:

The mother of a 20-year-old university student calls the campus counseling center and demands her daughter's counseling records, saying FERPA gives her a right to them because she pays tuition. The records are used only for the student's treatment. What is the BEST response?

- A. Release the records, since FERPA gives parents of college students full access
At the postsecondary level FERPA rights belong to the student, not the parent.
- B. Release a summary only, since paying tuition gives the parent a partial FERPA right
Paying tuition does not create a FERPA access right to counseling records.
- C. Decline; these are treatment records, and the student's consent would be needed ✓ KEY
Records made and used only for treatment are excluded from FERPA education records.
- D. Release them after a 45-day wait, the timeline FERPA sets for record requests
The 45-day timeline applies to eligible students' own access requests, not to this parent.

Rationale: Under FERPA, records on a student who is 18 or older or in postsecondary education that are made by a mental health professional, used only for treatment, and shared only with treating providers are treatment records, excluded from education records. FERPA rights at a university belong to the student. Counselors protect these records and release them with the student's consent or as law allows.

References: FERPA — Definition of education records; exclusion of treatment records (34 CFR 99.3) · ACA Code of Ethics — B.6.g Disclosure or Transfer

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor provides text-based counseling through a secure platform. A new client prefers to use only a screen name and has not turned on video. Under the ACA Code of Ethics, what should the counselor do?

A. Take steps to verify the client's identity at the outset and during the counseling ✓ KEY

H.3 calls for verifying client identity at the beginning and throughout distance counseling.

B. Accept the screen name alone, since anonymity is a core benefit of online counseling

Anonymity does not remove the need to know who the client is, for example in emergencies.

C. Require the client to mail in a notarized identity form before any text exchange

This is an unnecessary barrier; the Code suggests simpler methods such as code words.

D. Decline to work by text, since the Code permits distance counseling by video only

The Code addresses many technologies, including text, and does not limit services to video.

Rationale: ACA H.3 directs counselors who use distance counseling to verify a client's identity at the beginning and throughout the process, for example with code words, numbers or other identifiers. Verification matters for consent, for protecting confidentiality, and for responding if the client is in crisis.

References: ACA Code of Ethics — H.3 Client Verification

☐ Approve ☐ Revise ☐ Retire Notes:

Q11 of 200 · nce-s-eth-107 · Professional Counseling Orientation and Ethical Practice · telehealth and technology ethics · hard · In review

A counselor has seen a 66-year-old client by video for two months. The client's hearing loss and unreliable internet now make sessions hard to follow, and both agree little progress is being made. The counselor's office is a short drive from the client. What does the ACA Code of Ethics direct?

A. Keep using video, since the client chose telehealth at the start and consent is set

Informed consent is ongoing, and the Code addresses what to do when distance services are not working.

B. Consider delivering services face-to-face, since distance services are not effective ✓ KEY

H.4.d directs counselors to consider face-to-face services when distance counseling is ineffective.

C. End counseling, because poor technology means the client cannot benefit from services

Ending services ignores an available in-person option that could meet the client's needs.

D. Switch to email only, since written messages avoid both hearing and connection issues

Email-only work is a major change that may not be effective and should not be imposed this way.

Rationale: ACA H.4.d states that when distance counseling services are deemed ineffective by the counselor or client, counselors consider delivering services face-to-face; if that is not possible, they help the client find appropriate services. H.4.e also asks counselors to consider access issues. Here an in-person option is close at hand.

References: ACA Code of Ethics — H.4.d Effectiveness of Services

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in private practice wants to use a free consumer video-chat app for telehealth sessions because clients already have it on their phones. What is the MAIN ethical concern?

A. Free apps cost too little to count as a legitimate professional business expense

Cost is not the ethical issue; security of client information is.

B. Clients may enjoy video sessions more than in-person ones and stop coming to the office

Client preference for telehealth is not an ethical problem.

C. Video sessions cannot be billed, so offering them would mislead clients about fees

Many payers reimburse telehealth; billing rules are not the main concern here.

D. The platform may not use encryption and safeguards that meet legal privacy standards ✓ KEY

H.2.d requires current encryption standards that meet applicable legal requirements.

Rationale: ACA H.2.d directs counselors to use current encryption standards in technology-based communications that meet applicable legal requirements. Under HIPAA, a covered counselor also needs appropriate safeguards and, typically, a business associate agreement with the platform vendor. Convenience does not outweigh protecting client confidentiality.

References: ACA Code of Ethics — H.2.d Security · HIPAA — Security Rule safeguards for electronic PHI and business associate agreements

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old client has met all of his goals for managing panic attacks and has had no symptoms for three months. He enjoys sessions and asks to keep coming every week "just to talk." What is the counselor's MOST appropriate response?

A. Discuss ending counseling, since he no longer appears to need the counselor's help ✓ KEY

A.11.c says to terminate when it is reasonably apparent the client no longer needs assistance.

B. Continue weekly sessions indefinitely, since the client wants them and pays for them

Continuing services that are no longer needed does not serve the client's welfare.

C. End services at the next session without discussion, since his goals have been met

Termination should be planned and discussed, not abrupt.

D. Set new goals for him so that continued weekly sessions can be justified to the payer

Creating goals to justify services puts the counselor's or payer's needs ahead of the client.

Rationale: ACA A.11.c directs counselors to terminate when it becomes reasonably apparent that clients no longer need assistance, are not likely to benefit, or are being harmed by continued counseling. Termination is a planned process that reviews gains, prepares for setbacks, and leaves the door open to return if new concerns arise.

References: ACA Code of Ethics — A.11.c Appropriate Termination

☐ Approve ☐ Revise ☐ Retire Notes:

In week five of a 12-week closed process group for young adults, a member privately tells the leader he plans to quit and will not come back. He says he feels criticized by other members. What should the leader do FIRST?

A. Insist that he finish all 12 weeks, as he agreed to that commitment when he joined

Members have the right to leave; the leader cannot require them to stay.

B. Tell the group next week that he quit because the others had criticized him too much

Disclosing his private reasons without consent would breach his trust.

C. Explore his reasons and encourage him to raise the decision in the group before leaving ✓ KEY

Discussing the decision in group can resolve the conflict and lets members process the ending.

D. Accept his choice at once and remove him from the group without any more discussion

Quick acceptance misses a chance to address the conflict and can harm the member and the group.

Rationale: A member's abrupt departure affects both the member and the group. Leaders respect a member's right to leave but explore the reasons and encourage the member to talk about the decision in the group, where interpersonal tension can often be worked through. Policies on leaving the group are best explained during pre-group orientation.

References: Yalom & Leszcz (Group Psychotherapy) — Premature termination: dropouts and discussing departure in the group · ASGW Best Practice Guidelines — Planning and performing: informing members of group policies and procedures

☐ Approve ☐ Revise ☐ Retire Notes:

A client in private practice has missed three payments despite reminders and a reduced fee. The counselor's consent form explains what happens when fees go unpaid. The client is not in crisis. Which course BEST fits the ACA Code of Ethics?

A. Keep seeing the client for free, since ending services over money is forbidden

A.11.c permits termination for nonpayment of agreed fees.

B. Stop scheduling sessions and send a letter, with no need for further discussion

Ending without pretermination counseling risks abandonment.

C. Hold the client's records until the balance is paid, then plan a referral after

Withholding records over a debt harms the client and is not an appropriate step.

D. End services for nonpayment, with pretermination counseling and referral options ✓ KEY

The Code allows termination for unpaid fees when the counselor provides pretermination counseling and referrals.

Rationale: ACA A.11.c permits counselors to terminate when clients do not pay fees as agreed, but they provide pretermination counseling and recommend other service providers when necessary. Explaining nonpayment procedures in informed consent at the start makes this process fair and predictable.

References: ACA Code of Ethics — A.11.c Appropriate Termination

☐ Approve ☐ Revise ☐ Retire Notes:

Four months into talk therapy, a counselor believes a structured exposure-based approach would help a client with social anxiety. The client signed a general consent form at intake. What should the counselor do before starting the new approach?

A. Start it next session, since the intake consent already covered the counseling services

General consent at intake does not cover a major change in modality.

B. Explain the approach, its risks and benefits, and the client's right to decline it ✓ KEY

A.2.a and A.2.b describe consent as ongoing and give clients the right to refuse modality changes.

C. Get a supervisor's approval, since the client cannot judge the merits of the technique

Consultation may help, but it does not replace the client's own informed choice.

D. Introduce the exercises gradually without labeling them so the client does not worry

Hiding the nature of an intervention undermines informed consent and trust.

Rationale: ACA A.2.a treats informed consent as an ongoing part of counseling that is documented throughout the relationship. A.2.b includes the client's right to be informed about techniques, risks and benefits, and to refuse any services or modality changes and be told the consequences. A new approach calls for a fresh consent discussion.

References: ACA Code of Ethics — A.2.a Informed Consent; A.2.b Types of Information Needed

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor will lead a 16-week group for adults ordered by a court to attend after domestic violence convictions. The probation office requires attendance and progress reports. What must the counselor explain to members before the group begins?

A. What information will be shared with the probation office and with whom it is shared ✓ KEY

A.2.e requires explaining to mandated clients what information is shared and with whom.

B. That everything said in group stays confidential, the same as in voluntary counseling

This misstates the limits; reports to probation are required in a mandated group.

C. That members have no right to refuse services because the court ordered the group

Mandated clients may refuse; counselors discuss the possible consequences of refusing.

D. That progress reports will be shared only if a member breaks one of the group's rules

Reports are routine in this setting, and members need to know that up front.

Rationale: ACA A.2.e directs counselors working with mandated clients to discuss the required limits of confidentiality and to explain what type of information is shared and with whom before counseling begins. Clients may still choose to refuse services, and counselors discuss the potential consequences. In groups, this pre-group orientation is part of informed consent.

References: ACA Code of Ethics — A.2.e Mandated Clients · ASGW Best Practice Guidelines — Planning: informed consent and orientation of prospective members

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor educator wants students in her practicum course to complete surveys for her study on counselor self-efficacy. What must she make clear to the students under the ACA Code of Ethics?

A. Taking part is a course requirement, since research is part of professional training

Making participation mandatory removes the voluntary choice the Code protects.

B. Students who take part will get extra credit that others cannot earn any other way

Credit available only through participation pressures students; an equal alternative is needed.

C. Students may decline, but declining will be noted in the course evaluation file

Recording refusals in evaluations links participation to academic standing.

D. Choosing not to take part will not affect their grades, and an alternative is offered ✓ **KEY**

The Code requires that participation not affect academic standing and that an alternative be offered.

Rationale: Under ACA Section G, researchers who involve students or supervisees make clear that the decision to participate does not affect academic standing or the supervisory relationship. Students who choose not to participate are given an appropriate alternative way to meet any requirement. This protects voluntary consent where a power imbalance exists.

References: ACA Code of Ethics — G.2 Rights of Research Participants (Student/Supervisee Participation) · Erford (Research and Evaluation in Counseling) — Informed consent and voluntary participation

☐ Approve ☐ Revise ☐ Retire Notes:

An IRB approved a study in which participants were told it measured "attention" when it really measured reactions to staged peer feedback. Data collection for each participant has just ended. What is the researcher's main obligation at this point?

A. Keep the true purpose hidden so that future participants will not learn of it from them

Protecting the design does not override participants' right to a full explanation.

B. Wait to explain the study until results are published so the findings stay unbiased

Clarification is owed after data collection, not delayed until publication.

C. Fully explain the study's nature and correct any misconceptions the deception caused ✓ **KEY**

The Code requires full clarification after data collection to remove misconceptions.

D. Send a brief thank-you note, since the IRB approval already covers the deception used

IRB approval does not replace the duty to debrief participants.

Rationale: ACA Section G directs that after data are collected, counselors give participants full clarification of the nature of the study to remove any misconceptions. When deception was used, this debriefing explains what was concealed and why. Where withholding information is justified, researchers take reasonable steps to avoid harm.

References: ACA Code of Ethics — G.2 Rights of Research Participants (Explanations After Data Collection) · Erford (Research and Evaluation in Counseling) — Deception and debriefing in research with human participants

☐ Approve ☐ Revise ☐ Retire Notes:

Midway through a timed achievement test given to a 14-year-old student, a fire alarm forces a 20-minute evacuation. The counselor resumes the test afterward. How should the counselor handle the results?

A. Score and report the results as usual, since the student finished all of the test

An interruption is an irregularity that may affect validity and must not be ignored.

B. Note the irregularity in the interpretation, and treat results as questionable ✓ KEY

The Code requires noting nonstandard conditions; results may be invalid or of questionable validity.

C. Add 20 minutes of credit to the student's raw score to make up for the lost time

Adjusting scores by hand has no basis in the norms and distorts the results.

D. Destroy the answer sheet and record that the student did not take the test at all

Destroying data and misreporting events is dishonest and is not required.

Rationale: Under ACA Section E, counselors give assessments under the conditions set during standardization. When irregularities occur during administration, those conditions are noted in interpretation, and results may be designated as invalid or of questionable validity. Retesting with an alternate form may be considered if appropriate.

References: ACA Code of Ethics — E.7 Conditions of Assessment Administration (administration conditions and irregularities) · Hays (Assessment) — Standardized administration and threats to validity

☐ Approve ☐ Revise ☐ Retire Notes:

A school team wants to decide a 12-year-old's placement using intelligence and achievement scores from testing done when the student was 7, on an edition of the test that has since been revised. Under the ACA Code of Ethics, what should the counselor advise?

A. Do not rely on those outdated results for this decision; seek current assessment data ✓ KEY

E.11 bars using obsolete or outdated assessment results for the current purpose.

B. Use the old scores, since intelligence is stable and the earlier testing was valid

Results from five years earlier on an older edition are outdated for today's decision.

C. Use the old scores but raise each by a few points to adjust for the student's growth

Ad hoc score adjustment has no psychometric basis.

D. Let the team decide, since placement is an education matter outside assessment ethics

Counselors have an ethical duty to prevent misuse of outdated assessment data by others.

Rationale: ACA E.11 states that counselors do not use data or results from assessments that are obsolete or outdated for the current purpose, such as noncurrent versions of instruments, and they make every effort to prevent others from misusing obsolete measures and data. High-stakes decisions call for current, appropriate assessment.

References: ACA Code of Ethics — E.11 Obsolete Assessment and Outdated Results

☐ Approve ☐ Revise ☐ Retire Notes:

While screening for a new depression support group, a counselor learns that one applicant is her next-door neighbor, whose children play with hers. The applicant is a good clinical fit. What is the BEST course of action?

A. Admit the neighbor without comment, since screening is about clinical fit and not the leader

Screening also weighs relationships that could harm the member or the group.

B. Admit the neighbor but avoid any contact at home for the full length of the group

Avoiding the neighbor is unrealistic and does not address the dual relationship itself.

C. Ask other members to vote on whether the neighbor may join at the first session

This exposes the applicant's private information and shifts the leader's duty to members.

D. Discuss the dual-relationship risks with the applicant and help find another group ✓ KEY

A pre-existing personal relationship is a reason to explore risks and refer to another group.

Rationale: ACA A.9.a asks group leaders to screen prospective members and select those whose well-being will not be jeopardized by the group experience. A close personal relationship with the leader creates a dual relationship that can compromise objectivity and the member's comfort in group. When an alternative exists, referral is usually best.

References: ACA Code of Ethics — A.9.a Screening · ASGW Best Practice Guidelines — Screening and selection of members; avoiding dual relationships

☐ Approve ☐ Revise ☐ Retire Notes:

Q23 of 200 · nce-s-eth-119 · Professional Counseling Orientation and Ethical Practice · ethical decision-making models · medium · In review

A counselor using Forester-Miller and Davis's Practitioner's Guide has clearly identified the problem: a client's employer wants session updates. According to the model, what should the counselor do NEXT?

A. Brainstorm as many possible courses of action as she can with a trusted colleague

Generating options is a later step in the model.

B. Pick the action she favors and test it against justice, publicity and universality

Evaluating a chosen action comes after options are generated and weighed.

C. Apply the ACA Code of Ethics to see whether it addresses or resolves the issue ✓ KEY

Applying the ACA Code is the second step, right after identifying the problem.

D. Carry out the most cautious response at once and record her reasons in the file

Implementing a course of action is the final step of the model.

Rationale: Forester-Miller and Davis outline seven steps: identify the problem, apply the ACA Code of Ethics, determine the nature and dimensions of the dilemma, generate potential courses of action, consider consequences and choose, evaluate the selected course, and implement it. If the Code clearly resolves the issue, the counselor follows it; if not, the process continues.

References: Forester-Miller & Davis (Practitioner's Guide to Ethical Decision Making) — Steps 1 and 2: identify the problem, then apply the ACA Code of Ethics

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who leads psychoeducational and therapy groups wants to join the ACA division that publishes best practice guidelines and training standards for group workers. Which organization should she join?

A. American Group Psychotherapy Association

AGPA is an independent interdisciplinary group, not an ACA division.

B. Association for Specialists in Group Work ✓ KEY

ASGW is the ACA division for group work and publishes best practice and training standards.

C. American Mental Health Counselors Association

AMHCA represents clinical mental health counselors, not group work specifically.

D. National Board for Certified Counselors

NBCC is a credentialing body, not an ACA division.

Rationale: The Association for Specialists in Group Work (ASGW) is the ACA division devoted to group work. It publishes the Best Practice Guidelines, the Professional Standards for the Training of Group Workers and the Multicultural and Social Justice Competence Principles for Group Workers. Leaders often use these documents in planning, performing and processing groups.

References: ASGW Best Practice Guidelines — ASGW as the ACA division for group work and its guidelines

☐ Approve ☐ Revise ☐ Retire Notes:

Intake, Assessment, and Diagnosis

24 items · NCE weight 12% (19 of 160 scored items)

Q25 of 200 · nce-s-ass-097 · Assessment and Testing · intake interview structure · medium · In review

A counselor is starting a first telehealth intake with a 29-year-old client who has joined the video call from a parked car. Before moving into the presenting concern, which step BEST belongs at the opening of this intake?

A. Ask the client to describe the main reason they are seeking counseling now

Exploring the presenting concern matters, but it follows the setup steps that make remote work safe.

B. Give a brief symptom checklist so the session can begin with numbers

A screening measure can be useful later; it does not address consent or safety for remote sessions.

C. Confirm where the client is right now and an emergency plan for the session ✓ KEY

Knowing the client's location and how to get help locally is part of informed consent and emergency planning for distance counseling.

D. Suggest the client wait to talk until they can return home to a private room

Privacy can be discussed collaboratively, but sending the client away skips the needed consent and safety steps.

Rationale: An intake opens with orienting tasks before clinical content: introductions, informed consent, limits of confidentiality, and, for distance counseling, verification of the client's identity and current location plus emergency procedures. Location matters because the counselor needs to know which local resources to contact if a crisis arises during the session. Only after this foundation does the interview move to the presenting problem and history.

References: ACA Code of Ethics — H.2.a Informed Consent and Disclosure (distance counseling, including emergency procedures); H.3 Client Verification

☐ Approve ☐ Revise ☐ Retire Notes:

Q26 of 200 · nce-s-ass-098 · Assessment and Testing · intake interview structure · easy · In review

Many intake forms ask the counselor to record the presenting problem, or chief complaint, as a short quotation in the client's own words. What is the main reason for this practice?

A. It preserves how the client sees the problem before any clinical framing ✓ KEY

A direct quote keeps the client's perspective and language on record, separate from the counselor's interpretation.

B. It lets the counselor skip the later questions about history and context

The quote opens the interview; history and context are still gathered afterward.

C. It is required so that a diagnosis can be billed to the client's insurer

Billing depends on a diagnosis and documentation of necessity, not on quoting the client.

D. It shows the counselor has accepted the client's view as the diagnosis

Recording the client's words does not mean adopting them as the clinical formulation.

Rationale: The presenting problem is usually recorded first and, where possible, in the client's own words. This documents what brought the client in from their point of view, which later helps the counselor check whether the treatment plan addresses the client's actual concern. The counselor's clinical impressions are recorded separately.

References: Wiger (Documentation) — Intake documentation: presenting problem in the client's own words

☐ Approve ☐ Revise ☐ Retire Notes:

During a mental status examination, a 31-year-old client with insomnia mentions that a few times, just as she was drifting off to sleep, she heard someone call her name and then realized no one was there. She reports no other unusual perceptions, and her thinking is organized. How should the counselor record this?

- A. As auditory hallucinations that meet the first criterion for schizophrenia
Perceptions that occur only while falling asleep are not counted as psychotic hallucinations.
- B. As a disorder of thought process, since the voice reflects loosened associations
Hearing a voice is a perceptual experience; thought process concerns how ideas are connected.
- C. As a delusion, since she believed for a moment that someone was in the room
A delusion is a fixed false belief; she corrected her impression immediately.
- D. As a hypnagogic experience, which is common and not in itself a sign of psychosis ✓ KEY
Brief perceptions at sleep onset are hypnagogic and fall within the range of normal experience.

Rationale: The perception section of the mental status examination notes hallucinations and illusions, but context matters. DSM-5-TR notes that hallucinations occurring while falling asleep (hypnagogic) or waking (hypnopompic) are within the range of normal experience. With organized thinking and no other perceptual disturbance, the counselor records the experience accurately without labeling it psychotic.

References: DSM-5-TR — Schizophrenia spectrum: key features — hallucinations; hypnagogic and hypnopompic experiences

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old client calmly tells the counselor that the evening news anchor has been sending him personal messages about a secret mission. In which part of the mental status examination does this belong?

- A. Perception
Perception covers hallucinations and illusions; he describes a belief about real broadcasts.
- B. Thought content ✓ KEY
Delusions and ideas of reference are recorded under thought content.
- C. Thought process
Thought process describes how ideas connect, such as tangential or logical.
- D. Orientation
Orientation covers awareness of person, place, and time.

Rationale: The mental status examination separates what a person thinks (thought content) from how the thoughts are organized (thought process). Beliefs that television or other media carry personal messages are delusions of reference and are recorded under thought content, along with obsessions, preoccupations, and suicidal or homicidal ideation.

References: Hays (Assessment) — Mental status examination: thought content vs thought process

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor reviews intake notes on a 52-year-old client. Which piece of information is a WARNING SIGN of possible near-term suicide risk rather than a longer-standing risk factor?

A. A brother died by suicide when the client was in their early twenties

Family history of suicide is an enduring risk factor, not a sign of imminent risk.

B. The client recently began giving away prized items and saying goodbye ✓ **KEY**

Giving away possessions and saying goodbye are recognized warning signs that call for immediate assessment.

C. The client has lived with a painful chronic back condition for ten years

Chronic pain raises risk over time but is a background factor, not an acute warning sign.

D. The client made a suicide attempt by overdose about fifteen years ago

A past attempt is a strong long-term risk factor, but it is historical rather than a current warning sign.

Rationale: Risk factors such as prior attempts, family history, and chronic illness raise risk over a lifetime; warning signs signal that risk may be rising now. Warning signs include talking about wanting to die or being a burden, seeking means, withdrawing, giving away possessions, and saying goodbye. A warning sign calls for direct questioning about ideation, plan, intent, and access to means in the current session.

References: SAMHSA 988 Lifeline — Suicide warning signs · C-SSRS — Screening for current ideation, intent, plan, and recent behavior

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old warehouse worker, furious about being passed over for promotion, says of his supervisor, "One of these days he's going to get what's coming to him." He then changes the subject. What should the counselor do FIRST?

A. Ask directly what he means, including any plan, means, and past violence ✓ **KEY**

An ambiguous remark calls for direct assessment of intent, plan, access to weapons, and history before any decision.

B. Call the supervisor to warn him, since a target has now been identified

Warning is considered only after assessment shows serious and foreseeable danger under the governing law.

C. Follow the change of subject so the client does not feel accused or judged

Letting a possible threat pass unexamined leaves the counselor unable to judge the level of risk.

D. Schedule a personality inventory to measure his general level of hostility

A trait measure cannot answer the immediate question of whether he intends to harm a specific person.

Rationale: When a client makes an ambiguous statement about harming someone, the counselor's first job is to clarify it: whether there is intent, a plan, a means such as a weapon, an identified victim, and a history of violence. That assessment determines whether the situation reaches the threshold of serious and foreseeable harm. Duties to warn or protect, and how they are carried out, depend on that finding and on state law.

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements · State law — Duty-to-warn and duty-to-protect statutes vary by jurisdiction

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old client in Minnesota has had major depressive episodes that began in late October and fully remitted by April in each of the last three years. There have been no depressive episodes at other times of year, and no clear yearly stressor explains the timing. Which specifier BEST fits?

A. With melancholic features

Melancholic features describe symptom quality, such as loss of pleasure and early waking, not timing.

B. With atypical features

Atypical features describe mood reactivity and related symptoms, not a link to time of year.

C. With peripartum onset

Peripartum onset applies to episodes during pregnancy or within four weeks after delivery.

D. With seasonal pattern ✓ KEY

Regular onset and remission at set times of year, with no nonseasonal episodes, fit this specifier.

Rationale: The seasonal pattern specifier applies to recurrent major depressive disorder (and to bipolar episodes) when onset and full remission occur at characteristic times of year. In the last two years there must be at least two episodes showing the seasonal relationship and no nonseasonal episodes, and seasonal episodes must substantially outnumber nonseasonal ones over the lifetime. Timing explained by an obvious seasonal stressor, such as yearly unemployment, does not count.

References: DSM-5-TR — Depressive disorders: specifier "with seasonal pattern"

☐ Approve ☐ Revise ☐ Retire Notes:

For 14 months, a 26-year-old has had depressed mood plus four other depressive symptoms lasting about three days, roughly once a month, unrelated to the menstrual cycle. She has never met criteria for another depressive or bipolar disorder. The counselor wants the diagnosis to state why full criteria are not met. Which diagnosis fits?

A. Unspecified depressive disorder, with the reason left unstated

The unspecified category is used when the clinician chooses not to state the reason or lacks the information.

B. Major depressive disorder, recurrent, with the provisional label

Provisional is for a strong expectation that full criteria will be met; three-day episodes cannot meet the two-week rule.

C. Other specified depressive disorder (recurrent brief depression) ✓ KEY

The other specified category lets the clinician name the specific reason, here recurrent brief depression.

D. Adjustment disorder with depressed mood, recurrent episodes type

Adjustment disorder requires an identifiable stressor and has no recurrent subtype.

Rationale: DSM-5-TR offers two residual categories in each chapter. "Other specified" is used when the clinician chooses to communicate the specific reason criteria are not met, by recording the name of the category followed by the reason; "unspecified" is used when the clinician chooses not to give a reason or lacks enough information. Recurrent brief depression, with full-symptom episodes of 2 to 13 days at least monthly for 12 months, is a listed example of other specified depressive disorder.

References: DSM-5-TR — Other specified depressive disorder (recurrent brief depression); use of other specified vs unspecified categories

☐ Approve ☐ Revise ☐ Retire Notes:

An early intervention team wants an individually administered measure of cognitive, language, and motor development for an 18-month-old born prematurely. Which instrument is designed for a child this age?

A. Bayley Infant and Toddler Scales ✓ KEY

The Bayley scales assess cognitive, language, and motor development from early infancy to about 42 months.

B. Wechsler Intelligence Scale (WISC-V)

The WISC-V is for children aged 6 through 16 and is far above this child's age.

C. Wechsler Preschool Scale (WPPSI-IV)

The WPPSI-IV begins at 2 years 6 months, so it does not reach an 18-month-old.

D. Peabody Picture Vocabulary Test-5

The PPVT measures receptive vocabulary only, not cognitive and motor development.

Rationale: Infant and toddler assessment uses developmental scales rather than conventional intelligence tests. The Bayley Scales of Infant and Toddler Development cover roughly the first three and a half years and assess cognitive, language, and motor domains, with social-emotional and adaptive behavior gathered by caregiver report. Scores in infancy are less predictive of later IQ than scores in school age, so results guide early intervention rather than fixed labels.

References: Hays (Assessment) — Assessment of infants and young children: Bayley Scales · Berk (Development Through the Lifespan) — Infant intelligence tests and their predictive value

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at an adult education program wants a brief, individually administered screen of basic academic skills, such as word reading, spelling, and math computation, for a 38-year-old returning learner. Which instrument fits this purpose?

A. Wechsler Adult Intelligence Scale-IV

The WAIS-IV measures intellectual ability, not acquired academic skills.

B. Raven's Progressive Matrices

Raven's measures nonverbal reasoning with visual patterns, not academic skills.

C. Vineland Adaptive Behavior Scales-3

The Vineland measures adaptive behavior in daily life, not reading or math skills.

D. Wide Range Achievement Test-5 ✓ KEY

The WRAT-5 is a brief achievement measure of word reading, comprehension, spelling, and math computation.

Rationale: Achievement tests measure what a person has learned, while intelligence and aptitude tests estimate ability or potential to learn. The Wide Range Achievement Test is a short, individually administered achievement screen covering reading, spelling, and math computation across a wide age range. A full intelligence test or an adaptive behavior scale answers a different referral question.

References: Hays (Assessment) — Achievement testing: individually administered screening batteries

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor asks an 8-year-old to "draw a picture of everyone in your family, including you, doing something." Which projective technique is this?

A. House-Tree-Person

The House-Tree-Person asks for separate drawings of a house, a tree, and a person.

B. Kinetic Family Drawing ✓ **KEY**

The Kinetic Family Drawing asks for the whole family shown doing something, to reveal relationships and roles.

C. Bender-Gestalt Test

The Bender-Gestalt asks the person to copy geometric designs to assess visual-motor skill.

D. Draw-A-Person Test

The Draw-A-Person asks for a single human figure, not the family in action.

Rationale: In the Kinetic Family Drawing, the "kinetic" element is the instruction to show family members doing something. Clinicians look at who is included, relative size and placement, interaction, and activity as hypotheses about how the child experiences the family. Like other drawing techniques, it is best used to open conversation rather than to diagnose.

References: Hays (Assessment) — Projective drawing techniques: Kinetic Family Drawing

☐ Approve ☐ Revise ☐ Retire Notes:

A psychological report states that the Rorschach was administered and coded using the Rorschach Performance Assessment System (R-PAS), with the results compared against international reference norms. What does this description tell the counselor reading the report?

A. The client told stories about pictures that were scored for needs and press

Storytelling scored for needs and press describes Murray's Thematic Apperception Test.

B. The examiner read the inkblot responses freely with no set coding rules

R-PAS exists precisely to replace free interpretation with standardized coding.

C. The inkblots were given and coded with a standardized, normed system ✓ **KEY**

R-PAS specifies administration and coding rules and compares scores with reference norms.

D. A true-false self-report inventory was given in place of the inkblots

The report says the Rorschach itself was administered; R-PAS is a way of scoring it.

Rationale: The Rorschach has been used with several scoring approaches. Exner's Comprehensive System standardized administration and coding for decades, and R-PAS, developed later by researchers including Meyer and Mihura, updated the coding with standardized procedures and norm-referenced scores. These systems improve reliability compared with free interpretation, though the Rorschach remains a performance-based measure whose results are integrated with other data.

References: Hays (Assessment) — Projective and performance-based personality assessment: Rorschach scoring systems

☐ Approve ☐ Revise ☐ Retire Notes:

A 60-year-old client has taken a prescribed opioid exactly as directed for chronic back pain for two years. She needs a higher dose than at first for the same relief and feels ill if she misses a dose. She reports no other problems with the medication. How should these two findings be weighed toward opioid use disorder?

A. They are not counted, because she takes the drug as prescribed ✓ **KEY**

DSM-5-TR excludes tolerance and withdrawal from the count when an opioid is taken solely under medical supervision.

B. They count as two criteria, enough for a mild opioid use disorder

Two criteria would mean mild severity, but these two are not counted when use is as prescribed.

C. They count, but only once she has used the drug for over a year

Duration of use does not change the exclusion for appropriately prescribed medication.

D. They count as one criterion, since both reflect body adaptation

Tolerance and withdrawal are separate criteria; the issue is whether they count at all.

Rationale: Tolerance and withdrawal are expected physiological effects of long-term opioid treatment. DSM-5-TR therefore notes that these two criteria are not considered met for people taking opioids solely under appropriate medical supervision. A diagnosis would require other criteria, such as using more than prescribed, craving, or continued use despite harm.

References: DSM-5-TR — Opioid use disorder, Criteria 10 and 11 notes (tolerance and withdrawal under medical supervision)

☐ Approve ☐ Revise ☐ Retire Notes:

To get a detailed picture of a client's drinking, a counselor sits with him and a calendar of the past 90 days. Using paydays, holidays, and family events as memory anchors, they estimate how many drinks he had on each day. Which method is the counselor using?

A. The CAGE questionnaire

The CAGE is four yes-or-no screening questions, not a day-by-day record.

B. The AUDIT screening test

The AUDIT is a 10-item screen, not a calendar-based recall method.

C. Addiction Severity Index

The ASI is a structured interview rating problem severity across life areas.

D. The Timeline Followback ✓ **KEY**

The Timeline Followback uses a calendar and memory anchors to reconstruct daily use.

Rationale: The Timeline Followback is a calendar-based interview that reconstructs day-by-day substance use over a set window, often 30 to 90 days, using personal events to aid recall. It yields detailed data on frequency, quantity, and patterns such as weekend binges, and it can be repeated to track change. Screening tools such as the CAGE and AUDIT only flag possible problems.

References: Sobell & Sobell (Timeline Followback) — Calendar method with memory anchors for daily use estimates · Hays (Assessment) — Substance use assessment: structured and calendar-based methods

☐ Approve ☐ Revise ☐ Retire Notes:

A 17-year-old who arrived in the United States as a refugee eight months ago is referred for sleep problems and irritability. When taking the biopsychosocial history, which approach to the migration history is MOST complete?

A. Focus on current school stress, since past events are better left for later
Leaving out the migration history can miss trauma and losses that explain current symptoms.

B. Ask mainly about the war and danger that led the family to leave home
Pre-migration events matter, but stressors during flight and after arrival are also important.

C. Explore life before leaving, the journey itself, and life since arriving ✓ **KEY**
Covering pre-migration, migration, and post-migration phases captures the full range of stressors and supports.

D. Ask about legal status first, since it decides which services are offered
Opening with legal status can frighten the client and is not the core of a clinical history.

Rationale: Refugee and immigrant clients may face stressors in three phases: before leaving (violence, persecution, loss), during migration (danger, camps, separation from family), and after arrival (acculturation, discrimination, language barriers, poverty). A biopsychosocial history that covers all three, along with strengths and supports, gives a fuller picture than one focused on a single phase. Questions should be paced with care to avoid retraumatizing the client.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling refugees and immigrants: pre-migration, migration, and post-migration stressors

☐ Approve ☐ Revise ☐ Retire Notes:

A biopsychosocial intake with a 20-year-old college student shows that her mother has an anxiety disorder, panic attacks began after a car accident last month, she now skips classes held in the building where her first attack happened, and she is close to her older sister. Which finding is a PERPETUATING factor?

A. Her mother's long history of being treated for an anxiety disorder
Family history is a predisposing factor that raised vulnerability before the problem began.

B. Skipping classes held in the building where her first attack happened ✓ **KEY**
Avoidance keeps the fear from being disconfirmed, so it maintains the problem over time.

C. The car accident last month that came just before the first attack
An event that triggered onset is a precipitating factor, not one that maintains the problem.

D. Her close and supportive relationship with her older sister at home
A supportive relationship is a protective factor that can be drawn on in treatment.

Rationale: Biopsychosocial data are often organized into predisposing factors (vulnerabilities such as family history), precipitating factors (events that triggered onset), perpetuating factors (what keeps the problem going), and protective factors (strengths and supports). Perpetuating factors are especially useful for treatment planning because many can be changed; here, avoidance maintains panic and would be a target for exposure-based work.

References: Hays (Assessment) — Integrating intake and history data into a case conceptualization

☐ Approve ☐ Revise ☐ Retire Notes:

A client's score on a self-report scale converts to a z-score of -1.5. What is the equivalent T-score?

A. 15

This multiplies by 10 without adding the mean of 50.

B. 45

This treats -1.5 as half a standard deviation below the mean.

C. 65

This places the score 1.5 standard deviations above the mean.

D. 35 ✓ KEY

$T = 50 + 10(-1.5) = 50 - 15 = 35$.

Rationale: T-scores have a mean of 50 and a standard deviation of 10, so $T = 50 + 10z$. A z-score of -1.5 lies one and a half standard deviations below the mean, which is 15 T-score points below 50, giving a T-score of 35.

References: Hays (Assessment) — Standard scores: z-scores and T-scores

☐ Approve ☐ Revise ☐ Retire Notes:

Q42 of 200 · nce-s-ass-114 · Research and Program Evaluation · standard scores, percentiles and the normal curve · medium · In review

A normally distributed test has a mean of 50 and a standard deviation of 15. A client earns a raw score of 65. Approximately what is the client's percentile rank?

A. About the 84th percentile ✓ KEY

The z-score is $(65 - 50) / 15 = +1.0$; about $50\% + 34\% = 84\%$ of scores fall below it.

B. About the 65th percentile

This confuses the raw score with the percentile rank.

C. About the 98th percentile

The 98th percentile corresponds to about $z = +2.0$, two standard deviations above the mean.

D. About the 68th percentile

About 68% of scores fall between -1 and +1 SD; that is not the percentile for $z = +1$.

Rationale: First convert to a z-score: $(65 - 50) / 15 = +1.0$. In a normal distribution, 50% of scores fall below the mean and about 34% fall between the mean and +1 standard deviation, so a z-score of +1.0 corresponds to roughly the 84th percentile. Confusing this with the 68% that falls within one standard deviation on both sides is a common error.

References: Hays (Assessment) — The normal curve: converting z-scores to percentile ranks

☐ Approve ☐ Revise ☐ Retire Notes:

A test has a standard deviation of 10 and a reliability coefficient of .91. A client obtains a score of 60. Within which range does the client's true score fall with about 68% confidence?

A. 54 to 66

This is roughly the 95% band, using about two SEMs on each side.

B. 51 to 69

This uses three SEMs on each side, far wider than a 68% band.

C. 57 to 63 ✓ KEY

SEM = 10 x the square root of .09 = 3, so 60 plus or minus 3 gives 57 to 63.

D. 59 to 61

This uses $10 \times .09 = 0.9$ and forgets the square root in the formula.

Rationale: The standard error of measurement is SD multiplied by the square root of $(1 - r)$: $10 \times \text{the square root of } .09 = 10 \times .3 = 3$. About 68% of the time the true score lies within one SEM of the obtained score (60 plus or minus 3, or 57 to 63), and about 95% of the time within roughly two SEMs (about 54 to 66). Reporting a band rather than a single score reminds clients that every obtained score contains error.

References: Hays (Assessment) — Standard error of measurement and confidence intervals

☐ Approve ☐ Revise ☐ Retire Notes:

In career counseling, a 44-year-old client completes a work values exercise in which she must rank 12 values from most to least important. Security ranks first and Prestige ranks last. Which interpretation is accurate?

A. She values security more strongly than most adults in a comparison norm group

A forced ranking has no norm group comparison, so it cannot show standing relative to others.

B. Security outranks her other values; it says nothing about other people ✓ KEY

Ranked (ipsative) results show the relative order of values within one person only.

C. She has met a set cutoff that marks mastery of career values clarity

A values ranking has no mastery criterion; it is not a criterion-referenced test.

D. Prestige is of no importance to her, as it was ranked in last place

Last place means least important of the 12, not unimportant; all values may matter to some degree.

Rationale: Ipsative measures, such as forced-choice or ranking formats, compare a person's preferences with one another, so the results show what matters most and least to that individual. Because every person's rankings sum to the same total, ipsative results cannot be used to compare one person's strength of a value with other people's, which requires normative scoring. They are useful for clarifying priorities in career decisions.

References: Hays (Assessment) — Ipsative vs normative score interpretation · Sharf (Career Development Theory) — Assessing work values in career counseling

☐ Approve ☐ Revise ☐ Retire Notes:

Seven months after her husband's death, a 66-year-old client reports intense yearning for him nearly every day, trouble accepting the death, and feeling that part of her has died. These reactions impair her daily functioning. Why would prolonged grief disorder not be diagnosed at this point?

A. For adults, at least 12 months must have passed since the death ✓ **KEY**

DSM-5-TR requires that at least 12 months have elapsed since the death for adults (6 months for children).

B. Yearning for the person who died is not one of its core symptoms

Intense yearning or preoccupation with the deceased is a core required symptom.

C. The diagnosis applies only when the death was sudden or violent

Prolonged grief disorder does not require a particular manner of death.

D. It is listed only in Section III as a condition for further study

In DSM-5-TR, prolonged grief disorder is an official diagnosis in Section II.

Rationale: DSM-5-TR added prolonged grief disorder to the trauma- and stressor-related disorders. For adults, at least 12 months must have passed since the death (at least 6 months for children and adolescents), with intense yearning or preoccupation plus additional symptoms such as identity disruption or marked emotional pain. Before that point, intense grief is usually understood as an expected response, though the counselor should still assess for a co-occurring major depressive episode.

References: DSM-5-TR — Prolonged grief disorder, Criterion A (time since death)

☐ Approve ☐ Revise ☐ Retire Notes:

In a primary care behavioral health program, a 35-year-old patient scores 12 on the GAD-7. How should the counselor interpret and use this result?

A. As severe anxiety that confirms a diagnosis of generalized anxiety disorder

A score of 12 falls in the moderate range, and no screening score confirms a diagnosis.

B. As minimal anxiety, since only scores above 20 call for any further review

The maximum score is 21; scores of 10 or more are a common cutoff for further evaluation.

C. As a diagnosis of panic disorder, since the GAD-7 measures panic frequency

The GAD-7 rates generalized anxiety symptoms; it does not count panic attacks.

D. As moderate anxiety that calls for a fuller diagnostic assessment to follow ✓ **KEY**

Scores of 10 to 14 are moderate, and a positive screen leads to a clinical diagnostic evaluation.

Rationale: The GAD-7 is a seven-item screen scored 0 to 21, with cutoffs of 5, 10, and 15 marking mild, moderate, and severe anxiety. A score of 10 or higher is commonly used to flag possible clinically significant anxiety. Like all screening instruments, it identifies who needs further assessment; a diagnosis rests on a clinical interview against DSM-5-TR criteria.

References: Hays (Assessment) — Brief screening instruments: GAD-7 scoring and interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in an adolescent treatment program wants an MMPI-family inventory for a 16-year-old client. Which version is normed for this age group?

A. MMPI-3

The MMPI-3 is normed for adults aged 18 and older.

B. MMPI-2

The MMPI-2 is an adult instrument for ages 18 and older.

C. MMPI-A-RF ✓ KEY

The MMPI-A-RF is the restructured form normed for adolescents aged 14 to 18.

D. MMPI-2-RF

The MMPI-2-RF is the restructured form for adults, not adolescents.

Rationale: The MMPI family includes separate adult and adolescent versions. The MMPI-2, MMPI-2-RF, and MMPI-3 are normed on adults, while the MMPI-A and its restructured form, the MMPI-A-RF, are normed for adolescents aged 14 to 18. Using an adult form with a 16-year-old would compare the client with the wrong norm group.

References: Hays (Assessment) — Minnesota Multiphasic Personality Inventory: adult and adolescent versions

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor needs to reassess a client's reading comprehension three weeks after a first testing and worries that the client will remember specific passages and answers. The test publisher offers two versions built to the same specifications. Which kind of reliability evidence matters MOST for this plan?

A. Interrater reliability between two independent scorers

Interrater reliability concerns agreement between scorers, not equivalence of two versions.

B. Alternate-forms reliability between the two test versions ✓ KEY

Switching versions is sound only if the two forms are shown to yield equivalent scores.

C. Internal consistency of the items within a single version

Internal consistency shows items hang together but says nothing about whether forms match.

D. Test-retest reliability of the same version over 3 weeks

Reusing the same version invites the memory effects the counselor wants to avoid.

Rationale: Alternate-forms (parallel-forms) reliability is estimated by giving two equivalent versions of a test to the same people and correlating the scores. It is the relevant evidence when a counselor plans to retest with a different form to reduce practice and memory effects. Test-retest reliability uses the same form twice and is vulnerable to exactly those effects.

References: Hays (Assessment) — Reliability: alternate-forms (parallel-forms) reliability

☐ Approve ☐ Revise ☐ Retire Notes:

Areas of Clinical Focus

59 items · NCE weight 29% (47 of 160 scored items)

Q49 of 200 · nce-s-clf-237 · Assessment and Testing · depressive disorders · hard · In review

A 46-year-old who drank heavily every day for two years developed low mood, poor sleep and hopelessness during that period. He stopped drinking and completed withdrawal six weeks ago. His depressive symptoms have not eased since then. Which conclusion is BEST supported?

A. Alcohol-induced depressive disorder fits, since the mood began while he drank
Onset during use raises the question, but symptoms that persist well past withdrawal argue against a purely induced disorder.

B. No depressive diagnosis fits until he stays abstinent for a full twelve months
DSM-5-TR does not require a year of abstinence; about one month after withdrawal is the usual benchmark.

C. An independent depressive disorder is likely, as it has lasted past one month ✓ **KEY**
Persistence for a substantial period (about one month) after withdrawal ends is evidence of an independent depressive disorder.

D. His symptoms show a protracted alcohol withdrawal that will soon clear alone
Acute withdrawal resolves within days; six weeks of unchanged depression should not be assumed to be withdrawal.

Rationale: Substance/medication-induced depressive disorder is diagnosed when depressive symptoms develop during or soon after intoxication or withdrawal and are not better explained by an independent depressive disorder. DSM-5-TR names persistence of symptoms for a substantial period, about one month, after acute withdrawal or severe intoxication ends as evidence of an independent disorder. Ongoing reassessment after abstinence is key to the differential.

References: DSM-5-TR — Substance/Medication-Induced Depressive Disorder: Criterion C (evidence of an independent depressive disorder)

☐ Approve ☐ Revise ☐ Retire Notes:

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Parents of a 9-year-old say that since age 7 he has had severe temper outbreaks about four times a week at home and school, and is angry and irritable most of the day between outbursts. He has had no distinct periods of elevated mood or decreased need for sleep. Which diagnosis BEST fits?

A. Disruptive mood dysregulation disorder, given the chronic irritability ✓ **KEY**
Frequent severe outbursts plus persistently irritable mood for 12+ months in two settings, onset before 10, fits DMDD.

B. Bipolar I disorder, given his frequent rages and his very irritable mood
Pediatric bipolar disorder is episodic with distinct manic periods; he shows chronic, nonepisodic irritability.

C. Oppositional defiant disorder, as the outbursts occur in two settings
When DMDD criteria are met, ODD is not diagnosed separately; the severe persistent mood component points to DMDD.

D. Intermittent explosive disorder, due to his repeated angry outbursts
IED lacks the persistently irritable mood between outbursts, and it is not diagnosed when DMDD criteria are met.

Rationale: DMDD requires severe recurrent temper outbursts averaging three or more times per week, with persistently irritable or angry mood most of the day nearly every day between them, for 12 months or more in at least two settings. Onset is before age 10, and the diagnosis is not first made before age 6 or after age 18. It was added to the depressive disorders partly to reduce overdiagnosis of bipolar disorder in children with chronic, nonepisodic irritability.

References: DSM-5-TR — Disruptive Mood Dysregulation Disorder: Criteria A-K

☐ Approve ☐ Revise ☐ Retire Notes:

A 23-year-old had one manic episode last year that led to a brief hospital stay. She has never had a major depressive episode. A classmate in her counseling program says she "cannot have bipolar disorder without depression." Under DSM-5-TR, what is required for bipolar I disorder?

A. At least one manic episode plus at least one major depressive episode

This describes a common course, but a major depressive episode is not required for bipolar I disorder.

B. At least one hypomanic episode plus at least one major depressive episode

These are the requirements for bipolar II disorder, which has no lifetime manic episode.

C. Two or more manic episodes separated by at least two months of remission

A single manic episode suffices; recurrence and remission intervals are not required for the diagnosis.

D. At least one manic episode, with or without any past depressive episodes ✓ KEY

Bipolar I requires criteria for at least one manic episode; hypomanic or depressive episodes may occur but are not required.

Rationale: Bipolar I disorder is diagnosed when criteria have been met for at least one manic episode that is not better explained by a schizophrenia spectrum disorder. The manic episode may be preceded or followed by hypomanic or major depressive episodes, but these are not required. Bipolar II, in contrast, requires at least one hypomanic and at least one major depressive episode and no manic episode.

References: DSM-5-TR — Bipolar I Disorder: Criterion A and text on manic, hypomanic and depressive episodes

☐ Approve ☐ Revise ☐ Retire Notes:

A 37-year-old meets full criteria for a current manic episode. During most days of the episode she also feels intensely sad, has lost interest in her usual hobbies, feels worthless, and has recurrent thoughts of death. Which specifier is MOST appropriate?

A. With anxious distress, because she shows marked emotional turmoil daily

Anxious distress requires symptoms like feeling keyed up, tense, restless or fearful, not depressive symptoms.

B. With mixed features, because of her concurrent depressive symptoms ✓ KEY

Three or more depressive symptoms on most days of a manic episode meet the mixed features specifier.

C. With rapid cycling, because her mood moves between high and low

Rapid cycling requires four or more mood episodes in 12 months, not simultaneous symptoms in one episode.

D. With melancholic features, as she has lost interest in her hobbies

Melancholic features is a specifier for major depressive episodes, not for a current manic episode.

Rationale: The "with mixed features" specifier applies to a manic or hypomanic episode when at least three depressive symptoms (such as dysphoria, diminished interest, worthlessness or guilt, fatigue, psychomotor retardation, or thoughts of death) are present nearly every day during the episode. Mixed states carry elevated suicide risk, so counselors should assess safety closely. The specifier can also apply to major depressive episodes with concurrent manic or hypomanic symptoms.

References: DSM-5-TR — Bipolar and Related Disorders specifiers: with mixed features (manic or hypomanic episode)

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old has, for the past 7 months, felt intense dread whenever his spouse travels, called her many times a day to check that she is safe, and refused overnight work trips. He worries mainly that an accident will take her from him, not about many topics. Which diagnosis BEST fits?

A. Generalized anxiety disorder, since his worry has lasted over six months

GAD involves worry about many events or activities; his fear centers on separation from one attachment figure.

B. Separation anxiety disorder, since adult duration is typically 6 months ✓ **KEY**

Excessive fear of harm befalling an attachment figure, with reluctance to be apart, lasting typically 6+ months in adults fits.

C. Agoraphobia, since he avoids travel and being away from home overnight

Agoraphobic avoidance stems from fear that escape or help will be unavailable, not fear of losing a loved one.

D. No anxiety disorder, as separation anxiety is diagnosed only in childhood

DSM-5-TR permits separation anxiety disorder at any age; onset in adulthood is recognized.

Rationale: Separation anxiety disorder is developmentally inappropriate, excessive fear or anxiety about separation from attachment figures. The duration is at least 4 weeks in children and adolescents and typically 6 months or more in adults. DSM-5-TR removed the earlier requirement of onset before age 18, so adults whose fears center on harm to a partner or child may meet criteria.

References: DSM-5-TR — Separation Anxiety Disorder: Criteria A and B (duration in children vs adults)

☐ Approve ☐ Revise ☐ Retire Notes:

For 7 months, a 10-year-old has worried more days than not about grades, storms, her family's money and being late. She finds the worry hard to control and has trouble sleeping, but no other physical or associated symptoms. How many associated symptoms does DSM-5-TR require for a child to meet GAD criteria?

A. Three of the six associated symptoms, the same number used for adults

Three symptoms are required for adults; DSM-5-TR lowers the threshold for children.

B. Two of the six associated symptoms, one physical and one cognitive

DSM-5-TR sets no two-symptom rule and does not require a physical and cognitive mix.

C. Only one of the six associated symptoms, such as disturbed sleep ✓ **KEY**

For children, only one of the six associated symptoms is required, so her sleep problem can satisfy the criterion.

D. Four of the six associated symptoms, as worry in youth is so common

No criterion raises the threshold for youth; the requirement is lower for children, not higher.

Rationale: Generalized anxiety disorder requires excessive, hard-to-control worry about a number of events or activities on more days than not for at least 6 months. Adults need three or more of six associated symptoms (restlessness, fatigue, poor concentration, irritability, muscle tension, sleep disturbance), but children need only one. Children with GAD often worry about school performance, catastrophes and punctuality and may seek frequent reassurance.

References: DSM-5-TR — Generalized Anxiety Disorder: Criterion C (note: only one item required in children)

☐ Approve ☐ Revise ☐ Retire Notes:

A 3-year-old spent his first 2 years in a series of neglectful placements. Now with stable foster parents, he rarely seeks comfort when hurt and barely responds when they offer it. He shows little positive emotion and has unexplained bouts of fearfulness. He uses language and play typical for his age. Which diagnosis is MOST likely?

A. Reactive attachment disorder linked to his early social neglect ✓ KEY

Rarely seeking or responding to comfort, limited positive affect and unexplained fearfulness after neglect fit RAD.

B. Disinhibited social engagement disorder from his early care setting

DSED involves overly familiar approach to unfamiliar adults, not withdrawal and failure to seek comfort.

C. Autism spectrum disorder based on his limited social response

His age-typical language and play and the clear history of neglect point away from autism and toward RAD.

D. Posttraumatic stress disorder for children six years and under

No intrusion or avoidance tied to a specific traumatic event is described; the core issue is absent attachment behavior.

Rationale: Reactive attachment disorder is a consistent pattern of inhibited, emotionally withdrawn behavior toward caregivers: the child rarely seeks or responds to comfort when distressed, with limited positive affect and unexplained irritability, sadness or fear. It follows extremes of insufficient care, appears before age 5, and requires a developmental age of at least 9 months. Unlike autism spectrum disorder, RAD does not involve restricted, repetitive behaviors, and language and play are not specifically impaired.

References: DSM-5-TR — Reactive Attachment Disorder: Criteria A-G; differential diagnosis with autism spectrum disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A 51-year-old reports intrusive images, nightmares and avoidance since learning by phone that her brother died. Which detail would determine whether the death can meet PTSD Criterion A through learning about it?

A. Whether she had a close, loving relationship with her brother for years

A close relationship is needed, but closeness alone does not settle whether the event qualifies.

B. Whether she learned of the death within a day after it happened

DSM-5-TR sets no time window for learning about the event in Criterion A.

C. Whether her symptoms began within the first month after the death

Symptom timing bears on the delayed-expression specifier, not on whether the event qualifies under Criterion A.

D. Whether the death was violent or accidental rather than from illness ✓ KEY

When learning of a close other's actual or threatened death, the event must have been violent or accidental.

Rationale: PTSD Criterion A can be met by learning that a traumatic event happened to a close family member or close friend. When the event involves actual or threatened death, it must have been violent or accidental; a death from natural causes such as illness does not qualify. Intense distress after an expected or natural death may instead be assessed as grief, prolonged grief disorder, adjustment disorder or a depressive disorder.

References: DSM-5-TR — Posttraumatic Stress Disorder: Criterion A3 (learning of violent or accidental death)

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old attorney insists that every memo follow her exact formatting rules, redoes her team's work, and works so many hours she has no hobbies. She sees her standards as correct and is puzzled that others complain. She has no intrusive, unwanted thoughts or rituals. Which feature MOST points to obsessive-compulsive personality disorder rather than OCD?

A. Her rigid perfectionism feels right to her and has no true obsessions ✓ **KEY**

OCPD is an ego-syntonic, pervasive perfectionism without the intrusive obsessions and compulsions of OCD.

B. Her drive for order is so strong that it interferes with her work life

Impairment occurs in both OCD and OCPD, so it does not distinguish the two disorders.

C. She spends more than one hour a day on her work and her formatting

The one-hour marker is an OCD time-consuming example, and long work hours alone do not separate the disorders.

D. She is perfectionistic only at work and is relaxed with her friends

A personality disorder is pervasive across contexts; a work-limited pattern would argue against OCPD.

Rationale: Obsessive-compulsive personality disorder is a pervasive pattern of preoccupation with orderliness, perfectionism and control that the person usually experiences as reasonable (ego-syntonic). OCD is defined by intrusive, unwanted obsessions and compulsions performed to reduce distress. When both sets of criteria are met, both can be diagnosed, but the absence of true obsessions and compulsions points to OCPD.

References: DSM-5-TR — Obsessive-Compulsive Personality Disorder: differential diagnosis with obsessive-compulsive disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old pulls out her eyebrow hairs while studying, leaving visible bare patches. She feels relief as she pulls, has tried many times to stop, and is embarrassed by how her face looks. She has no concerns about a flaw in her appearance before pulling. Which diagnosis BEST fits?

A. Body dysmorphic disorder, since she is upset about how her face looks

In BDD, hair removal aims to fix a perceived defect; her distress follows the pulling rather than driving it.

B. Excoriation disorder, since she has repeatedly tried to stop pulling

Excoriation disorder involves recurrent skin picking that causes lesions, not hair pulling.

C. Trichotillomania, given hair loss from pulling and attempts to stop ✓ **KEY**

Recurrent hair pulling causing hair loss, with repeated efforts to stop and distress, meets trichotillomania criteria.

D. Obsessive-compulsive disorder, since pulling eases her inner tension

Her pulling is not driven by obsessions or rules, so a body-focused repetitive behavior disorder fits better.

Rationale: Trichotillomania (hair-pulling disorder) requires recurrent pulling out of one's hair resulting in hair loss, repeated attempts to decrease or stop, and clinically significant distress or impairment. Pulling is often preceded by tension or boredom and followed by relief or gratification. It is distinguished from BDD, where hair is removed to correct a perceived appearance flaw, and from OCD, where behavior responds to obsessions.

References: DSM-5-TR — Trichotillomania (Hair-Pulling Disorder): Criteria A-E and differential diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old who smoked cannabis several times a day for two years stopped 4 days ago. She reports irritability, anxiety, trouble sleeping with vivid dreams, poor appetite and restlessness. She says, "Weed isn't addictive, so this must be something else." How should the counselor understand her symptoms?

A. As a new anxiety disorder, because cannabis has no withdrawal state

DSM-5-TR includes cannabis withdrawal; attributing her symptoms to a new disorder ignores the clear timing.

B. As likely cannabis withdrawal, which DSM-5-TR recognizes in heavy use ✓ **KEY**

Three or more of these symptoms within about a week of stopping heavy, prolonged use meets cannabis withdrawal criteria.

C. As rebound mood symptoms, which only appear in people with depression

Withdrawal symptoms occur in people without depression; they reflect physiological adaptation to heavy use.

D. As symptoms that cannot be judged until she has two weeks of abstinence

Withdrawal is identified by symptoms that start within about one week; there is no two-week waiting rule.

Rationale: DSM-5-TR includes cannabis withdrawal, which follows cessation of heavy and prolonged use (daily or almost daily over at least a few months). Three or more symptoms develop within about one week, such as irritability or anger, anxiety, sleep difficulty or disturbing dreams, decreased appetite, restlessness, depressed mood or physical discomfort. Recognizing withdrawal helps counselors normalize symptoms and plan relapse prevention.

References: DSM-5-TR — Cannabis Withdrawal: Criteria A-B

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling intern drafts a report stating that a client who drinks six energy drinks daily has "caffeine use disorder, moderate." Her supervisor asks her to check DSM-5-TR. Which correction is accurate?

A. Caffeine use disorder exists but is only coded as mild in severity

DSM-5-TR has no official caffeine use disorder diagnosis, so no severity levels apply.

B. Caffeine use disorder is coded only if withdrawal has been observed

There is no official caffeine use disorder to code; observed withdrawal is a separate diagnosis.

C. Caffeine problems are coded as an other specified stimulant use disorder

Caffeine is not classed with stimulants such as amphetamine and cocaine for use disorder coding.

D. Caffeine use disorder is listed only as a condition for further study ✓ **KEY**

DSM-5-TR places caffeine use disorder in Section III, though caffeine intoxication and withdrawal are diagnosable.

Rationale: In DSM-5-TR, caffeine intoxication and caffeine withdrawal are official diagnoses in the substance-related chapter, but caffeine use disorder appears only among the Conditions for Further Study in Section III. Counselors should not assign a diagnosis that is proposed for research rather than clinical use. Caffeine withdrawal, for example, involves headache plus symptoms such as fatigue, dysphoric mood, poor concentration or flu-like symptoms after abrupt cessation.

References: DSM-5-TR — Section III Conditions for Further Study: Caffeine Use Disorder; Caffeine-Related Disorders

☐ Approve ☐ Revise ☐ Retire Notes:

For the past 5 months, a 7-year-old has had rapid eye blinking, facial grimacing and occasional grunting sounds that come and go. A medical cause and substance effects have been ruled out. Which diagnosis is MOST appropriate now?

A. Tourette's disorder, as he has both motor tics and a vocal tic

Tourette's disorder requires tics persisting more than 1 year since first onset; his tics began 5 months ago.

B. Persistent motor or vocal tic disorder, since tics recur daily

Persistent tic disorder needs more than 1 year of tics and either motor or vocal tics, not both.

C. Provisional tic disorder, as tics have lasted under one year ✓ KEY

Motor and/or vocal tics present for less than 1 year since first onset, beginning before age 18, fit provisional tic disorder.

D. Stereotypic movement disorder, as the movements are repetitive

Stereotypies are rhythmic, often soothing movements starting earlier in childhood; sudden blinks and grunts are tics.

Rationale: Tic disorders are distinguished by tic type and duration. Provisional tic disorder covers single or multiple motor and/or vocal tics present for less than 1 year since first onset, beginning before age 18. Tourette's disorder requires both multiple motor tics and at least one vocal tic for more than 1 year, and persistent (chronic) tic disorder requires only motor or only vocal tics for more than 1 year.

References: DSM-5-TR — Tic Disorders: Provisional Tic Disorder Criteria A-D; comparison with Tourette's and persistent tic disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A 7-year-old with average intelligence and normal vision is far behind peers in catching a ball, using scissors and writing legibly. His teachers say the clumsiness has been present since he was a toddler, and no neurological condition has been found. Which diagnosis BEST fits?

A. Developmental coordination disorder affecting his motor skills ✓ KEY

Motor skills well below age expectations, starting early and not due to intellectual, visual or neurological causes, fit DCD.

B. Specific learning disorder with impairment in written expression

His handwriting problem is part of a broader motor deficit, not a deficit in spelling, grammar or written organization.

C. Mild intellectual developmental disorder with motor slowness

He has average intelligence, so intellectual developmental disorder does not explain the motor deficits.

D. Attention-deficit/hyperactivity disorder, combined presentation

Clumsiness can co-occur with ADHD, but no inattention or hyperactivity symptoms are described.

Rationale: Developmental coordination disorder involves acquisition and execution of coordinated motor skills substantially below age expectations, interfering with daily life and school, with onset in the early developmental period. The deficits are not better explained by intellectual disability, visual impairment or a neurological condition. Because early motor development varies widely, DSM-5-TR notes that DCD is typically not diagnosed before age 5.

References: DSM-5-TR — Developmental Coordination Disorder: Criteria A-D and development and course

☐ Approve ☐ Revise ☐ Retire Notes:

For 8 months, a 25-year-old has shown marked avolition, flat affect and grossly disorganized behavior. A careful evaluation finds no delusions, hallucinations or disorganized speech at any point. Why does this presentation fail schizophrenia Criterion A?

A. Negative symptoms count toward Criterion A only after the first year

Negative symptoms can count toward Criterion A at any time; there is no one-year waiting rule.

B. One of the two must be delusions, hallucinations or disorganized speech ✓ **KEY**

Criterion A requires two of five symptoms, and at least one must be delusions, hallucinations or disorganized speech.

C. Criterion A requires three or more of the five characteristic symptom groups

Only two of the five symptoms are needed; the problem is which symptoms are present, not how many.

D. Disorganized behavior is counted as a negative symptom, not a positive one

Grossly disorganized or catatonic behavior is its own Criterion A item and is not a negative symptom.

Rationale: Schizophrenia Criterion A requires two or more of five symptoms (delusions, hallucinations, disorganized speech, grossly disorganized or catatonic behavior, negative symptoms), each present for a significant portion of a 1-month period. At least one must be delusions, hallucinations or disorganized speech. A presentation limited to negative symptoms and disorganized behavior calls for further assessment of other causes, such as depression, catatonia or a medical condition.

References: DSM-5-TR — Schizophrenia: Criterion A (at least one of items 1-3 required)

☐ Approve ☐ Revise ☐ Retire Notes:

A community educator preparing a talk for parents asks a counselor when schizophrenia most often first appears. Which statement BEST matches DSM-5-TR?

A. Most cases first begin in middle childhood, before puberty starts

Onset before adolescence is rare; childhood-onset schizophrenia is the exception, not the rule.

B. Onset peaks in the mid-40s, with similar ages for men and for women

Peak onset is in the 20s, and the timing differs by gender, with men tending to be earlier.

C. Onset typically occurs after age 60, mainly in women who live alone

Late-life onset is uncommon; new psychosis in older adults calls for a search for medical causes.

D. It usually starts in late teens to mid-30s, often earlier in men ✓ **KEY**

Psychotic features typically emerge between the late teens and mid-30s, peaking in early-to-mid 20s for men, late 20s for women.

Rationale: DSM-5-TR describes typical onset of schizophrenia between the late teens and the mid-30s, with onset before adolescence rare. Peak age at onset is in the early-to-mid 20s for males and the late 20s for females. Many people show a prodromal phase of social withdrawal and declining functioning before psychotic symptoms appear, which is why early detection in adolescence and young adulthood matters.

References: DSM-5-TR — Schizophrenia: Development and Course (age at onset and gender differences)

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old lab technician has always lived alone and spends free time on solitary hobbies. He says he has no wish for close relationships, gets little pleasure from social activity, and is unbothered by praise or criticism. He came to counseling only at his sister's urging. Which diagnosis is MOST consistent?

A. Avoidant personality disorder, given his long pattern of isolation

People with avoidant PD want relationships but avoid them from fear of rejection; he has no such desire.

B. Schizotypal personality disorder, as he is distant from other people

Schizotypal PD adds odd beliefs, perceptual distortions and eccentric behavior, none of which are described.

C. Social anxiety disorder, since he limits his social contact so much

Social anxiety centers on fear of scrutiny; he reports indifference to others, not fear of judgment.

D. Schizoid personality disorder, given his indifference to closeness ✓ **KEY**

Neither desiring nor enjoying close relationships, choosing solitary activities and indifference to praise fit schizoid PD.

Rationale: Schizoid personality disorder is a pervasive pattern of detachment from social relationships and restricted emotional expression, with little desire for closeness and indifference to praise or criticism. The key differential is motivation: people with avoidant personality disorder long for connection but withdraw out of fear of rejection and inadequacy. Schizotypal personality disorder adds cognitive and perceptual distortions and eccentricity.

References: DSM-5-TR — Schizoid Personality Disorder: Criterion A and differential diagnosis with avoidant and schizotypal PD

☐ Approve ☐ Revise ☐ Retire Notes:

A 16-year-old has shown chronic emptiness, unstable relationships, frantic efforts to avoid abandonment and impulsive self-harm across home, school and peer settings for 18 months. A colleague says personality disorders "cannot be diagnosed before 18." What does DSM-5-TR state?

A. No personality disorder at all may be diagnosed until the person turns 18

This overstates the rule; only antisocial personality disorder carries an age-18 minimum.

B. Most may apply under 18 if traits are pervasive and last at least 1 year ✓ **KEY**

Personality disorders other than antisocial PD may be diagnosed in minors when features have lasted at least 1 year.

C. Borderline personality disorder alone may be diagnosed before age 18

The exception is not limited to borderline PD; other personality disorders may also be diagnosed in minors.

D. Personality disorders may apply under 18 after traits last 6 months

DSM-5-TR requires features to be present for at least 1 year, not 6 months, in individuals under 18.

Rationale: DSM-5-TR allows personality disorder diagnoses in children and adolescents when the maladaptive traits are pervasive, persistent and unlikely to be limited to a developmental stage or another mental disorder. Features must have been present for at least 1 year in someone under 18. Antisocial personality disorder is the exception: it requires age 18 or older, with conduct disorder evident before age 15.

References: DSM-5-TR — Personality Disorders: General Personality Disorder, development and course (diagnosis under age 18)

☐ Approve ☐ Revise ☐ Retire Notes:

An adult client meets criteria for anorexia nervosa, and her physician reports a current body mass index (BMI) of 16.4. Using DSM-5-TR severity ranges for adults, which specifier applies?

A. Moderate, since her BMI falls within 16 to 16.99 ✓ KEY

DSM-5-TR rates a BMI of 16 to 16.99 as moderate severity in adults.

B. Mild, since her BMI is still above the level of 16

Mild severity is a BMI of 17 or higher; 16.4 falls in the moderate range.

C. Severe, since her BMI is under the level of 17.5

Severe severity is a BMI of 15 to 15.99; 17.5 is not a DSM-5-TR cutoff.

D. Extreme, as any BMI under 18.5 is called extreme

Extreme severity is a BMI under 15; 18.5 marks the general underweight threshold.

Rationale: DSM-5-TR rates current anorexia nervosa severity in adults by BMI: mild is 17 or higher, moderate 16 to 16.99, severe 15 to 15.99, and extreme under 15. For children and adolescents, BMI percentiles are used instead. Severity may be raised to reflect clinical symptoms, functional disability and the need for supervision.

References: DSM-5-TR — Anorexia Nervosa: severity specifiers based on BMI

☐ Approve ☐ Revise ☐ Retire Notes:

Parents worry that their 18-month-old puts sand and paper in her mouth at the park. Their neighbor's 6-year-old has eaten paint chips and chalk daily for 2 months, which is not part of any family or cultural practice. Which conclusion about pica is MOST accurate?

A. Both children meet criteria, as each eats things that are not food

Mouthing objects is developmentally normal in toddlers; pica requires eating that is inappropriate for development.

B. Neither child can be diagnosed, as pica is a diagnosis for adults

Pica is diagnosed at any age, and it is often seen in childhood.

C. The 6-year-old may meet criteria; the toddler's mouthing is typical ✓ KEY

Pica requires developmentally inappropriate eating of nonfood items for at least 1 month; a minimum age of 2 is suggested.

D. Only the toddler meets criteria, as such symptoms in older children fade

The toddler's behavior is developmentally expected, while the older child's persistent eating is not.

Rationale: Pica is persistent eating of nonnutritive, nonfood substances for at least 1 month that is inappropriate to the person's developmental level and not part of a culturally supported or socially normative practice. DSM-5-TR suggests a minimum age of 2 years to exclude normal mouthing of objects by infants and toddlers. Paint chips also raise concern for lead exposure, so medical evaluation is warranted.

References: DSM-5-TR — Pica: Criteria A-D and diagnostic features (minimum age of 2 years)

☐ Approve ☐ Revise ☐ Retire Notes:

A 74-year-old was diagnosed with Parkinson's disease 6 years ago after developing a tremor and slowed movement. Over the past year, he has developed gradual problems with attention, planning and memory that now interfere with paying bills. Which diagnosis BEST fits?

A. Neurocognitive disorder with Lewy bodies, since he has parkinsonism

In Lewy body NCD, cognitive decline comes before or within about a year of motor signs, not years later.

B. Major frontotemporal neurocognitive disorder, given his poor planning

Frontotemporal NCD features early behavioral or language change, not decline years after established Parkinson's.

C. Major neurocognitive disorder due to Parkinson's disease ✓ **KEY**

Decline that develops gradually in the setting of long-established Parkinson's disease fits NCD due to Parkinson's.

D. Delirium caused by his Parkinson's medications and motor symptoms

Delirium has an acute onset with fluctuating attention; his decline has been gradual over a year.

Rationale: Neurocognitive disorder due to Parkinson's disease is diagnosed when cognitive decline arises insidiously in the setting of established Parkinson's disease. When cognitive decline begins before, or within about one year of, the onset of parkinsonian motor signs, NCD with Lewy bodies is more likely. The timing of motor versus cognitive symptoms is therefore the key differential clue.

References: DSM-5-TR — Major or Mild Neurocognitive Disorder Due to Parkinson's Disease: criteria and differential diagnosis with Lewy bodies

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old construction worker lost consciousness briefly after a fall from scaffolding 5 months ago. Since regaining consciousness, he has had persistent problems with attention, slowed thinking and memory, and he struggles to follow work instructions. Which diagnosis should the counselor consider FIRST?

A. Neurocognitive disorder due to traumatic brain injury ✓ **KEY**

Deficits beginning right after a head injury with loss of consciousness and persisting past the acute period fit NCD due to TBI.

B. Adjustment disorder with anxiety following his work fall

Adjustment disorder involves emotional or behavioral distress, not objective cognitive deficits after head injury.

C. Major depressive disorder causing his memory complaints

Depression can impair concentration, but no mood symptoms are described and a head injury explains the onset.

D. Mild neurocognitive disorder due to Alzheimer's disease

Alzheimer's disease shows insidious, gradual onset, usually in late life, not sudden deficits after an injury.

Rationale: Neurocognitive disorder due to traumatic brain injury requires evidence of a TBI with loss of consciousness, posttraumatic amnesia, disorientation or confusion, or neurological signs. The cognitive disorder must present immediately after the injury or after recovery of consciousness and persist past the acute post-injury period. Counselors should coordinate with medical providers for neuropsychological evaluation.

References: DSM-5-TR — Major or Mild Neurocognitive Disorder Due to Traumatic Brain Injury: Criteria A-C

☐ Approve ☐ Revise ☐ Retire Notes:

A 21-year-old falls asleep in class and at meals several times a day despite sleeping 8 hours a night, a pattern lasting 5 months. When she laughs hard, her knees suddenly buckle and her jaw goes slack for a few seconds, though she stays fully aware. Which condition do these features MOST suggest?

A. Hypersomnolence disorder with long sleep and daytime naps

Hypersomnolence disorder involves excessive sleepiness but not cataplexy triggered by laughter.

B. Narcolepsy, with episodes of cataplexy set off by laughing ✓ KEY

Irresistible sleep episodes plus brief loss of muscle tone with laughter, consciousness intact, point to narcolepsy.

C. Panic disorder, with sudden weak spells during strong emotion

Panic attacks involve surges of fear with autonomic symptoms, not laughter-triggered loss of muscle tone.

D. A seizure disorder with brief absences when she gets excited

Absence seizures impair awareness; in cataplexy the person remains conscious throughout.

Rationale: Narcolepsy involves recurrent periods of irrepressible need to sleep, lapsing into sleep or napping within the same day, at least three times per week for 3 months. It also requires cataplexy, hypocretin deficiency, or characteristic REM findings on sleep studies. Cataplexy is brief, sudden bilateral loss of muscle tone with preserved consciousness, often set off by laughter or joking, and should prompt referral to a sleep specialist.

References: DSM-5-TR — Narcolepsy: Criteria A-B (cataplexy)

☐ Approve ☐ Revise ☐ Retire Notes:

Parents say their 6-year-old sits up screaming about an hour after falling asleep several nights a month. He is sweaty, wide-eyed and hard to comfort, then settles back to sleep. In the morning he remembers nothing. Which explanation BEST fits?

A. Nightmare disorder, since he wakes up afraid during the night

Nightmares occur later in the night in REM sleep, and the child wakes fully and recalls the dream.

B. Panic attacks at night, since he shows sweating and great fear

Nocturnal panic involves waking fully alert from sleep, not being unresponsive with amnesia.

C. Separation anxiety disorder, as he becomes upset at bedtime

He is not upset at bedtime about separation; the episodes arise from deep sleep.

D. Sleep terrors, a non-REM arousal pattern common in children ✓ KEY

Screaming arousals early in the night with poor response to comfort and no recall fit sleep terrors.

Rationale: Sleep terrors are a type of non-rapid eye movement sleep arousal disorder: abrupt terrified arousals, usually in the first third of the night, often beginning with a scream, with autonomic arousal, poor response to comfort, and little or no recall. They are most common in childhood and usually decrease with age. Nightmares, by contrast, occur in REM sleep later in the night, and the person wakes fully and remembers the dream.

References: DSM-5-TR — Non-Rapid Eye Movement Sleep Arousal Disorders: sleep terror type; differential with nightmare disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A 48-year-old in counseling for insomnia says that most evenings when she sits or lies down she feels a crawling, uncomfortable urge to move her legs. Walking around eases it, and it is far worse at night than in the morning. This has happened 4 nights a week for 6 months. Which condition should the counselor suspect?

A. Restless legs syndrome, given the evening urge eased by moving ✓ KEY

An urge to move the legs that starts with rest, is relieved by movement and is worse at night defines restless legs syndrome.

B. Generalized anxiety disorder with restlessness during the day

GAD restlessness is tied to worry, not leg sensations that follow rest and are worst at night.

C. Akathisia from a medication, since she feels inner restlessness

Akathisia follows a medication start or change and lacks the clear evening pattern; none is described.

D. Insomnia disorder alone, since her leg discomfort is from fatigue

Insomnia is not diagnosed alone when another sleep-wake disorder, here RLS, better explains the sleep problem.

Rationale: Restless legs syndrome is an urge to move the legs, usually with uncomfortable sensations, that begins or worsens during rest, is partly or fully relieved by movement, and is worse in the evening or night. Symptoms occur at least three times per week for at least 3 months and cause distress or impairment. Because RLS often presents as trouble falling asleep, counselors should ask about leg sensations and refer for medical evaluation.

References: DSM-5-TR — Restless Legs Syndrome: Criteria A-C

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old who burns herself when overwhelmed also reports passive thoughts of death. Her previous counselor had her sign a "no-harm contract" promising not to hurt herself. She asks whether they should sign one again. Which response BEST reflects current practice?

A. Sign a new contract, since a written promise reliably lowers her risk

No-harm contracts lack evidence of reducing self-harm or suicide and can discourage honest disclosure.

B. Sign one only if she agrees to call the counselor before each urge

Adding conditions does not fix the contract's weak evidence, and it shifts focus away from building her own coping.

C. Build a written safety plan with her triggers, coping and contacts ✓ KEY

Collaborative safety planning names warning signs, coping steps, supports and crisis resources and has research support.

D. Skip planning for now, as her self-harm is not suicidal in its intent

Self-injury plus thoughts of death calls for active risk management, not delay.

Rationale: No-harm or no-suicide contracts are not supported by evidence and may give a false sense of security. A collaborative safety plan, such as the Stanley-Brown Safety Planning Intervention, lists warning signs, internal coping strategies, social contacts and settings for distraction, people to ask for help, professional and crisis contacts, and steps to make the environment safer. Because NSSI raises later suicide risk, ongoing risk assessment remains part of care.

References: Stanley-Brown SPI — Safety Planning Intervention components; contrast with no-suicide contracts

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor reviewing an intake form sees that a 20-year-old has several piercings, a large tattoo and ceremonial scarring from a coming-of-age ritual in her community. She denies any other injury to her body. How should the counselor classify these behaviors?

A. As nonsuicidal self-injury, since each one caused deliberate tissue harm

The proposed NSSI criteria exclude socially sanctioned behaviors such as piercing, tattooing and ritual scarring.

B. As socially sanctioned practices that the NSSI criteria exclude ✓ **KEY**

Body modification and culturally sanctioned rituals are excluded from nonsuicidal self-injury.

C. As NSSI only if she reports feeling relief after each of the procedures

Relief does not convert a socially sanctioned practice into NSSI; the exclusion applies regardless.

D. As a warning sign that requires a full suicide risk assessment today

Cultural and cosmetic body modification is not itself a suicide warning sign; routine screening suffices.

Rationale: The DSM-5-TR proposed criteria for nonsuicidal self-injury disorder state that the behavior is not socially sanctioned, such as body piercing, tattooing or part of a religious or cultural ritual, and is not restricted to picking a scab or nail biting. Counselors assess the function and context of body changes with cultural humility rather than labeling all tissue damage as self-injury.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal Self-Injury Disorder, Criterion E (socially sanctioned behavior excluded)

☐ Approve ☐ Revise ☐ Retire Notes:

A school district asks its counselors when to begin prevention programming about nonsuicidal self-injury. According to DSM-5-TR, at what point in development does NSSI MOST often begin?

A. In early childhood, before children start attending school

Self-injury in very young children is uncommon and often linked to developmental conditions, not typical NSSI.

B. In middle adulthood, often after job or relationship losses

Onset in midlife is uncommon; NSSI most often starts much earlier.

C. In later adulthood, often alongside chronic medical illness

Older adults who self-harm are a high-risk group, but late life is not the typical onset period.

D. In the early teen years, and it can go on for many years ✓ **KEY**

DSM-5-TR notes NSSI most often begins in the early teen years and may continue for many years.

Rationale: DSM-5-TR text on nonsuicidal self-injury notes that it most often starts in the early teen years and can continue for many years. Hospital admissions for NSSI peak around ages 20 to 29 before declining. School-based prevention and staff training in the middle-school years therefore target the developmental window when NSSI typically begins.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal Self-Injury Disorder, Development and Course

☐ Approve ☐ Revise ☐ Retire Notes:

A 17-year-old began an antidepressant prescribed by his pediatrician 2 weeks ago. His mother asks the counselor what the FDA boxed warning on the medication means for her son. Which response is MOST accurate?

A. Watch for new or worse suicidal thinking, mainly early in treatment ✓ **KEY**

The boxed warning notes increased suicidal thoughts and behaviors in youth, calling for close monitoring early on.

B. It means antidepressants cannot be given to anyone under age eighteen

The warning does not ban use in youth; some antidepressants are approved for adolescents with monitoring.

C. It is about weight gain, so his diet should be checked each week

The antidepressant boxed warning concerns suicidality in young people, not weight change.

D. It is about risk only for adults over 65, so it does not apply to him

The warning applies to children, adolescents and young adults under 25, not older adults.

Rationale: The FDA boxed warning on antidepressants states that, in short-term studies, they increased the risk of suicidal thinking and behavior in children, adolescents and young adults under age 25. It calls for close monitoring for worsening mood, agitation or suicidality, especially in the first months of treatment and after dose changes. Counselors educate families, encourage communication with the prescriber and monitor risk.

References: APA CPG — Depression across the lifespan: antidepressants in adolescents and suicidality monitoring · FDA Antidepressant Boxed Warning — 2007 labeling revision: suicidality warning extended through age 24; close monitoring early in treatment

☐ Approve ☐ Revise ☐ Retire Notes:

A client who started a new antipsychotic last week arrives with a high fever, heavy sweating, very stiff muscles, a racing heart and confusion. Which action should the counselor take FIRST?

A. Help the client cut the dose in half until a prescriber can be reached

Counselors do not adjust medication, and these signs need emergency care, not a dose change.

B. Arrange emergency medical care for possible neuroleptic malignant syndrome ✓ **KEY**

Fever, rigidity, autonomic instability and confusion after antipsychotic start suggest NMS, a medical emergency.

C. Treat the confusion as psychosis and schedule a session for later in the week

Fever, rigidity and confusion point to a medical crisis; delay could be fatal.

D. Suggest rest and fluids for a likely viral illness and check in tomorrow

Muscle rigidity with confusion after a new antipsychotic is not a routine illness and needs urgent evaluation.

Rationale: Neuroleptic malignant syndrome is a rare, potentially fatal reaction to dopamine-blocking agents, most often antipsychotics. Hallmarks are high fever, severe muscle rigidity, altered mental status and autonomic instability (sweating, fast heart rate, unstable blood pressure), usually within days of starting or raising the dose. Counselors recognize it as a medical emergency and arrange immediate care.

References: DSM-5-TR — Medication-Induced Movement Disorders and Other Adverse Effects of Medication: Neuroleptic Malignant Syndrome

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old whose brother died by suicide five months ago says neighbors avoid the subject, she keeps asking "why" and replaying their last call, and she blames herself for missing signs. Using Worden's mediators of mourning, which mediator MOST shapes the features of her grief?

A. Historical antecedents, since her earlier losses predict how she copes with this one

Prior losses matter, but nothing in the case points to earlier losses as the source of her guilt and searching.

B. Personality variables, since her age and coping style decide the course of grief

Personality shapes all grief, but it does not explain the stigma, "why" questions and self-blame tied to this death.

C. How the person died, since suicide loss often brings stigma, guilt and searching ✓ **KEY**

Mode of death is a core mediator; suicide loss is marked by "why" questions, guilt, blame and social stigma.

D. Concurrent stresses, since other demands in her life slow her work on the tasks

No competing stressors are described; her symptoms track the nature of the death itself.

Rationale: Worden lists mediators that explain why grief differs between people, including who the person was, the nature of the attachment, how the person died, historical antecedents, personality, social mediators and concurrent stresses. Deaths by suicide commonly leave survivors with intense searching for reasons, guilt, blame and stigma that limits social support. Counselors address these features directly and often link survivors with suicide-loss support groups.

References: Worden (Grief Counseling) — Mediators of mourning: how the person died; grief after suicide

☐ Approve ☐ Revise ☐ Retire Notes:

A father whose wife died suddenly asks whether his 8-year-old son should attend her funeral. The boy has asked to go, but the father worries it will be "too much" for him. What should the counselor recommend?

A. Prepare the boy for what he will see and hear, then let him choose to attend ✓ **KEY**

Children who are prepared and given a choice usually benefit from taking part in family mourning rituals.

B. Keep him home with a sitter so that his memories of his mother stay happy ones

Excluding a child who wants to attend can leave him feeling shut out of the family's grief.

C. Bring him to the service without discussing it so he does not worry ahead

Attending without preparation can be confusing and frightening; explanation beforehand is key.

D. Wait to decide on funerals until he reaches age ten, when grief is clearer

There is no age cutoff; an 8-year-old can understand death as final and can take part when prepared.

Rationale: Research on bereaved children, including the Harvard Child Bereavement Study that Worden directed, supports including children in funerals and other rituals when they are prepared and allowed to choose. Preparation means explaining in concrete terms what will happen, who will be there and how people may act. Taking part helps children accept the reality of the loss and feel included in the family's mourning.

References: Worden (Grief Counseling) — Grieving children: Harvard Child Bereavement Study; children and funerals

☐ Approve ☐ Revise ☐ Retire Notes:

A 35-year-old comes in the day after being robbed at gunpoint. She cannot sleep and is afraid to leave home. The counselor notices her parents' divorce comes up and is tempted to explore how it shaped her. What is the PRIMARY goal of crisis intervention at this point?

A. Uncover how her childhood losses set the stage for her current reaction

Exploring early history is a longer-term therapy goal and can destabilize a client in acute crisis.

B. Reach a full diagnosis of her symptoms before offering any form of help

Crisis work assesses and helps at once; a full diagnostic workup is not a precondition for support.

C. Change the personality traits that left her vulnerable to the robbery

Personality change is not a crisis goal, and the framing blames the client for being a victim.

D. Restore her to at least her pre-crisis level of functioning and safety ✓ KEY

Crisis intervention is brief and focused on safety, coping and a return to prior functioning.

Rationale: Crisis intervention is short-term, present-focused and directive as needed. Its aim is to ensure safety, reduce acute distress and help the client regain at least the level of functioning she had before the crisis. Deeper exploration of developmental history belongs to later counseling, once the client has stabilized.

References: James & Gilliland (Crisis Intervention Strategies) — Goals of crisis intervention: restoring equilibrium and pre-crisis functioning

☐ Approve ☐ Revise ☐ Retire Notes:

A hurricane floods an entire coastal county. Schools, clinics and businesses close for weeks, and thousands of residents lose homes and jobs at once. In James and Gilliland's classification of crises, which type does this event BEST represent?

A. A developmental crisis, arising from a normal change in the course of life

Developmental crises arise from expected life changes such as a birth or retirement, not natural disasters.

B. An ecosystemic crisis, a disaster that strikes a whole community at once ✓ KEY

Ecosystemic crises are natural or human-caused disasters that overwhelm people across a community or region.

C. An existential crisis, rooted in inner conflict over purpose and meaning

Existential crises center on inner questions of purpose and freedom, not a shared external catastrophe.

D. A situational crisis, marked by a sudden event within one person's life

Situational crises are sudden events for an individual or family; a regional disaster goes beyond that scale.

Rationale: James and Gilliland describe four domains of crisis: developmental, situational, existential and ecosystemic. Ecosystemic crises are natural or human-made disasters, such as hurricanes, pandemics or economic collapse, that strike many people and community systems at once. Response therefore involves community-level coordination, not only individual counseling.

References: James & Gilliland (Crisis Intervention Strategies) — Types of crises: developmental, situational, existential and ecosystemic

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old client whose husband has hit her several times says she will pack the car and leave with her two children tomorrow while he is at work. He does not know about her plan. Which counselor response BEST addresses her safety?

A. Suggest she tell him calmly first so the leaving does not come as a shock

Announcing the plan to an abusive partner can trigger escalation before she is safely away.

B. Build a detailed safety plan, since the time of leaving carries added risk ✓ **KEY**

Separation is a period of heightened danger, so planning routes, documents, shelter and contacts is essential.

C. Urge her to delay until a few couples sessions can try to calm the conflict

Couples work is contraindicated with ongoing battering and could expose her plan to her partner.

D. Affirm her courage and reassure her that once she leaves, the danger is over

Risk often rises during and after separation; false reassurance leaves her unprepared.

Rationale: The period around separation is one of the most dangerous times for a person leaving an abusive partner, because the abuser may escalate to regain control. Safety planning covers timing, a safe destination, key documents, money, children, a code word with trusted people and how to reach help. The counselor supports the client's decision while helping her prepare for the added risk.

References: James & Gilliland (Crisis Intervention Strategies) — Intimate partner violence: danger at separation and safety planning

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old client tells her counselor that her partner pushed her into a wall last week. She has no children or dependents in the home, is not in immediate danger, and asks the counselor not to tell anyone. In most U.S. jurisdictions, how should the counselor proceed?

A. Keep the disclosure private, assess risk, and work with her on safety planning ✓ **KEY**

Abuse of a competent adult is generally not reportable by counselors, so they respect autonomy and plan for safety.

B. Report the assault to the police, since counselors must report all partner abuse

Mandated reporting typically covers children and vulnerable adults, not competent adults abused by a partner.

C. Contact adult protective services, since she is a victim of physical abuse

APS serves elders and dependent or vulnerable adults; nothing suggests she falls in either group.

D. Tell her you can no longer see her unless she agrees to leave the partner

Making care conditional on leaving is coercive, ignores her autonomy and abandons her when risk is high.

Rationale: Mandated reporting laws generally cover child abuse and the abuse of elders or dependent adults; a competent adult's disclosure of partner violence usually is not reportable by a counselor. The counselor keeps the information confidential within the usual limits, assesses danger and lethality, and helps the client build a safety plan. If the client faces serious and foreseeable harm, the counselor reassesses the confidentiality limits.

References: State law — Mandated reporting statutes: child, elder and dependent-adult abuse vs adult partner violence · ACA Code of Ethics — B.1.c Respect for Confidentiality; B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is seeing several 5-year-olds referred by a preschool. Which observation should raise the GREATEST concern about possible sexual abuse?

A. A boy who touches his own genitals at nap time and stops when he is redirected

Self-touching that responds to redirection is common, developmentally expected behavior at this age.

B. A girl who asks where babies come from after her aunt has a new baby at home

Curiosity about birth after a new sibling or cousin is normal for young children.

C. A girl who acts out specific adult sex acts with dolls and describes them ✓ **KEY**

Detailed knowledge of adult sexual acts is beyond what a 5-year-old would normally know and warrants concern.

D. A boy who plays doctor with a same-age friend and looks at body differences

Mutual, playful looking between same-age peers is common sexual curiosity in early childhood.

Rationale: Many sexual behaviors in young children, such as self-touching, questions about babies and playful looking with peers, are normal. Behaviors that show knowledge of adult sexual acts beyond a child's developmental level, or that involve force, coercion or large age gaps, are stronger indicators of possible sexual abuse. Such observations meet the reasonable-suspicion standard and should be reported.

References: Gladding (Counseling: A Comprehensive Profession) — Child abuse: indicators of sexual abuse in young children · State law — Child abuse reporting: reasonable suspicion standard

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor tells the principal that a 12-year-old described being burned with a cigarette by his father. The principal says, "Let me handle it. I'll call the parents in and decide whether CPS needs to know." What should the counselor do?

A. Let the principal decide, since school policy assigns him the reporting role

A school policy cannot shift a mandated reporter's personal legal duty to an administrator.

B. Wait until the principal meets with the parents, then report if he agrees

Contacting the alleged abuser first can endanger the child and delay a required report.

C. Ask the student to write a statement so the principal has proof to act on

Gathering evidence is the role of investigators; the counselor needs only reasonable suspicion to report.

D. Make the report to child protective services herself, as the law requires ✓ **KEY**

Mandated reporters must report personally and promptly; telling a supervisor does not fulfill the duty.

Rationale: In nearly every state, the duty to report suspected child abuse rests with each mandated reporter individually. Informing a principal or supervisor does not discharge it, and supervisors may not block a report. Counselors also avoid alerting a suspected abuser before child protective services can assess the child's safety.

References: State law — Child abuse reporting statutes: individual duty of mandated reporters

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old with a clear Artistic-Social Holland code works as a payroll clerk, a highly Conventional job. She does the work well but feels restless and drained and talks about quitting. Which Holland concept BEST explains her dissatisfaction?

A. Low congruence, since her personality type is a poor fit for her work setting ✓ KEY

Congruence is the match between personality and environment; an Artistic type in a Conventional job is a poor fit.

B. Low consistency, since her two code letters sit far apart on the hexagon

Artistic and Social are adjacent on the hexagon, so her code is consistent.

C. Low differentiation, since her interest scores are flat across all of the six types

Her code is described as clear, which signals a differentiated profile, not a flat one.

D. Low identity, since she lacks any stable picture of her goals and talents

Nothing suggests she is unclear about herself; the problem lies in the fit with her job.

Rationale: Holland proposed that people search for environments that let them use their skills and express their values, and that satisfaction and stability depend on congruence between personality type and work environment. Artistic types value self-expression and unstructured work, which sits opposite Conventional on the hexagon. Consistency, differentiation and identity describe the person's profile rather than the person-environment fit.

References: Sharf (Career Development Theory) — Holland's theory: congruence, consistency, differentiation and identity

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants an up-to-date, free U.S. Department of Labor database that describes occupations by tasks, skills, education needed, and wage and outlook data. Which resource should the counselor use?

A. The Dictionary of Occupational Titles

The DOT was the older Labor Department reference; it has been replaced and is no longer updated.

B. The Strong Interest Inventory profile

The Strong is a commercial interest inventory, not an occupational information database.

C. The O*NET occupational database ✓ KEY

*O*NET is the Labor Department's free online database that replaced the DOT.*

D. The Self-Directed Search code booklet

The SDS is a self-scored Holland interest measure, not a federal occupational database.

Rationale: O*NET, the Occupational Information Network, is sponsored by the U.S. Department of Labor and replaced the Dictionary of Occupational Titles. It describes hundreds of occupations by tasks, knowledge, skills, abilities, work context and education, and links to wage and outlook data. Interest inventories such as the Strong and SDS can be paired with O*NET to explore matching occupations.

References: Sharf (Career Development Theory) — Occupational information: O*NET and the Dictionary of Occupational Titles

☐ Approve ☐ Revise ☐ Retire Notes:

An elementary school counselor plans an eight-week small group for students whose parents are divorcing. Several teachers have referred students. What should the counselor do BEFORE the group begins?

A. Place every referred student in the group so that no child is left out

Accepting all referrals skips screening, which may place children whose needs the group cannot meet.

B. Screen each student to judge fit, and get parent consent as policy requires ✓ **KEY**

ASGW guidelines call for screening members for fit, and schools usually require parent consent for groups.

C. Ask each student to share their family story in front of the whole group first

Pressing children to disclose family details at the outset is premature and can feel unsafe.

D. Mix in children from intact homes so members hear about stable families

A topic-focused group is built around shared experience; adding unaffected peers weakens that purpose.

Rationale: The ASGW Best Practice Guidelines call for group workers to screen prospective members so that each person's needs and goals fit the group's purpose. In schools, a brief individual meeting also lets the counselor explain the group and spot children who need individual help instead. Parent or guardian consent is commonly required for small-group counseling under district policy.

References: ASGW Best Practice Guidelines — A.7 Screening and assessment of prospective members · ASCA National Model — Direct student services: small-group counseling

☐ Approve ☐ Revise ☐ Retire Notes:

A high school principal asks the counselor to focus college and career planning on the top 20 percent of the class, since "the others will figure it out." Which response BEST fits the ASCA National Model?

A. Agree, since top students gain the most from the limited counseling time

Rationing career services by achievement conflicts with the model's commitment to every student.

B. Offer career planning only to students who ask for an appointment first

Waiting for requests favors students with more resources and leaves many unserved.

C. Refer the lower-achieving students to outside agencies for their career needs

Career development is a core part of the school counseling program, not something to send elsewhere.

D. Explain that college and career readiness is a program goal for all students ✓ **KEY**

The ASCA National Model frames school counseling, including career development, as serving every student.

Rationale: The ASCA National Model describes a comprehensive school counseling program that promotes academic, career and social/emotional development for all students. Limiting career planning to high achievers would widen equity gaps, especially for first-generation and low-income students. Counselors can use data to advocate for program delivery that reaches every student.

References: ASCA National Model — Comprehensive program for all students: academic, career and social/emotional domains

☐ Approve ☐ Revise ☐ Retire Notes:

In couples counseling, one partner presses for talk and closeness and grows critical when ignored, while the other goes quiet and leaves the room, which makes the first press harder. A counselor using emotionally focused therapy (EFT) would MOST likely do which of the following?

A. Frame the pursue-withdraw cycle as the enemy and reach for the fears beneath it ✓ KEY

EFT de-escalates by naming the negative cycle and accessing underlying attachment emotions.

B. Give the quieter partner a paradoxical task to withdraw on purpose at set times each day

Paradoxical directives belong to strategic therapy, not to EFT's attachment-focused approach.

C. Teach both partners a structured speaker-listener rule to use when they argue

Communication skills drills are part of behavioral couples work, not EFT's emotional focus.

D. Decide which partner is to blame for the conflict and coach that person to change

EFT avoids blame; it treats the cycle, not either partner, as the problem.

Rationale: Emotionally focused therapy, developed by Sue Johnson and Les Greenberg, views couple distress through attachment theory. In early stages the counselor identifies the negative interaction cycle, such as pursue-withdraw, and helps the couple see the cycle as their common enemy. The counselor then accesses the attachment fears and longings beneath each partner's moves and restructures interactions toward safe connection.

References: Goldenberg (Family Therapy) — Emotionally focused couple therapy: negative cycles and attachment

☐ Approve ☐ Revise ☐ Retire Notes:

A 14-year-old who was adopted as an infant from Guatemala by White parents has started asking about her birth mother and her heritage. Her parents feel hurt and worry they have failed her. What should the counselor convey to the family?

A. Her questions suggest a weak attachment that should be treated before more talk

Curiosity about origins does not signal attachment problems; it is common in securely attached adoptees.

B. Parents should steer her away from these topics until she turns eighteen

Shutting down the topic can leave her feeling her questions are shameful or disloyal.

C. Such questions are a normal part of identity work and open talk will help ✓ KEY

Adolescent adoptees often explore origins as part of identity, and open family communication supports them.

D. Her interest shows she wishes to leave the family and live with her birth kin

Interest in birth family and heritage is not a rejection of the adoptive family.

Rationale: Adolescence brings intensified identity exploration, and adopted teens often add questions about birth parents and, in transracial or international adoption, about ethnic and cultural heritage. Adoptees fare best when parents are open, accept these questions and support connection to the child's heritage. The counselor normalizes the questions and helps parents respond without taking them as rejection.

References: Berk (Development Through the Lifespan) — Adoptive families and adolescent identity development; transracial adoption

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old says that nothing bad has happened, yet she feels a deep loss. She had always expected to marry and have children, and now believes that will not happen. In Schlossberg's transition model, what kind of transition is she facing?

A. An anticipated transition, since she had long planned for these milestones

Anticipated transitions are expected events that do occur, such as a planned move or graduation.

B. An unanticipated transition, since a sudden event changed her life course

Unanticipated transitions are unexpected events that happen, like a layoff; nothing happened here.

C. A developmental crisis, since her distress reflects a stage she has failed

Schlossberg does not frame transitions as failed stages; her model is based on the event or non-event.

D. A non-event transition, since an expected life event did not take place ✓ KEY

Non-events are expected events that fail to occur, which can change roles, routines and assumptions.

Rationale: Schlossberg defines a transition as any event or non-event that changes relationships, routines, assumptions or roles. She names three types: anticipated transitions, unanticipated transitions and non-events, which are expected events that do not occur, such as a promotion or child that never comes. Non-events are often unrecognized by others, so counselors help clients name and grieve them.

References: Sharf (Career Development Theory) — Schlossberg's transition model: anticipated, unanticipated and non-event transitions

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor is planning support around puberty for middle school students. Based on research on pubertal timing, which group of students is at GREATEST risk for depression, early sexual activity and substance use?

A. Girls who reach puberty well before most of their same-age classmates ✓ KEY

Early-maturing girls face mismatched social pressure and are at raised risk for these outcomes.

B. Boys who reach puberty well before most of their same-age school classmates

Early-maturing boys are often viewed positively, though some show added risk-taking.

C. Girls who reach puberty at about the same time as their classmates do

On-time maturers generally show the fewest adjustment problems.

D. Boys who reach puberty at about the same time as their classmates do

On-time maturation is not linked to elevated risk in either sex.

Rationale: Pubertal timing relative to peers shapes adjustment. Early-maturing girls tend to have more depression, poorer body image, earlier dating and sexual activity, and more substance use, partly because they attract older peers before they are emotionally ready. Early maturation in boys is more often linked with popularity, though both early-maturing boys and late-maturing boys can face difficulties.

References: Berk (Development Through the Lifespan) — Adolescence: pubertal timing and adjustment

☐ Approve ☐ Revise ☐ Retire Notes:

A 46-year-old client says, "Everyone says I'm due for a midlife crisis. Should I expect to blow up my life soon?" Which statement BEST reflects life-span research on midlife?

A. Most adults go through a crisis between ages forty and forty-five

Large studies find dramatic midlife crises affect only a minority of adults.

B. Few people have a true crisis, though many reflect on goals and time ✓ **KEY**

Midlife reappraisal is common, but sudden turmoil and upheaval are not the typical course.

C. A midlife crisis is needed to move on to a healthy later adulthood

No theory or research shows a crisis is required for healthy development.

D. Midlife crises affect men almost only, as their careers start to peak

Both men and women reassess at midlife, and crises are uncommon in both.

Rationale: Popular culture portrays midlife as a time of crisis, but research finds that only a minority of adults experience intense turmoil. Many do engage in midlife reappraisal, weighing time left, goals and relationships, and most report turning points rather than upheaval. Counselors can normalize reflection without predicting crisis.

References: Berk (Development Through the Lifespan) — Middle adulthood: is there a midlife crisis?

☐ Approve ☐ Revise ☐ Retire Notes:

A retired 74-year-old librarian still reads daily, leads a weekly book club and keeps in touch with former coworkers. She says, "I'm still the same person, just with more time." Which theory of aging BEST describes her adjustment?

A. Disengagement theory, as she pulls back from her social roles

Disengagement theory predicts mutual withdrawal from society, which she is not doing.

B. Activity theory, as she replaces each role she has lost

Activity theory stresses replacing lost roles with new ones; she is extending her existing patterns.

C. Continuity theory, as she keeps long-held interests going ✓ **KEY**

Continuity theory holds that older adults adapt by preserving familiar activities, ties and identity.

D. Socioemotional selectivity, as she sheds most social ties

This theory involves narrowing ties to close ones; she has kept her wider network.

Rationale: Continuity theory proposes that older adults strive to maintain consistency between past and present, using familiar skills, activities and relationships to adapt to aging. Disengagement theory, now largely discredited, described mutual withdrawal, while activity theory emphasized replacing lost roles. Socioemotional selectivity theory describes a focus on emotionally close relationships.

References: Berk (Development Through the Lifespan) — Late adulthood: disengagement, activity, continuity and socioemotional selectivity theories

☐ Approve ☐ Revise ☐ Retire Notes:

An 81-year-old man whose wife died four months ago has given away his tools and fishing gear, says he is "just tired of it all," and lives alone with his hunting rifles. He denies being depressed. What should the counselor do FIRST?

A. Accept his denial, since older men often show grief as plain fatigue

Denial does not rule out risk; his warning signs call for direct assessment.

B. Plan grief counseling to help him work through the recent loss of his wife

Grief work may help later, but warning signs of suicide must be assessed first.

C. Refer him to his doctor to check for a physical cause of his tiredness

A medical check may be useful, but it does not address the immediate suicide risk.

D. Ask him directly about suicidal thoughts, plans, intent and gun access ✓ KEY

Recent widowhood, giving away belongings, hopelessness and firearms call for a direct risk assessment.

Rationale: Older men, especially those who are widowed and live alone, have among the highest suicide rates, and they often deny depression or present with fatigue and withdrawal. Warning signs here include giving away possessions, hopeless statements and access to firearms. The counselor asks directly about suicidal ideation, plans, intent and means, using a structured tool such as the C-SSRS, and then addresses lethal means and safety.

References: C-SSRS — Screening for suicidal ideation, intent, plan and behavior · Berk (Development Through the Lifespan) — Late adulthood: suicide risk among older adults

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old accountant lost most of her vision to diabetic retinopathy over the past year. Her mood has improved in counseling, and she now wants to return to work but is unsure how she would manage the tasks. Which next step BEST supports her goal?

A. Advise her to apply for long-term disability, since her field needs sight

Assuming she cannot work in her field limits her options and ignores available adaptations.

B. Refer her to the state vocational rehabilitation agency for job services ✓ KEY

State VR agencies provide assessment, assistive technology, training and job placement for people with disabilities.

C. Suggest she pick a new career with no reading so her vision is not a factor

Steering her away from her career ignores screen readers and other tools that make the work possible.

D. Wait to talk about work until she has fully accepted her loss of eyesight

Adjustment is ongoing; delaying her stated goal is not needed and may slow recovery.

Rationale: Public vocational rehabilitation programs, funded under the Rehabilitation Act, help eligible people with disabilities prepare for, find and keep jobs. Services include vocational assessment, assistive technology such as screen readers, training and placement, often delivered by rehabilitation counselors. Supporting a return to valued work also aids psychological adjustment to an acquired disability.

References: Gladding (Counseling: A Comprehensive Profession) — Rehabilitation counseling: vocational services for people with disabilities

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor writes intake notes on a new adult client as "a person with autism." In session, the client says, "I'm autistic. It is part of who I am, not something I carry around." What should the counselor do?

A. Use the identity-first language the client prefers in talk and notes ✓ KEY

Following the client's own language for their identity shows respect and cultural humility.

B. Keep using person-first language, since it is the professional standard

Person-first language is not required when a client prefers identity-first terms.

C. Avoid naming autism at all so the client does not feel defined by it

Avoiding the term ignores the client's stated view that autism is part of their identity.

D. Explain why person-first terms are more respectful and kinder to use

Correcting the client imposes the counselor's values over the client's self-definition.

Rationale: Person-first language ("person with a disability") was promoted to avoid defining people by a condition, but many autistic and Deaf people prefer identity-first language that treats disability as part of identity and culture. Culturally responsive counselors ask about and use the client's preferred terms. Disability is a dimension of diversity, and language choices convey respect.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling persons with disabilities: language, identity and disability culture · MSJCC (Ratts et al., 2016) — Client worldview: understanding clients' identities as they define them

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old gay man works at a small firm where coworkers often tell anti-gay jokes. He is out to family and friends but not at work, and asks whether he "should" come out there. Which counselor response is MOST appropriate?

A. Encourage him to come out soon, since hiding will harm his mental health

Pushing disclosure ignores the real risks in his workplace and his right to decide.

B. Advise him to keep it private, since disclosure at work is seldom worth the risk

Steering him toward concealment is also directive and overlooks possible benefits of disclosure.

C. Tell him the jokes are likely harmless and not worth worrying much about

Dismissing the jokes invalidates a real minority stressor and his concern for safety.

D. Help him weigh safety, supports, rights and values to reach his own choice ✓ KEY

Disclosure is the client's decision; the counselor helps him weigh risks, resources and goals.

Rationale: Coming out is a series of decisions made in each setting, and the risks and benefits differ by context. Concealment can add stress, but disclosure in a hostile workplace can bring harassment or job loss. An affirming counselor helps the client weigh safety, legal protections, support and personal values, and respects his choice.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ individuals: disclosure decisions and workplace climate · MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions: client-led decisions in oppressive settings

☐ Approve ☐ Revise ☐ Retire Notes:

A transgender man, assigned female at birth, tells his counselor that he is attracted only to men. A student in practicum assumes this means the client is "really straight." Which statement corrects the student's error?

A. Sexual orientation follows gender assigned at birth, so he is straight

This repeats the error; orientation is described relative to a person's gender identity.

B. Transgender clients cannot be described by any sexual orientation term

Transgender people hold the full range of sexual orientations, as cisgender people do.

C. Orientation and gender identity differ; a man drawn to men may be gay ✓ **KEY**

As a man attracted to men, he may identify as gay; identity and attraction are separate dimensions.

D. His orientation can be known only after any medical transition is done

Orientation does not depend on medical transition and is defined by the client.

Rationale: Gender identity is a person's internal sense of their gender, while sexual orientation describes whom a person is attracted to. The two are independent, and orientation labels are understood in relation to the person's gender identity, not the sex assigned at birth. Counselors use the terms clients choose for themselves.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ individuals: sexual orientation vs gender identity

☐ Approve ☐ Revise ☐ Retire Notes:

A bilingual 29-year-old who immigrated from Brazil at 15 speaks fluent English in sessions. When describing her grandmother's death, she pauses, says a phrase in Portuguese, then apologizes. What should the English-speaking counselor do?

A. Ask her to keep to English so the counselor does not miss anything said

Limiting her to English can block access to emotional material stored in her first language.

B. Invite her to use Portuguese when it fits, then explore what it meant ✓ **KEY**

Feelings tied to early memories are often more available in the first language; inviting it deepens the work.

C. Refer her to a Portuguese-speaking counselor for the rest of treatment

Referral is unnecessary for a fluent client; the counselor can welcome both languages.

D. Treat the language switch as a sign of avoidance to gently confront

Code-switching here reflects emotional access, not resistance, and confronting it would shame her.

Rationale: For bilingual clients, emotions and memories formed in the first language often remain more vivid in that language. Switching languages can signal emotional depth rather than avoidance. Counselors who welcome the first language and then explore its meaning with the client honor culture and can reach material that a second language keeps at a distance.

References: Sue & Sue (Counseling the Culturally Diverse) — Language barriers and bilingual clients in counseling

☐ Approve ☐ Revise ☐ Retire Notes:

A 50-year-old who was a surgeon in Syria now works nights stocking shelves while his license is reviewed. He is irritable, sleeps poorly, and says, "Here I am nobody." Which factor MOST likely drives his distress?

A. Loss of status and role, a common source of stress after immigration ✓ KEY

Downward occupational mobility strips identity and respect and is a well-known acculturative stressor.

B. Integration, since he has adopted both his old and new cultures fully

Integration is the strategy linked to the best adjustment and is not shown in his account.

C. A personality disorder, as shown by his long-standing pride and anger

His irritability began after migration; nothing shows a long-standing, pervasive pattern.

D. Culture shock that will fade on its own once his first year has passed

His distress stems from an ongoing loss of role that time alone may not resolve.

Rationale: Many immigrants and refugees face downward occupational mobility because credentials are not recognized, which brings loss of income, status and professional identity. This role loss is a major source of acculturative stress and can show up as irritability, low mood and sleep problems. Counselors validate the loss and may help with credentialing resources and advocacy.

References: Sue & Sue (Counseling the Culturally Diverse) — Immigrants and refugees: loss of status and acculturative stress

☐ Approve ☐ Revise ☐ Retire Notes:

A 57-year-old client says her imam has been a steady source of comfort since her son's arrest, and she wishes the imam and counselor could "work together" to support her. How should the counselor respond?

A. Decline, since religious leaders should stay apart from clinical care

The Code encourages considering religious leaders as supports when the client wants this.

B. Call the imam that day to share an update on her progress in sessions

Contacting the imam without the client's consent would breach confidentiality.

C. Advise her to rely on the counselor alone so her care stays consistent

Discouraging a valued support ignores her worldview and narrows her resources.

D. Discuss what she wants and, with written consent, coordinate with him ✓ KEY

With consent, counselors may enlist religious and community leaders as positive supports.

Rationale: ACA Code standard A.1.d directs counselors to recognize that support networks hold various meanings in clients' lives and to consider enlisting religious or spiritual leaders, family and friends as positive resources, with client consent. The counselor clarifies what the client hopes for and what information may be shared before any contact. This respects both her faith and her confidentiality.

References: ACA Code of Ethics — A.1.d Support Network Involvement

☐ Approve ☐ Revise ☐ Retire Notes:

A Muslim client who usually meets at 5 p.m. mentions that Ramadan begins next week and that she fasts from dawn until sunset. What is the BEST response?

A. Suggest she eat a light snack before sessions so that she can stay focused

Eating would break her fast; the suggestion puts the counselor's convenience over her religious practice.

B. Ask how fasting affects her, and plan times together that suit her needs ✓ **KEY**

Asking about her experience and planning together respects her practice and her role in decisions.

C. Suggest pausing counseling for the month so her fasting is not disrupted

Suspending care assumes her needs and may interrupt progress without her input.

D. Keep the usual time, since changing it might imply that she is different

Ignoring a stated practice is not neutral; it overlooks a real effect on her day.

Rationale: Religious practices such as fasting during Ramadan affect energy, schedules and evening routines, since many people break the fast at sunset. Culturally responsive counselors ask how a practice affects the client and adapt logistics collaboratively. This conveys respect for the client's faith without assuming what she needs.

References: Sue & Sue (Counseling the Culturally Diverse) — Religious diversity: counseling Muslim clients

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old Army veteran with PTSD also has a history of mild traumatic brain injury from a blast eight years ago. He has some trouble concentrating. His previous counselor said trauma-focused therapy was "off the table" because of the brain injury. Which statement BEST reflects VA/DoD guidance?

A. Trauma therapy should wait until neuropsychological testing rules out deficits

Testing may guide adaptations, but it is not a required step before trauma-focused care.

B. Medication should be the main treatment, as therapy is too hard after a TBI

Guidance favors trauma-focused psychotherapy, and mild TBI does not make it too demanding.

C. A history of mild TBI does not rule out evidence-based trauma-focused therapy ✓ **KEY**

VA/DoD guidance treats PTSD with the same first-line therapies when mild TBI is present.

D. PTSD symptoms should be treated as brain injury effects with cognitive rehab

Assigning all symptoms to TBI overlooks treatable PTSD and the first-line therapies for it.

Rationale: Mild TBI and PTSD often co-occur in veterans and share symptoms such as poor concentration, irritability and sleep problems. VA/DoD guidance recommends treating PTSD with the same evidence-based trauma-focused psychotherapies, such as prolonged exposure or cognitive processing therapy, whether or not a history of mild TBI is present. Counselors may adapt delivery, for example with written aids, but should not withhold first-line care.

References: VA/DoD CPG — Management of PTSD and Acute Stress Disorder (2023): co-occurring mild TBI

☐ Approve ☐ Revise ☐ Retire Notes:

A Marine veteran back from two deployments drives down the center of the road, swerves around trash bags, and sits with his back to the wall in restaurants. His wife calls him "paranoid." Which counselor framing is MOST helpful?

A. These actions were trained survival skills in combat that now need to be reset ✓ **KEY**

Framing them as once-adaptive responses reduces shame and sets up recalibration for civilian life.

B. These actions are early signs of a psychotic disorder needing urgent referral

Learned, situation-linked vigilance with intact reality testing is not psychosis.

C. These actions show a personality flaw that he must take responsibility to correct

Blaming his character ignores the trauma and training behind the behavior and adds shame.

D. These actions are minor quirks that will fade without help as time goes by

Hypervigilance can persist and strain family life; dismissing it delays useful help.

Rationale: Trauma-informed care views many post-trauma reactions as adaptations that once protected the person. For combat veterans, scanning, avoiding roadside objects and guarding exits were trained, life-saving skills. Naming them as adaptive in theater but costly at home reduces shame and opens the door to recalibrating them and to assessing for PTSD.

References: SAMHSA TIP 57 — Trauma-informed care: trauma reactions as adaptive responses; veterans and service members

☐ Approve ☐ Revise ☐ Retire Notes:

Treatment Planning

17 items · NCE weight 9% (14 of 160 scored items)

Q108 of 200 · nce-s-trt-069 · Assessment and Testing · matching level of care to need · medium · In review

A 52-year-old client drinks about a fifth of vodka daily and wants to stop. The last time he quit on his own, he had a withdrawal seizure and was taken to an emergency department. He has stable housing and a supportive sister. Which recommendation is MOST appropriate as the first step?

A. Begin weekly outpatient counseling with a goal of stopping alcohol use within two weeks

Weekly outpatient care offers no medical monitoring; stopping abruptly with his seizure history puts him at serious risk.

B. Enroll him in an intensive outpatient program and have him taper his drinking on his own

A self-directed taper is unreliable and unsupervised; IOP does not provide the medical management his withdrawal risk requires.

C. Refer him for medically monitored withdrawal management before planning ongoing treatment ✓ **KEY**

A history of withdrawal seizures signals high withdrawal risk, so medically supervised withdrawal management is the safe first step.

D. Arrange a stay in a sober living home so he can stop drinking in an alcohol-free setting

Sober living provides a substance-free environment but no medical care; it does not address dangerous withdrawal.

Rationale: Level-of-care decisions in addiction treatment consider several dimensions, and acute intoxication and withdrawal potential is assessed first because it can be life-threatening. A prior withdrawal seizure with heavy daily drinking signals high risk for complicated alcohol withdrawal. He should first receive medically monitored withdrawal management; his supportive environment can then help him step into an appropriate ongoing level of care.

References: ASAM Criteria — Withdrawal potential dimension: history of withdrawal seizures and medically monitored withdrawal management

☐ Approve ☐ Revise ☐ Retire Notes:

Q109 of 200 · nce-s-trt-070 · Assessment and Testing · matching level of care to need · medium · In review

A 36-year-old teacher meets criteria for mild alcohol use disorder. She has no withdrawal symptoms or medical problems, is motivated to cut back, and lives with a supportive partner. Her counselor recommends a 28-day residential program "just to be safe." What is the MAIN concern with this recommendation?

A. It places her at a more intensive level than needed and may disrupt her work and supports ✓ **KEY**

Care should be the least intensive level that safely meets her needs; residential care would pull her from her job and partner without clinical need.

B. Residential programs are designed only for clients who have already failed outpatient care

Placement is based on current multidimensional need, not on a required sequence of failed lower levels.

C. A 28-day stay is too short to change a drinking pattern that has lasted for several years

The problem is not the length of the stay but that residential care is not indicated for her at all.

D. Her motivation shows she does not need any formal treatment for her alcohol use at this time

Motivation is a strength, but she still meets criteria for a disorder and can benefit from outpatient treatment.

Rationale: Placement criteria aim to match clients to the least intensive level of care that is safe and effective, based on a multidimensional assessment. Over-placement wastes resources and can harm clients by disrupting protective factors such as employment, family and community ties. With no withdrawal risk, no medical or psychiatric complications and a supportive home, outpatient treatment fits this client.

References: ASAM Criteria — Multidimensional assessment and placement in the least intensive level of care that is safe and effective

☐ Approve ☐ Revise ☐ Retire Notes:

A 58-year-old retired nurse has had trouble falling and staying asleep at least four nights a week for over a year. Medical and substance-related causes have been ruled out, and she would prefer not to take sleep medication. Which psychological treatment has the strongest evidence as a first-line approach?

A. Sleep hygiene education given on its own, such as limiting caffeine and keeping a dark room

Sleep hygiene is a useful component, but guidelines advise against using it alone because it is not effective as a single therapy.

B. Psychodynamic therapy aimed at uncovering the early conflicts that are keeping her awake

Psychodynamic therapy is not an established first-line treatment for chronic insomnia.

C. Progressive muscle relaxation practiced in session, with no change to her sleep schedule

Relaxation can help, but alone it lacks the support of the multicomponent package that restructures sleep behavior.

D. Cognitive behavioral therapy for insomnia, including stimulus control and sleep restriction ✓ **KEY**

CBT-I is the recommended first-line treatment for chronic insomnia in adults, with strong research support.

Rationale: Cognitive behavioral therapy for insomnia (CBT-I) is a multicomponent treatment that combines stimulus control, sleep restriction, cognitive work on sleep-related beliefs and sleep hygiene education. Clinical practice guidelines strongly recommend it as the first-line treatment for chronic insomnia in adults. Sleep hygiene education is helpful as part of the package but is not recommended as a stand-alone therapy.

References: AASM Clinical Practice Guideline (Insomnia) — Behavioral and psychological treatments for chronic insomnia: strong recommendation for multicomponent CBT-I; sleep hygiene not recommended as a single therapy

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old client with bipolar I disorder takes lithium as prescribed. Her mood episodes tend to follow disruptions such as overnight shifts, travel across time zones and conflicts with her partner. Her psychiatrist asks the counselor to provide an adjunctive psychotherapy suited to this pattern. Which approach BEST fits?

A. Exposure and response prevention focused on reducing her worry about future mood swings

ERP is a treatment for obsessive-compulsive disorder and does not target the rhythm disruptions linked to her episodes.

B. Interpersonal and social rhythm therapy to stabilize daily routines and address conflicts ✓ **KEY**

IPSRT was developed for bipolar disorder; it regularizes sleep and daily routines and addresses interpersonal problems that trigger episodes.

C. Person-centered therapy alone, letting her choose the focus of each session as it unfolds

A supportive nondirective approach does not provide the structured rhythm and interpersonal work her pattern calls for.

D. Psychoanalytic therapy several times weekly to explore unconscious roots of her moods

Intensive psychoanalytic therapy is not an evidence-based adjunct for bipolar disorder and does not target rhythm stability.

Rationale: Interpersonal and social rhythm therapy (IPSRT), developed by Ellen Frank, combines interpersonal psychotherapy with behavioral work on stabilizing daily routines and sleep-wake cycles. It rests on the idea that disrupted social rhythms destabilize circadian functioning and can trigger mood episodes in bipolar disorder. It is used as an adjunct to medication, which fits a client whose episodes follow schedule disruptions and relationship conflicts.

References: IPSRT — Social rhythm stabilization and interpersonal problem areas as an adjunct to medication in bipolar disorder

☐ Approve ☐ Revise ☐ Retire Notes:

Q112 of 200 · nce-s-trt-073 · Research and Program Evaluation · progress monitoring and outcome measures · hard · In review

An experienced counselor declines to use session-by-session outcome measures, saying, "After 20 years, I can tell when a client is getting worse." Which research finding MOST directly challenges this view?

- A. Clinicians rarely predict which clients will deteriorate; measure-based alerts flag most ✓ KEY
Studies comparing clinician predictions with actuarial alerts found that therapists identified very few deteriorating clients, while measures flagged most of them.
- B. Experienced clinicians tend to underestimate how much progress their clients are making
Research suggests the opposite bias: clinicians tend to overestimate client improvement.
- C. Most clients who get worse in counseling say so openly to their counselor within a session
Clients often do not voice deterioration directly, which is one reason structured measures are useful.
- D. Standardized measures are accurate only when they are given at intake and at the final session
Pre-post measurement cannot detect deterioration while there is still time to respond; frequent monitoring is the point.

Rationale: Research on routine outcome monitoring found that clinicians, including experienced ones, are poor at predicting which clients will deteriorate, while statistical early-warning systems based on expected recovery curves identify most of them. Clinicians also tend to overrate their own effectiveness. Feedback from frequent standardized measures helps counselors catch clients who are not on track and change course before dropout or treatment failure.

References: Lambert (Outcome Monitoring) — Clinician judgment versus actuarial prediction in detecting client deterioration

☐ Approve ☐ Revise ☐ Retire Notes:

Q113 of 200 · nce-s-trt-074 · Research and Program Evaluation · progress monitoring and outcome measures · easy · In review

A counselor plans to track a client's anxiety with a brief standardized self-report measure over the course of treatment. When should the counselor give the measure for the FIRST time?

- A. Once the client has learned the coping skills, so the scores show what they have gained
Waiting until skills are learned removes the starting point needed to show how much the client has changed.
- B. At the midpoint of treatment, when the counselor is deciding whether to change approaches
A midpoint score has nothing to be compared against unless an earlier baseline was taken.
- C. At intake or before treatment starts, to set a baseline for comparing later scores ✓ KEY
A pretreatment score is the baseline against which every later score is compared.
- D. At the final session, so the counselor can document the client's status at discharge
A discharge score alone cannot show change; it needs a baseline taken before treatment.

Rationale: Progress monitoring depends on a baseline: a measurement taken at intake or before treatment begins. Later scores are interpreted relative to that starting point, which allows the counselor to judge whether change is meaningful. Without a baseline, improvement or deterioration cannot be shown.

References: Hays (Assessment) — Outcome assessment: establishing a baseline before treatment for progress monitoring

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old client has met all the goals in her treatment plan for depression, and her scores have stayed in the nonclinical range for two months. She says she feels ready to stop. Her counselor would like to keep meeting weekly to "work on deeper issues." What should the counselor do?

A. Continue weekly sessions, since further personal growth is still possible for the client

Continuing when goals are met and the client wants to stop serves the counselor's agenda, not a clinical need.

B. Review her gains, plan for maintaining them, and discharge her with an option to return ✓ **KEY**

When the client no longer needs counseling, the counselor ends it with a planned discharge and aftercare guidance.

C. End services at this session without further discussion, since the treatment goals are met

Ending abruptly skips the review, relapse-prevention planning and aftercare that a planned discharge includes.

D. Write new long-term goals in the plan so that continued weekly sessions are justified

Adding goals the client did not ask for to justify services is not collaborative and is ethically questionable.

Rationale: Counselors end counseling when it is reasonably apparent that the client no longer needs assistance, is unlikely to benefit or is being harmed. A planned discharge reviews progress toward goals, consolidates gains, identifies early warning signs and coping strategies, and tells the client how to return if needed. The client's readiness and goal attainment, not the counselor's wish to continue, guide the decision.

References: ACA Code of Ethics — A.11.c Appropriate Termination · Wiger (Documentation) — Discharge summary: progress toward goals and aftercare recommendations

☐ Approve ☐ Revise ☐ Retire Notes: _____

A new counselor asks why the agency uses a written safety plan with clients at risk of suicide instead of asking them to sign a "no-suicide contract." Which explanation is MOST accurate?

A. A signed contract legally shields the counselor, but it is too formal for most clients

No-suicide contracts offer no reliable legal protection, and formality is not the main problem with them.

B. A contract is the better tool, but a safety plan takes less session time to complete

Contracts are not the better tool, and a good safety plan takes time to build collaboratively.

C. A safety plan replaces the need for any further suicide risk assessment of the client

A safety plan supplements ongoing risk assessment; it does not replace it.

D. A safety plan builds specific coping steps, while a contract lacks evidence of benefit ✓ **KEY**

Safety plans give clients concrete steps and resources for a crisis; no-suicide contracts lack research support and can give false reassurance.

Rationale: No-suicide contracts ask a client to promise not to harm themselves but give them nothing to do in a crisis, lack evidence of effectiveness and may give counselors false reassurance. A safety plan, such as the Stanley-Brown Safety Planning Intervention, is developed collaboratively and lists warning signs, internal coping strategies, social contacts, people to ask for help, professional and crisis resources, and steps to make the environment safer.

References: Stanley-Brown SPI — Safety planning as an alternative to no-suicide contracts

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 47-year-old client who has smoked for 30 years tells his counselor that he has picked a quit date two weeks from now, bought nicotine patches and told his family. Which counselor response BEST matches his current stage of change?

A. Help him build a specific quit plan that anticipates triggers and names his coping steps ✓ KEY

He is in preparation, so the task is to firm up a concrete, workable action plan.

B. Give him information about the health risks of smoking to raise his awareness of them

Consciousness raising fits precontemplation; he is already convinced and taking steps.

C. Explore his mixed feelings by listing the pros and cons of smoking and of quitting

A decisional balance exercise suits contemplation, when the client is still ambivalent about changing.

D. Focus on preventing relapse in the months after he has stayed smoke-free for a while

Relapse prevention for sustained change belongs to the maintenance stage, which he has not reached.

Rationale: In the Transtheoretical Model, the preparation stage describes people who intend to act within about the next month and have already taken small steps, such as setting a date or buying supplies. Stage-matched interventions in preparation help the client turn intention into a concrete plan, anticipate obstacles and strengthen commitment. Interventions suited to earlier stages can feel redundant to a client who has already decided.

References: Transtheoretical Model — Preparation stage: intention to act within 30 days and stage-matched action planning

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old client stopped using methamphetamine eight months ago but used again for three days after a breakup. She tells her counselor, "I'm back at square one." According to Prochaska and DiClemente's model, which statement BEST reflects how relapse is understood?

A. Relapse sends most people back to precontemplation, so change must restart from the start

Only a small share of people who relapse regress to precontemplation; most do not start over.

B. Relapse after six months shows the client had not truly reached the maintenance stage

Relapse can occur from maintenance; it does not mean the earlier progress was not real.

C. Relapse is common, and most people recycle to contemplation or preparation and try again ✓ KEY

The spiral model shows that most people who relapse recycle to contemplation or preparation and draw on what they learned.

D. Relapse shows that the stages of change model does not apply to stimulant use disorders

The model has been applied across many addictive behaviors, including stimulant use, and expects relapse.

Rationale: Prochaska, DiClemente and Norcross described change as a spiral rather than a straight line. Relapse is common, and most people who relapse do not regress all the way to precontemplation; they recycle to contemplation or preparation and try again, often with lessons from earlier attempts. Framing relapse this way helps counselors counter the client's "square one" thinking and reduce shame.

References: Transtheoretical Model — Spiral model of change: relapse and recycling through the stages

☐ Approve ☐ Revise ☐ Retire Notes:

A client's primary care physician calls the counselor to coordinate care and asks for "everything you have," including the private process notes the counselor keeps separate from the client's clinical record. Under HIPAA, which statement is accurate?

A. HIPAA bars the counselor from sharing any client information with a physician without consent

HIPAA allows disclosure of protected health information to another provider for treatment; it does not bar all sharing.

B. The physician may obtain the process notes because they are part of the client's medical record

Psychotherapy notes kept separate from the record are not part of the general medical record and receive extra protection.

C. All records, process notes included, may be sent because disclosure for treatment is permitted

The treatment exception does not cover psychotherapy notes, which need specific authorization.

D. The process notes need the client's specific authorization; record information may be shared ✓ **KEY**

Psychotherapy notes generally need a separate authorization, while information in the clinical record may be shared for treatment.

Rationale: HIPAA gives psychotherapy notes, the counselor's separate notes analyzing session content, special protection: most disclosures, including to other providers, require the client's specific authorization. Information in the clinical record, such as diagnosis, treatment plan, symptoms and progress summaries, may be disclosed to another provider for treatment purposes. Many counselors still obtain a release before coordinating care, and state law or ethics codes may require stricter consent.

References: HIPAA — Privacy Rule 45 CFR 164.508(a)(2): authorization required for psychotherapy notes; 164.506 disclosures for treatment

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old client seeks counseling because panic attacks keep her from driving on the highway to work. Her counselor adds this objective: "Client will walk 30 minutes three times a week for the next eight weeks, as recorded in an exercise log." Which SMART criterion is this objective MOST likely to fail?

A. Measurable, because walking cannot be counted or recorded in any reliable way

Walking time and frequency are easy to record in a log, so the objective is measurable.

B. Relevant, because it is not clearly tied to the problem the client came in for ✓ **KEY**

Nothing links walking to her highway-driving panic, so the objective does not fit her presenting concern.

C. Time-bound, because the objective does not give any time period for the change

The objective states an eight-week period, so it is time-bound.

D. Specific, because the objective does not state how often or how long she will walk

The objective names the activity, duration and frequency, so it is specific.

Rationale: SMART objectives are Specific, Measurable, Achievable, Relevant and Time-bound. This objective is precise, countable and time-limited, but it does not connect to the client's stated problem of panic while driving on highways. Relevant objectives flow from the presenting concern and the goal, such as graded driving exposure tracked in a log.

References: Wiger (Documentation) — Treatment plans: objectives linked to the presenting problem and long-term goals

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old man with a mild intellectual disability lives in a group home and is starting counseling for anger after conflicts with a housemate. The group home manager offers to write his treatment goals for him to save time. What is the BEST approach?

A. Build goals with him in plain words with visual aids, inviting staff input if he agrees ✓ KEY

Adapting the process to his abilities keeps him at the center of planning, with staff involved only with his consent.

B. Accept the manager's goals, since staff observe his daily behavior more than he can report

Staff observations are useful input, but letting staff set his goals removes him from his own treatment planning.

C. Write the goals in standard clinical language and read them aloud to him for his signature

Having him sign a plan he did not shape, in language he may not understand, is not meaningful collaboration.

D. Delay planning until a court-appointed guardian can be found to decide on his goals

Nothing indicates he lacks capacity or has a guardian; a disability alone does not remove his right to participate.

Rationale: Counselors and clients work jointly to devise counseling plans that fit the client's abilities, developmental level and circumstances. For a client with an intellectual disability, collaboration means adapting the process, using plain language, concrete examples and visual supports, rather than handing planning to others. Support staff can contribute valuable information when the client agrees.

References: ACA Code of Ethics — A.1.c Counseling Plans: plans consistent with the abilities, developmental level and circumstances of clients

☐ Approve ☐ Revise ☐ Retire Notes:

At the end of its first full year, a middle school's peer mediation program is evaluated so the school board can decide whether to continue funding it. Which type of evaluation is this?

A. Formative evaluation

Formative evaluation gathers feedback during a program to improve it, not to judge it for a funding decision.

B. Needs assessment

A needs assessment identifies gaps before a program is designed; it does not judge an existing program.

C. Single-subject design

A single-subject design tracks one person's behavior across phases, not a whole program's worth.

D. Summative evaluation ✓ KEY

Summative evaluation judges a program's overall effectiveness at the end of a cycle to inform decisions such as continuing funding.

Rationale: Summative evaluation takes place at the end of a program or program cycle and judges its overall merit or effectiveness, often to inform decisions about continuing, expanding or ending it. Formative evaluation, by contrast, happens while a program is running and is used to improve it. A board deciding on future funding needs a summative judgment.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: formative vs summative evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

To estimate the need for youth mental health services in a county, an agency analyzes existing census data, school dropout rates, juvenile arrest records and emergency department visits for self-harm. No residents or providers are contacted directly. Which needs assessment method is the agency using?

A. Key informant approach

This method gathers views from knowledgeable people such as clinicians or community leaders, who were not contacted here.

B. Community forum approach

A community forum invites residents to public meetings to voice needs; no residents were involved.

C. Social indicators approach ✓ KEY

This method infers need from existing public statistics such as census, school and health records.

D. Field survey of residents

A field survey collects new data directly from a sample of residents, which the agency did not do.

Rationale: Needs assessments can draw on several methods. The social indicators approach infers community need from existing public data, such as census figures, health statistics and school or court records. Key informant interviews, community forums and field surveys all collect new information directly from people. Many needs assessments combine methods because existing data alone may miss what residents and providers see.

References: Warheit et al. (Needs Assessment) — Social indicators approach: inferring need from existing public statistics · Erford (Research and Evaluation in Counseling) — Needs assessment methods in program evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

An evaluator of a new parenting program at a community center records how many sessions are offered, who attends, whether the target families of toddlers are being reached and whether facilitators cover each planned module. She does not yet measure changes in parenting. Which type of evaluation is she conducting?

A. Outcome evaluation

An outcome evaluation measures changes in participants, such as parenting skills, which she is not yet doing.

B. Process evaluation ✓ KEY

Process evaluation examines how a program is delivered: reach, attendance and whether it runs as planned.

C. Needs assessment

A needs assessment identifies gaps before a program is built; this program is already running.

D. Cost-benefit study

A cost-benefit study compares program costs with outcomes converted to dollars; no costs are examined here.

Rationale: Process (implementation) evaluation asks whether a program is being delivered as intended and to the intended population, tracking things like attendance, reach and coverage of planned content. Outcome evaluation asks whether participants changed as a result. Process data also help interpret outcomes, because a program that is not delivered as planned cannot fairly be judged ineffective.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: process vs outcome evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a rural clinic refers a 66-year-old client with worsening depression to a telepsychiatry service for a medication evaluation. The client signs a release so the two providers can share information. What should the counselor do to BEST complete the referral?

A. Follow up to confirm the appointment took place and share relevant information as agreed ✓ **KEY**

Closing the loop confirms the client connected with care and lets both providers coordinate treatment.

B. Give the client the service's phone number and leave the scheduling and follow-up to him

Passing on a number without follow-up is a passive referral and raises the risk that the client never connects.

C. End counseling once the referral is made, since the psychiatrist now manages his depression

A medication evaluation adds to counseling; ending services would leave the client without psychotherapy.

D. Wait to discuss the referral until his next monthly session in case he brings it up himself

Waiting passively for the client to raise it can leave a worsening depression without timely follow-up.

Rationale: A referral is complete only when the client actually connects with the new service. Good care coordination includes helping the client schedule, confirming the appointment occurred, and, with consent, sharing information so both providers work from the same plan. Active follow-up matters especially for older or rural clients, who may face transportation, technology or access barriers.

References: Gladding (Counseling: A Comprehensive Profession) — Clinical mental health counseling settings: referral, follow-up and coordination with medical providers

☐ Approve ☐ Revise ☐ Retire Notes:

Counseling Skills and Interventions

60 items · NCE weight 30% (48 of 160 scored items)

Q125 of 200 · nce-s-cou-241 · Counseling and Helping Relationships · gestalt therapy · medium · In review

A 34-year-old nurse works through a high fever because "real professionals do not take sick days," a rule her first charge nurse drilled into her. When her gestalt counselor asks whether she herself agrees with the rule, she looks puzzled and cannot say. Which contact boundary disturbance does this BEST illustrate?

A. Projection

Projection disowns parts of oneself by attributing them to others; she is not assigning her own traits to anyone.

B. Introjection ✓ **KEY**

She has swallowed another person's standard whole, without chewing it over to decide whether it fits her.

C. Deflection

Deflection avoids contact through vagueness, humor or changing the subject; the issue here is an unexamined rule.

D. Retroflection

Retroflection turns onto oneself what one would like to do to others, such as self-blame in place of anger.

Rationale: Gestalt therapy describes disturbances at the contact boundary between person and environment. In introjection a person uncritically accepts the beliefs and standards of others without assimilating them, so the rules feel like "shoulds" that are not truly the person's own. Gestalt work helps the client become aware of introjects and decide which to keep and which to reject.

References: Corey (Theory & Practice) — Gestalt Therapy: contact and resistances to contact (introjection)

☐ Approve ☐ Revise ☐ Retire Notes:

Q126 of 200 · nce-s-cou-242 · Counseling and Helping Relationships · gestalt therapy · easy · In review

While describing how her brother dismissed her at a family dinner, a client keeps making a small brushing-away motion with one hand. Her gestalt counselor asks her to make the motion larger and larger and then to give the hand a voice. Which technique is the counselor using?

A. Empty-chair dialogue

The empty chair has the client speak to an imagined person or part of the self, not amplify a gesture.

B. Making the rounds

Making the rounds is a group exercise in which a member speaks to or does something with each other member.

C. Reversal technique

Reversal asks the client to play the opposite of how they usually present, such as a shy client acting bold.

D. Exaggeration exercise ✓ **KEY**

Exaggeration amplifies a small movement or gesture so the client becomes more aware of the feeling it carries.

Rationale: Gestalt counselors attend closely to nonverbal behavior as a clue to feelings outside awareness. In the exaggeration exercise the client repeats and enlarges a movement, posture or phrase, which intensifies the feeling attached to it and helps the client own what the body is expressing.

References: Corey (Theory & Practice) — Gestalt Therapy: techniques (the exaggeration exercise)

☐ Approve ☐ Revise ☐ Retire Notes:

Midway through a session, a client says, "I'm stuck. I don't know what I feel, and I need you to tell me what to do," and then falls silent. Her gestalt counselor sees this as the impasse layer that Perls described. What would the counselor MOST likely do next?

A. Invite the client to stay with and fully experience the stuck feeling ✓ KEY

Gestalt counselors help clients remain with the impasse so they can work through it, rather than rescuing them from it.

B. Offer concrete advice so the client can move past the stuck point

Advice would accept the client's bid for outside support and block her from finding her own way through.

C. Teach a relaxation exercise to reduce the client's discomfort quickly

Lowering the discomfort right away would help her escape the impasse, not experience and move through it.

D. Interpret the silence as resistance rooted in an early childhood conflict

Interpreting the past is a psychoanalytic move; gestalt work stays with present awareness.

Rationale: Perls described layers of neurosis (phony, phobic, impasse, implosive and explosive). At the impasse, clients feel stuck and try to get the environment, including the counselor, to provide direction and support. The gestalt counselor resists rescuing and encourages the client to stay with the stuckness, because fully experiencing it allows the client to find inner support and move through it.

References: Corey (Theory & Practice) — Gestalt Therapy: layers of neurosis (the impasse)

☐ Approve ☐ Revise ☐ Retire Notes:

A retired bus driver tells his counselor, "I can't stand waiting at the pharmacy. Ten minutes in that line is unbearable, so I just leave without my medication." In Ellis's REBT, which irrational derivative BEST describes his belief?

A. Personalization

Personalization is a Beck distortion in which a person takes undue responsibility for outside events.

B. Global self-rating

Global rating condemns a whole person, as in "I am worthless"; he is not rating himself.

C. Low frustration tolerance ✓ KEY

Insisting that discomfort is unbearable ("I can't stand it") is Ellis's low frustration tolerance.

D. Overgeneralization

Overgeneralization draws a sweeping rule from one event; his belief concerns tolerating discomfort.

Rationale: Ellis held that rigid demands produce derivative irrational beliefs: awfulizing, low frustration tolerance ("I can't stand it") and global rating of self or others. Low frustration tolerance leads people to avoid short-term discomfort at the cost of long-term goals, as when this client leaves without his medication. REBT disputes the belief that discomfort cannot be borne.

References: Corey (Theory & Practice) — Rational Emotive Behavior Therapy: irrational beliefs and low frustration tolerance

☐ Approve ☐ Revise ☐ Retire Notes:

A client insists that his partner must agree with him on every major decision. His REBT counselor asks, "How does it follow that because you would strongly prefer her agreement, she therefore must agree?" Which type of disputing is this?

A. Empirical disputing

Empirical disputing asks for evidence, such as "Where is the proof that she must agree?"

B. Functional disputing

Functional disputing asks whether the belief helps, such as "Where does this demand get you?"

C. Logical disputing ✓ KEY

Logical disputing shows that a demand does not follow from a preference, however strong the preference is.

D. Philosophical disputing

Philosophical disputing asks whether life can still be satisfying even if the wish is not met.

Rationale: In the D step of the ABCDE model, REBT counselors challenge irrational beliefs in several ways. Logical disputes expose the leap from "I want" to "it must be"; empirical disputes test the belief against evidence; functional disputes question whether the belief serves the client's goals; philosophical disputes broaden the client's view of a satisfying life.

References: Corey (Theory & Practice) — Rational Emotive Behavior Therapy: disputing irrational beliefs (logical, empirical, functional, philosophical)

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old graduate student with panic attacks tells her REBT counselor, "Now I hate myself for being so weak that I panic at all." Following Ellis, what would the counselor typically address FIRST?

A. Her self-damning beliefs about having panic, before the panic itself ✓ KEY

Ellis often targets the secondary disturbance first, because self-condemnation about a symptom blocks work on it.

B. Graded exposure to the bodily cues of panic, before any self-anger

Exposure may come later; Ellis viewed the disturbance about the disturbance as the first obstacle.

C. The childhood experiences that may first have produced the panic

REBT focuses on present beliefs rather than on tracing the historical origins of symptoms.

D. Reassurance that panic is not dangerous and will fade with time

Reassurance does not dispute the demand that she must not be weak, so the self-damning belief remains.

Rationale: REBT distinguishes primary disturbance (here, the panic) from secondary disturbance, the "disturbance about the disturbance" such as hating oneself for panicking. Ellis advised that counselors often tackle the secondary problem first, because self-condemnation adds suffering and interferes with work on the primary problem. Unconditional self-acceptance helps the client view the symptom as a problem to solve rather than proof of worthlessness.

References: Ellis & Dryden (REBT) — Secondary disturbance (disturbance about disturbance) and addressing it first · Corey (Theory & Practice) — Rational Emotive Behavior Therapy: secondary disturbance (disturbance about disturbance)

☐ Approve ☐ Revise ☐ Retire Notes:

At the end of a first session, a counselor tells a couple who fight about chores, "Between now and the next time we meet, notice what happens in your relationship that you want to keep happening." In de Shazer's approach, this assignment is known as:

A. A paradoxical directive

A paradoxical directive prescribes the symptom; this task asks the couple to notice what already works.

B. An externalizing question

Externalizing separates the person from the problem in narrative therapy; this is an observation task.

C. A skills-rehearsal assignment

Nothing new is being practiced; the couple is asked only to notice existing positive events.

D. The formula first session task ✓ KEY

De Shazer's formula first session task asks clients to observe what they want to continue.

Rationale: De Shazer and colleagues developed the formula first session task as a standard assignment that shifts attention from problems to what is already going well. Because clients notice and report on positive events, the next session can build on existing strengths and exceptions rather than on a problem history.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: formula first session task

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor opens a second session by asking, "What's been better since we met?" A woman caring for her father with Parkinson's disease says there was one calm evening at home. Which response is MOST consistent with solution-focused practice?

A. "That's good. Now, what has been hardest for you this past week?"

Moving straight to what was hardest drops the exception before it can be explored and built on.

B. "What did you do that helped make that evening calmer than usual?" ✓ KEY

Asking how she made it happen amplifies the exception and credits her with the change.

C. "One calm evening is a start. What usually ruins the other evenings?"

Asking what ruins evenings returns the focus to problem talk rather than to what worked.

D. "Let's look at what stops calm evenings from happening more often."

Analyzing barriers is problem-focused; SFBT studies the exception to find what to do more of.

Rationale: Solution-focused counselors open later sessions by asking what is better and then elicit, amplify and reinforce any improvement (sometimes summarized as EARS). Asking in detail how the client made the better time happen highlights her own resources and points to what she can do more of. Problem analysis is set aside in favor of building on exceptions.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: exceptions and building on what works in later sessions

☐ Approve ☐ Revise ☐ Retire Notes:

Solution-focused brief therapy was developed mainly by which pair of clinicians at the Brief Family Therapy Center in Milwaukee?

A. Steve de Shazer and Insoo Kim Berg ✓ KEY

De Shazer and Berg developed SFBT at the Brief Family Therapy Center in Milwaukee.

B. Michael White and David Epston

White and Epston developed narrative therapy in Australia and New Zealand.

C. Jay Haley and Cloé Madanes

Haley and Madanes are associated with strategic family therapy.

D. Paul Watzlawick and John Weakland

Watzlawick and Weakland worked at the Mental Research Institute in Palo Alto.

Rationale: Steve de Shazer and Insoo Kim Berg developed solution-focused brief therapy at the Brief Family Therapy Center in Milwaukee, drawing on the MRI brief therapy tradition but shifting attention from problems to solutions. Its hallmark techniques include the miracle question, exception-finding and scaling questions.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: key figures (de Shazer and Berg)

☐ Approve ☐ Revise ☐ Retire Notes:

A narrative counselor first asks a 15-year-old, "How much of your school week has the Panic taken over?" Later the counselor asks, "When have you had some influence over the Panic, even a little?" This paired line of questioning is known as:

A. Scaling questioning

Scaling questions ask for a 0 to 10 rating and come from solution-focused therapy.

B. Circular questioning

Circular questions, from the Milan team, ask family members about relationships and differences among them.

C. The miracle question

The miracle question asks clients to imagine life after the problem is gone overnight.

D. Relative influence questioning ✓ KEY

Mapping the problem's influence on the person, then the person's influence on the problem, is relative influence questioning.

Rationale: White and Epston described relative influence questioning in two parts. The counselor first maps the influence of the externalized problem on the person's life and relationships, then maps the person's influence over the problem. The second set of questions brings forward unique outcomes that can anchor a preferred story.

References: Corey (Theory & Practice) — Narrative Therapy: externalizing conversations and relative influence questioning

☐ Approve ☐ Revise ☐ Retire Notes:

A 70-year-old widower says his life feels empty since his wife died. His narrative counselor asks who belongs in his "club of life," what his late wife would say she valued in him, and whether some members whose voices put him down should have less say. This practice is called:

A. Outsider-witness retelling

Outsider witnesses are invited audiences who listen and retell; no audience is present here.

B. Re-membering conversation ✓ KEY

Re-membering reviews and revises the membership of the client's club of life, upgrading some voices and downgrading others.

C. Deconstructive listening

Deconstructive listening attends to gaps and ambiguities in a story; it does not review life's membership.

D. Scaffolding conversation

Scaffolding moves clients in small steps from the familiar toward new ideas, not toward reviewing relationships.

Rationale: Drawing on Barbara Myerhoff, Michael White described identity as shaped by an "association" or club of life whose members include living and dead, present and absent figures. In re-membering conversations, the counselor helps the client honor members who support a preferred identity, such as a deceased spouse, and revoke or reduce the membership of those whose voices are unhelpful.

References: White (Maps of Narrative Practice) — Ch. 3, re-membering conversations and the club of life · Corey (Theory & Practice) — Narrative Therapy: re-authoring and re-membering conversations

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling student is asked to describe the counselor's position in narrative therapy as Michael White described it. Which description is MOST accurate?

A. An expert interpreter who uncovers the hidden meaning in the client's story

Narrative counselors avoid the expert-interpreter role; clients are the experts on their own lives.

B. A neutral observer who avoids any influence over the story being told

Narrative counselors do shape the conversation through their questions; they are not neutral observers.

C. Decentered but influential, treating the client as the expert on their life ✓ KEY

White described the counselor as decentered, with the client's knowledge central, yet influential through questioning.

D. A directive strategist who assigns tasks designed to disrupt the problem

Assigning disruptive tasks is characteristic of strategic therapy rather than narrative practice.

Rationale: Michael White described the ideal narrative stance as "decentered and influential." The client's own knowledge and skills are at the center, and the counselor does not impose expert interpretations; at the same time, the counselor shapes the conversation through careful questions that help the client externalize problems and build a richer preferred story.

References: Corey (Theory & Practice) — Narrative Therapy: the therapist's role and the client as expert

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old Latina office manager was told by human resources that she is "too emotional" about repeated sexual comments from a supervisor. Her feminist counselor describes her anger and poor sleep as understandable responses to an unjust situation rather than signs of personal pathology. Which intervention is the counselor using?

A. Gender-role analysis

Gender-role analysis explores the gender messages a client absorbed and their effects; this changes how symptoms are framed.

B. Reframing and relabeling ✓ KEY

Feminist reframing moves the problem from the client's defects to social and environmental conditions.

C. Bibliotherapy

Bibliotherapy uses readings to educate the client; no reading is assigned in this scenario.

D. Assertiveness training

Assertiveness training builds skills for stating needs; the counselor is reinterpreting her reactions.

Rationale: In feminist therapy, reframing and relabeling shift the focus from "blaming the victim" toward the social and political factors that contribute to a client's distress. Symptoms such as anger or poor sleep may be understood as coping or survival responses to oppression or harassment, which reduces self-blame and can open the way to change and social action.

References: Corey (Theory & Practice) — Feminist Therapy: techniques (reframing and relabeling)

☐ Approve ☐ Revise ☐ Retire Notes:

A client's insurer requires a diagnosis before it will pay for counseling. Her counselor practices from a feminist perspective. Which approach to DSM-5-TR diagnosis is MOST consistent with feminist therapy?

A. Use diagnosis cautiously and collaboratively, weighing social context and bias ✓ KEY

Feminist counselors share the process with clients and consider how oppression and bias shape symptoms.

B. Refuse to diagnose, since any label is a tool of patriarchal social control

Some feminist counselors decline to diagnose, but most use it with caution and collaboration.

C. Diagnose as any counselor would, since a diagnosis is a neutral clinical fact

Feminist therapy questions the neutrality of diagnosis and its potential for gender and cultural bias.

D. Diagnose only after the client has finished a complete gender-role analysis

No such rule exists; the issue is how diagnosis is used, not which technique must come first.

Rationale: Feminist therapists are wary of diagnostic labels that locate problems inside the individual and that may reflect gender, cultural or other bias. When a diagnosis is needed, many feminist counselors discuss it openly with the client, decide on it together and consider how social context, oppression and trauma contribute to symptoms. This fits the approach's egalitarian relationship and "personal is political" principle.

References: Corey (Theory & Practice) — Feminist Therapy: the role of assessment and diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

A DBT skills trainer describes a state of mind that integrates "emotion mind" and "reasonable mind," allowing a person to sense what is true and act from inner balance. What is this state called?

A. Beginner's mind

Beginner's mind refers to approaching each moment freshly; it is not the synthesis of the two states.

B. Radical acceptance

Radical acceptance is a distress tolerance skill for accepting reality fully, not a state of mind.

C. Self-soothing

Self-soothing is a distress tolerance skill using the five senses to comfort oneself.

D. Wise mind ✓ KEY

Wise mind is the integration of emotion mind and reasonable mind taught in the mindfulness module.

Rationale: Linehan's mindfulness module describes three states of mind. Emotion mind is driven by feelings, reasonable mind by logic and facts, and wise mind integrates both with intuitive knowing. The "what" and "how" mindfulness skills help clients reach wise mind before acting.

References: Linehan (DBT Skills Training Manual) — Mindfulness skills: wise mind (emotion mind and reasonable mind)

☐ Approve ☐ Revise ☐ Retire Notes:

After a minor slip of the tongue at a party, a client feels intense shame and wants to hide in her room all weekend. She and her DBT therapist agree that the shame does not fit the facts. Which emotion regulation skill would the therapist MOST likely coach?

A. Self-soothing with the five senses in her room

Self-soothing is a distress tolerance skill and would not counter the urge to hide.

B. Radical acceptance of the mistake she made

Radical acceptance is a distress tolerance skill; it does not change an unjustified emotion.

C. Opposite action, going out to see her friends ✓ KEY

When an emotion does not fit the facts, acting opposite to its urge (hiding) helps reduce it.

D. DEAR MAN, asking her friends for reassurance

DEAR MAN is an interpersonal effectiveness skill for making requests, not for changing shame.

Rationale: Opposite action is a core emotion regulation skill in DBT. When an emotion and its intensity do not fit the facts, the client acts fully opposite to the action urge; for unjustified shame, that means approaching and engaging rather than hiding. Repeated opposite action reduces the emotion over time.

References: Linehan (DBT Skills Training Manual) — Emotion regulation skills: checking the facts and opposite action

☐ Approve ☐ Revise ☐ Retire Notes:

A client's DBT diary card shows that this week she cut herself once, skipped her skills group twice and had a painful argument with her sister. According to the DBT target hierarchy for individual sessions, which issue should the therapist address FIRST?

A. The self-harm, as a life-threatening behavior ✓ **KEY**

Life-threatening behaviors, including self-harm, are the top priority in individual DBT sessions.

B. The missed group, as therapy-interfering behavior

Therapy-interfering behavior is the second target, addressed after life-threatening behavior.

C. The argument, as a quality-of-life interfering issue

Quality-of-life issues come third in the hierarchy, after the two higher targets.

D. Whichever topic the client most wishes to discuss

DBT uses the diary card to set the agenda by target priority, not by client preference alone.

Rationale: In stage 1 of DBT, individual therapy follows a target hierarchy: first life-threatening behaviors such as suicidal acts and self-harm, then therapy-interfering behaviors such as missed sessions, then quality-of-life interfering behaviors, and then increasing behavioral skills. The diary card shows which targets occurred during the week and guides the session agenda.

References: Linehan (DBT Skills Training Manual) — Overview of DBT: stage 1 target hierarchy and diary cards · Corey (Theory & Practice) — Dialectical behavior therapy: treatment targets

☐ Approve ☐ Revise ☐ Retire Notes:

In a child-centered play therapy session, a 4-year-old silently drives a toy truck into a tower of blocks and watches it fall. The counselor says, "You're driving that right into there, and down it goes." This response is BEST described as:

A. Interpretation

Interpretation assigns meaning to the play, such as anger at a sibling; the counselor only describes it.

B. Tracking ✓ **KEY**

Tracking describes what the child is doing, conveying the counselor's full attention without judgment.

C. Limit setting

Knocking down blocks harms no one, and the counselor sets no limit.

D. Esteem building

Esteem-building responses credit the child's effort or ability, such as "You figured that out."

Rationale: Child-centered play therapists use tracking to describe the child's play behavior aloud, often without naming toys the child has not named. Tracking shows the child that the counselor is present and interested, and it lets the child lead. Interpretations and evaluations are avoided because they shift direction away from the child.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: child-centered play therapy responses (tracking)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor about to begin child-centered play therapy with a 7-year-old wonders whether to read a list of playroom rules in the first few minutes. Following Landreth's guidance, what is the BEST approach to limits?

- A. Review all of the playroom rules at the start so the child knows what to expect
A list of rules at the outset can inhibit play and suggests that the counselor expects misbehavior.
- B. Ask the parent to explain the rules before the child enters the playroom
Limits belong in the child-counselor relationship and are set by the counselor when needed.
- C. Post the written rules on the wall and point to them when problems arise
Written rules do not fit a young child's needs and replace the relationship with an impersonal list.
- D. Set each limit only when it is needed, as situations arise during the play ✓ KEY
Landreth advises that limits are not needed until they are needed, so they are set when they arise.

Rationale: Landreth recommends against opening play therapy with a list of rules, which can make a child cautious and signal distrust. The counselor offers a general, permissive introduction and sets specific limits, such as not hurting oneself or the counselor, only when a situation calls for them, using the ACT steps.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: child-centered play therapy and limit setting

☐ Approve ☐ Revise ☐ Retire Notes:

A parent asks why the counselor plans to use play therapy with her 5-year-old instead of talking through the child's fears about a new baby in the family. Which explanation BEST reflects developmental reasoning?

- A. Play keeps the child busy so the counselor can watch for any hidden symptoms
Play is the medium of therapy itself, not a distraction that allows observation.
- B. Children this age find talk boring, so playing is a reward for coming each week
Play is not used as a reward; it matches how young children process and express experience.
- C. Young children lack abstract verbal reasoning, so play is their natural language ✓ KEY
Preoperational children express feelings and work through experience in play more readily than in words.
- D. Play lets the counselor interpret unconscious drives without the child's input
Modern play therapy follows the child's lead; it is not a way to bypass the child to interpret drives.

Rationale: In Piaget's preoperational stage (about ages 2 to 7), children think symbolically but lack the abstract reasoning and vocabulary needed to describe complex feelings. Play is their natural means of expression; as Landreth put it, toys are children's words and play is their language. Play therapy therefore meets young children at their developmental level.

References: Berk (Development Through the Lifespan) — Early childhood: Piaget's preoperational stage and make-believe play · Gladding (Counseling: A Comprehensive Profession) — Counseling children: rationale for play therapy

☐ Approve ☐ Revise ☐ Retire Notes:

For several weeks a psychodynamic counselor has noticed that a client's rage at his wife's mild comments seems to echo his anger at his critical mother. When is an interpretation of this link MOST likely to be useful?

A. When the material is close to awareness and the client can tolerate hearing it ✓ KEY

Interpretations work best when the client is nearly ready to see the link and can absorb it.

B. As early as possible, so the client gains insight before his defenses harden

Premature interpretations tend to raise anxiety and resistance, and the client may reject them.

C. Only after the client asks the counselor directly to explain what is happening

Timing depends on the client's readiness, not on whether the client explicitly requests an interpretation.

D. Once the counselor is certain of the link, whatever the client's current readiness

Counselor certainty does not make an interpretation usable; the client's readiness is what matters.

Rationale: In psychoanalytic practice, interpretation explains the meaning of behavior, dreams, resistance and transference. Corey notes that timing matters: interpretations should be offered when the phenomenon is close to conscious awareness, should start from the surface and move only as deep as the client can go emotionally, and should follow a sound working relationship.

References: Corey (Theory & Practice) — Psychoanalytic Therapy: interpretation and its timing

☐ Approve ☐ Revise ☐ Retire Notes:

A doctoral student loves her evening run but keeps putting off writing her dissertation. Her counselor suggests that she go running only after she has written for one full hour. Which behavioral principle is the counselor applying?

A. Shaping

Shaping reinforces successive approximations of a target behavior, not one activity used as a reward for another.

B. The Premack principle ✓ KEY

A high-frequency activity (running) is made contingent on a low-frequency one (writing) to reinforce it.

C. Negative reinforcement

Negative reinforcement removes an aversive stimulus; here a preferred activity is added after writing.

D. Stimulus generalization

Generalization is responding to stimuli similar to the original; it is not a reinforcement plan.

Rationale: The Premack principle, sometimes called "Grandma's rule," holds that a more probable behavior can reinforce a less probable one when access to it depends on first doing the less probable behavior. Counselors use it in self-management plans by making a favorite activity contingent on completing an avoided task.

References: Corey (Theory & Practice) — Behavior Therapy: operant techniques and self-management (Premack principle)

☐ Approve ☐ Revise ☐ Retire Notes:

In a family session, a quiet 16-year-old's complaints are repeatedly overridden by her father and older brother, who form a strong coalition. A structural family therapist deliberately sides with the teenager for a period, backing her view until the family's hierarchy begins to shift. This technique is:

A. Detriangulation

Detriangulation is a Bowenian goal of removing oneself from a triangle, not siding with a member.

B. Reframing

Reframing gives a new meaning to a behavior; the therapist here is changing alliances, not meanings.

C. Enactment

An enactment has the family act out a transaction in session; it does not involve taking a side.

D. Unbalancing ✓ KEY

Minuchin's unbalancing has the therapist temporarily join one member to change the family's power structure.

Rationale: In structural family therapy, Minuchin used unbalancing to challenge a rigid hierarchy or coalition. By temporarily affiliating with one family member, often the less powerful one, the therapist disrupts the usual distribution of power and forces the family to find new ways of relating. The therapist must later restore balance and avoid becoming fixed in a coalition.

References: Goldenberg (Family Therapy) — Structural family therapy: restructuring techniques (unbalancing)

☐ Approve ☐ Revise ☐ Retire Notes:

A shy 23-year-old says he wishes he were the kind of person who could start conversations with coworkers. His Adlerian counselor asks him, for one week, to behave as though he already were that person. Which technique is this?

A. Spitting in the soup

Spitting in the soup names the hidden payoff of a behavior to make it less appealing.

B. Catching oneself

Catching oneself means noticing an old pattern as it starts; it does not ask for new role behavior.

C. Acting as if ✓ KEY

Acting as if invites the client to try on the role of the person he wishes to be.

D. Push-button technique

The push-button technique has clients alternate pleasant and unpleasant memories to see that they create feelings.

Rationale: Adlerian counselors use "acting as if" to help clients test new behavior that they believe is beyond them. Like an actor trying on a role, the client behaves for a set time as though he already had the desired quality, which can challenge mistaken beliefs about his limits and build courage.

References: Corey (Theory & Practice) — Adlerian Therapy: reorientation techniques (acting as if)

☐ Approve ☐ Revise ☐ Retire Notes:

Rogers described accurate empathic understanding as sensing the client's private world "as if" it were the counselor's own. What does the "as if" quality emphasize?

A. The counselor appears to share the client's feelings in order to build rapport faster

Rogers did not mean pretending; genuine empathy requires congruence, not an act.

B. The counselor enters the client's world while keeping a separate sense of self ✓ **KEY**

The counselor feels the client's world deeply without losing the "as if" and merging with it.

C. The counselor imagines how they themselves would feel in the client's situation

Imagining one's own reaction projects the counselor's frame onto the client instead of grasping the client's.

D. The counselor treats the client's account as a hypothesis to be checked against facts

Testing the client's account against facts is a cognitive stance, not person-centered empathy.

Rationale: For Rogers, accurate empathy means sensing the client's feelings and meanings as if they were one's own, but without ever losing the "as if" quality. The counselor stays a separate person, which keeps the counselor from being overwhelmed or confusing personal reactions with the client's experience, while still communicating deep understanding.

References: Corey (Theory & Practice) — Person-Centered Therapy: accurate empathic understanding

☐ Approve ☐ Revise ☐ Retire Notes:

An existential counselor notices that a 52-year-old engineer talks easily about his job and his duties to relatives, but goes blank when asked about his own values, feelings and sense of who he is. Which mode of being-in-the-world appears MOST neglected?

A. Eigenwelt (own world) ✓ **KEY**

The Eigenwelt is one's relationship with oneself, including values, feelings and self-awareness.

B. Umwelt (natural world)

The Umwelt is the physical and biological world; nothing suggests he neglects his body or surroundings.

C. Mitwelt (social world)

The Mitwelt is the world of relationships with others, which he engages in readily.

D. Überwelt (spiritual world)

The Überwelt concerns ideals and beliefs about the world; the clearest gap is his relation to himself.

Rationale: Existential therapists, building on Binswanger and May, describe being-in-the-world in several modes: the Umwelt (natural or physical world), the Mitwelt (world of relationships), the Eigenwelt (relationship with oneself) and, in van Deurzen's addition, the Überwelt (spiritual or ideal world). A person can be engaged in some modes while neglecting others, and therapy explores the neglected dimension.

References: Corey (Theory & Practice) — Existential Therapy: modes of being in the world (Umwelt, Mitwelt, Eigenwelt, Überwelt)

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "If I make a mistake in the meeting, people will notice." The counselor asks, "And if they notice, what would that mean about you?" The counselor repeats this kind of question after each answer until the client says, "That I'm incompetent." Which technique is this?

A. Decatastrophizing

Decatastrophizing examines how likely and how bad a feared outcome is, rather than its meaning about the self.

B. Thought stopping

Thought stopping interrupts unwanted thoughts; it does not explore what lies beneath them.

C. Cognitive rehearsal

Cognitive rehearsal mentally practices coping with a future situation step by step.

D. Downward arrow technique ✓ KEY

Repeatedly asking what a thought would mean moves from automatic thoughts down to a core belief.

Rationale: In cognitive therapy, the downward arrow technique uncovers intermediate and core beliefs that lie beneath automatic thoughts. The counselor repeatedly asks what the thought would mean if it were true, until the client reaches a basic belief about the self, such as "I'm incompetent," which can then become a target for change.

References: J. Beck (CBT: Basics and Beyond) — Identifying intermediate and core beliefs: the downward arrow technique · Corey (Theory & Practice) — Cognitive Behavior Therapy: Beck's cognitive therapy, identifying core beliefs

☐ Approve ☐ Revise ☐ Retire Notes:

A father admits that he nags, criticizes and threatens his 14-year-old about grades, and their relationship has grown cold. In Glasser's choice theory, what would a reality therapist encourage him to use in place of these "deadly habits"?

A. Consistent consequences set out in a written behavior contract

Consequences used to force compliance still rely on external control, which Glasser rejected.

B. Praise and tangible rewards delivered right after good grades

Rewarding to control, or bribing, is itself one of Glasser's seven deadly habits.

C. Caring habits such as listening, supporting and encouraging ✓ KEY

Glasser's caring habits build the relationship that external control damages.

D. Firmer limits paired with calm, logical explanation of rules

Firm limits imposed to control the teen do not replace control with connection.

Rationale: Glasser contrasted external control psychology with choice theory. His seven deadly habits (criticizing, blaming, complaining, nagging, threatening, punishing and bribing or rewarding to control) damage relationships, while the seven caring habits (supporting, encouraging, listening, accepting, trusting, respecting and negotiating differences) strengthen the connection people need to meet their basic needs.

References: Corey (Theory & Practice) — Reality Therapy: choice theory, external control and caring versus deadly habits

☐ Approve ☐ Revise ☐ Retire Notes:

In a primary care clinic, a 58-year-old client asks a counselor, "Is it really that bad to have four beers every night?" Which response BEST follows the elicit-provide-elicít approach used in motivational interviewing?

A. "What have you heard about it? I can share what we know, then hear your view." ✓ KEY

This asks what the client knows, offers information with consent, and plans to ask for the client's reaction.

B. "Yes, that is well above safe limits, so you should cut back to two at most."

Giving unsolicited advice and a directive reflects the righting reflex and invites sustain talk.

C. "Why do you think you need four beers a night just to unwind after work?"

A "why" question about his need can sound judgmental and draws reasons for drinking.

D. "Let's hold that question and first talk about how your week has gone."

Deflecting the question misses a natural chance to exchange information.

Rationale: Motivational interviewing uses elicit-provide-elicít to share information in a way that respects autonomy. The counselor first asks what the client already knows or wants to know, then offers information with permission and in a neutral way, and then asks what the client makes of it. This avoids the righting reflex and keeps the client active in the conversation.

References: Miller & Rollnick (MI) — Exchanging information: elicit-provide-elicít

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old with PTSD and alcohol use disorder is in early recovery and becomes overwhelmed when trauma memories come up. Her counselor begins Seeking Safety, the model developed by Lisa Najavits. Which feature BEST describes this model?

A. It requires a detailed trauma narrative before coping skills are taught

Seeking Safety does not require clients to tell their trauma story in detail.

B. It is present-focused, teaching safe coping for trauma and substance use ✓ KEY

Seeking Safety is an integrated, present-focused model that builds safe coping skills for both conditions.

C. It treats the drinking first and delays trauma work until sobriety is stable

Seeking Safety addresses trauma and substance use at the same time rather than in sequence.

D. It uses repeated imaginal exposure to the worst memory in every session

Repeated imaginal exposure belongs to prolonged exposure, not to Seeking Safety.

Rationale: Seeking Safety is an integrated, present-focused treatment for co-occurring trauma and substance use. Its goal is safety: it teaches cognitive, behavioral and interpersonal coping skills that apply to both problems, without requiring clients to explore trauma memories in detail. This makes it suited to early recovery, when detailed trauma processing may destabilize the client.

References: SAMHSA TIP 57 — Trauma-specific services: integrated models for trauma and substance use (Seeking Safety)

☐ Approve ☐ Revise ☐ Retire Notes:

In the first two sessions of a closed group for adults coping with job loss, members talk politely about safe topics, look to the leader for direction and quietly test whether it is safe to share. According to Corey's model of group development, which stage is this group MOST likely in?

A. Transition stage

The transition stage is marked by open anxiety, defensiveness, conflict and challenges to the leader, not polite caution.

B. Working stage

The working stage features trust, cohesion and deep self-disclosure, which this group has not yet reached.

C. Initial stage ✓ KEY

Orientation, tentative and superficial sharing, reliance on the leader and testing of trust define the initial stage.

D. Final stage

The final stage involves consolidating learning and preparing to end, which does not fit a group in its first sessions.

Rationale: Corey describes groups moving from a pregroup stage through initial, transition, working and final stages. In the initial stage members orient themselves, share cautiously, look to the leader for structure and test whether the group is a safe place. Leaders help by clarifying norms and goals and modeling openness so that trust can develop.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: stages of group development (initial stage)

☐ Approve ☐ Revise ☐ Retire Notes:

After several tense sessions in which members argued and challenged the leader, a workplace communication group begins agreeing on how to give feedback. Members start supporting each other and speak of the group as "we." According to Tuckman, which stage has the group entered?

A. Norming ✓ KEY

Norming follows storming: the group settles on shared rules, cohesion grows and members begin to support one another.

B. Performing

Performing comes after norming, when roles are flexible and the group's energy goes mainly into productive work.

C. Forming

Forming is the opening stage of orientation and dependence on the leader, which this group has passed.

D. Adjourning

Adjourning, added later by Tuckman and Jensen, concerns ending the group and is not described here.

Rationale: Tuckman's sequence is forming, storming, norming and performing, with adjourning added later with Jensen. After the conflict of storming, a group that works through its disagreements develops shared norms, cohesion and a sense of "we" in the norming stage. Performing follows, when the group can focus its energy on its task.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: Tuckman's stages of group development

☐ Approve ☐ Revise ☐ Retire Notes:

A closed 10-week counseling group for college students has two sessions left. Which leader focus BEST fits Corey's description of the tasks of the final stage?

A. Introducing deeper confrontation so that members can open up new core issues to work on
Opening major new issues near the end leaves members with work they cannot finish in the time left.

B. Reviewing ground rules and helping each member define personal goals for the coming weeks
Setting ground rules and goals are initial-stage tasks, not tasks for the last two sessions.

C. Encouraging members to challenge the leader's authority so that hidden conflict can surface
Challenges to the leader are typical of the transition stage and are not a final-stage aim.

D. Helping members consolidate learning, voice feelings about ending and plan to apply gains ✓ KEY
Consolidating learning, dealing with feelings about separation and planning transfer to daily life are core final-stage tasks.

Rationale: In the final stage, the leader helps members review what they have learned, express feelings about the group ending, finish unfinished business and plan how to carry changes into their lives. Leaders avoid opening major new issues that cannot be worked through. Follow-up after the group can support transfer of learning.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: termination of a group and the final stage

☐ Approve ☐ Revise ☐ Retire Notes:

A hospital counselor chairs a committee of nurses, chaplains and social workers that meets monthly to develop a new discharge-planning protocol. Under the Association for Specialists in Group Work (ASGW) classification of groups, this is BEST described as which type?

A. Psychoeducation group
Psychoeducation groups teach information and skills to members to prevent problems or build coping.

B. Task and work group ✓ KEY
Task and work groups, such as committees and planning teams, exist to accomplish a defined work goal.

C. Counseling group
Counseling groups address members' personal and interpersonal concerns, not an organizational product.

D. Psychotherapy group
Psychotherapy groups treat members with more serious or long-standing psychological problems.

Rationale: ASGW identifies four group work specializations: task and work groups, psychoeducation groups, counseling groups and psychotherapy groups. A committee meeting to produce a protocol is a task group, because its purpose is to complete work rather than to change members personally. Leaders of task groups attend to both the task and the group's process.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: types of groups (task/work, psychoeducational, counseling, psychotherapy)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor runs an open-ended outpatient therapy group. A new member will join next week, and several veteran members worry that "starting over" will slow the group down. What is the BEST way to bring the newcomer in?

A. Prepare the newcomer beforehand and let veteran members convey the group's norms and work ✓ KEY

Pre-group preparation plus the veterans' modeling lets the newcomer learn the culture while the group keeps working.

B. Pause the group's ongoing themes for several weeks so the newcomer can catch up on its history

Halting the group's work to review history frustrates veterans and is not needed for the newcomer to join.

C. Ask the newcomer to stay silent and only observe for the first few sessions before taking part

Enforced silence isolates the newcomer and delays the engagement that helps a new member settle in.

D. Tell the veterans their worry is unfounded and move straight back to the planned session topic

Dismissing the veterans' reactions misses useful here-and-now material about change in the group.

Rationale: In open groups, new members are prepared individually before joining, and veteran members usually transmit the group's norms quickly by example. The group does not need to stop its work, and members' reactions to a newcomer, such as worry or rivalry, can be explored as here-and-now material.

References: Yalom & Leszcz (Group Psychotherapy) — Problems of membership: adding new members to an ongoing group

☐ Approve ☐ Revise ☐ Retire Notes:

In a substance use recovery group, a new member says, "Hearing Rosa talk about two years sober makes me think I might actually get there too." Which of Yalom's therapeutic factors is MOST evident?

A. Universality

Universality is relief at learning others share one's problems, not optimism drawn from another's progress.

B. Imitative behavior

Imitative behavior means copying another's behaviors; the member describes hope, not modeling a skill.

C. Instillation of hope ✓ KEY

Seeing a member who has improved with the same problem builds belief that one's own recovery is possible.

D. Development of socializing techniques

This factor concerns learning basic social skills, which is not what the member describes.

Rationale: Instillation of hope is the belief that the group can help, and it grows when members see others further along in recovery. Yalom notes that self-help and recovery groups lean heavily on this factor by featuring members who have succeeded. Hope keeps members in treatment long enough for other factors to work.

References: Yalom & Leszcz (Group Psychotherapy) — The therapeutic factors: instillation of hope

☐ Approve ☐ Revise ☐ Retire Notes:

A retired teacher in a depression group says the best part of the group was learning that her encouragement helped another member apply for a job: "For the first time in months, I feel useful to someone." Which of Yalom's therapeutic factors does this BEST illustrate?

A. Universality

Universality is discovering that others share one's problems; she is describing the value of helping.

B. Altruism ✓ KEY

Being able to help another member raises self-worth and pulls the helper out of self-absorption.

C. Catharsis

Catharsis is the release of strong feeling, which is not the source of her sense of usefulness.

D. Imparting information

Imparting information is didactic teaching or advice received, not the benefit of giving help to others.

Rationale: Yalom's altruism factor holds that members benefit from giving as well as receiving: offering support, insight and encouragement shows them they have something of value to others. This is especially powerful for people with depression or low self-esteem who see themselves as a burden.

References: Yalom & Leszcz (Group Psychotherapy) — The therapeutic factors: altruism

☐ Approve ☐ Revise ☐ Retire Notes:

Late in a cancer survivors' group, a member says the group taught him that however much support he receives, he must still face his own mortality and take ultimate responsibility for how he lives. Which of Yalom's therapeutic factors does this BEST reflect?

A. Universality

Universality is the relief that others share one's problems; his insight is about facing life alone and choosing.

B. Catharsis

Catharsis is emotional release; his statement describes an insight about mortality and responsibility.

C. Interpersonal learning

Interpersonal learning is about how one affects and is seen by others, not about existential givens.

D. Existential factors ✓ KEY

Recognizing mortality, aloneness and ultimate responsibility for one's life defines the existential factors.

Rationale: Yalom's existential factors include recognizing that life can be unfair, that pain and death cannot be escaped, that one faces life alone however close one is to others, and that one is ultimately responsible for how one lives. Members of groups facing serious illness often rank these factors highly. The distractors name other factors with a different focus.

References: Yalom & Leszcz (Group Psychotherapy) — The therapeutic factors: existential factors

☐ Approve ☐ Revise ☐ Retire Notes:

In the fifth session of a support group for new parents, one member has said little since the first meeting, though she listens closely and nods often as others talk. Which leader response is BEST?

A. Note her nonverbal reactions and invite her, without pressure, to share what is resonating ✓ KEY

Drawing out a quiet member gently, using what the leader observes, invites participation while respecting her pace.

B. Ask the other members to wait in silence until she takes a turn and speaks to the group today

Putting her on the spot in front of the group creates pressure and can increase her withdrawal.

C. Tell her privately after the session that she must speak up more if she wants to stay in the group

A coercive ultimatum violates members' right to decide what they share and damages trust.

D. Leave her alone, since a quiet member is unaffected by the group and unlikely to benefit from it

Silent members can be deeply engaged; ignoring her misses a chance to include her and learn what silence means.

Rationale: Silence can reflect anxiety, cultural norms, deep engagement or feeling unwelcome, so leaders explore it rather than assume.

Drawing out is a leadership skill that invites a quiet member to participate, often by noting observable reactions, while respecting the member's right to choose what to share. Pressuring or ignoring the member are both unhelpful.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: leadership skills and problem behaviors (silent members, drawing out)

☐ Approve ☐ Revise ☐ Retire Notes:

In a high school counseling group, a member starts describing in detail what an absent member's boyfriend supposedly did at a party, and other members lean in to hear more. Which leader skill is MOST needed right now?

A. Linking

Linking connects members' shared concerns; it would not stop harmful talk about an absent member.

B. Drawing out

Drawing out invites quieter members to speak, which would add to the gossip rather than stop it.

C. Summarizing

Summarizing pulls themes together; it does not intervene in a counterproductive behavior.

D. Blocking ✓ KEY

Blocking stops counterproductive behavior such as gossip or invading an absent member's privacy.

Rationale: Blocking is the leader's intervention to stop behavior that harms members or the group, such as gossiping, breaking confidentiality, scapegoating or pressuring disclosure. The leader blocks the behavior, not the person, and can redirect the speaker toward her own feelings in the here and now.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: group leadership skills (blocking)

☐ Approve ☐ Revise ☐ Retire Notes:

An 11-year-old tells his counselor he has crossed hairstylist and bus driver off his list because "people in my family go to college and get important jobs." In Gottfredson's theory of circumscription and compromise, this reasoning reflects which stage?

A. Orientation to size and power

This first stage (about ages 3 to 5) involves seeing jobs as adult roles, not ranking them by status.

B. Orientation to sex roles

At about ages 6 to 8 children rule out jobs as wrong for their gender, which is not his reason here.

C. Orientation to social valuation ✓ KEY

At about ages 9 to 13 children rule out jobs seen as too low in prestige for their social class or ability.

D. Orientation to internal unique self

From about age 14, youth weigh interests, values and personality within the options left, a later stage.

Rationale: Gottfredson proposed that children progressively eliminate (circumscribe) occupations: first by size and power, then by sex type, then by prestige and social valuation, and finally by fit with the internal, unique self. Around ages 9 to 13, children become aware of social class and set a floor of acceptable prestige, ruling out jobs seen as beneath their family's expectations.

References: Sharf (Career Development Theory) — Gottfredson's theory of circumscription and compromise: stages of circumscription

☐ Approve ☐ Revise ☐ Retire Notes:

Using the CASVE cycle from the cognitive information processing approach, a counselor has helped a client understand her values, skills and the options open to her. They now expand her list of possible occupations and then narrow it to three or four realistic choices. Which phase are they in?

A. Analysis

Analysis, the step already completed, builds understanding of self and options.

B. Synthesis ✓ KEY

Synthesis elaborates options and then crystallizes them into a short list of realistic choices.

C. Valuing

Valuing comes next, weighing each short-listed option's costs and benefits for self and others.

D. Communication

Communication is the first phase, when a person recognizes a gap between where they are and want to be.

Rationale: The CASVE cycle of Sampson, Peterson, Lenz and Reardon moves through Communication, Analysis, Synthesis, Valuing and Execution. In Synthesis, the person first elaborates possible options and then crystallizes them into a manageable set of about three to five. Valuing then ranks those options, and Execution turns the first choice into an action plan.

References: Sharf (Career Development Theory) — Cognitive information processing approach: the CASVE cycle

☐ Approve ☐ Revise ☐ Retire Notes:

In Savickas's career construction interview, a counselor asks a client whom she admired when she was growing up, other than her parents. According to career construction theory, what do her answers MOST directly reveal?

A. The occupations her parents expected her to enter and how she has met those expectations

The question deliberately excludes parents; it targets figures she chose for herself.

B. Her current level of career adaptability across concern, control, curiosity and confidence

Adaptability resources are assessed separately; the role model question is about self-concept.

C. Her self-concept: the traits she has chosen as a blueprint for the self she is constructing ✓ KEY

Role models reveal the qualities a person has chosen to incorporate, a blueprint for self-construction.

D. Her Holland code, since admired figures predict which work setting will be most congruent

Magazines and favorite settings point to preferred work environments; role models speak to the self.

Rationale: In Savickas's career construction interview, early role models are treated as a blueprint for the self: the traits a client admired are ones she has chosen to build into her own identity. Other questions, such as favorite magazines or television shows, point to preferred work settings, and early recollections reveal life themes or preoccupations. Together they help the client author a coherent career story.

References: Sharf (Career Development Theory) — Savickas's career construction theory: the career construction interview and role models

☐ Approve ☐ Revise ☐ Retire Notes:

A 21-year-old who earlier explored several fields has committed to nursing and is now deciding to specialize in pediatric intensive care. In Ginzberg, Ginsburg, Axelrad and Herma's theory, she is in which substage of the realistic period?

A. Specification ✓ KEY

Specification is the final realistic substage, when a person selects a specific job or specialty.

B. Crystallization

Crystallization is committing to a general field, which she has already done by choosing nursing.

C. Exploration

Exploration is the first realistic substage, when options are still being sampled broadly.

D. Transition

Transition is the last substage of the tentative period in adolescence, not of the realistic period.

Rationale: Ginzberg and colleagues described fantasy, tentative and realistic periods of career choice. The tentative period includes interest, capacity, value and transition substages. The realistic period, from about age 17 into the early twenties, moves through exploration, crystallization around a field and specification of a particular job or specialty.

References: Sharf (Career Development Theory) — Developmental theories: Ginzberg and colleagues' periods of career choice

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old says he has not thought much about what he wants in a career, religion or politics, has no plans and does not see why it matters. He is not distressed about this. Which of Marcia's identity statuses BEST fits?

A. Identity moratorium

Moratorium involves active exploration without commitment, often with some anxiety.

B. Identity foreclosure

Foreclosure is commitment without exploration, usually to values taken from parents.

C. Identity achievement

Achievement follows exploration with firm commitments, neither of which he shows.

D. Identity diffusion ✓ KEY

Diffusion is neither exploring nor committing, often with apathy about identity questions.

Rationale: Marcia classified identity status by two dimensions: exploration and commitment. Achievement has both, moratorium has exploration without commitment, foreclosure has commitment without exploration, and diffusion has neither. Diffused young people often seem aimless or unconcerned about who they are becoming.

References: Berk (Development Through the Lifespan) — Adolescence: Marcia's identity statuses

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor sets up regular meetings between a 9-year-old's parents and his teacher so that home and school follow the same homework and behavior plan. In Bronfenbrenner's ecological systems theory, the counselor is strengthening which system?

A. Microsystem

Each setting alone, home or classroom, is a microsystem; the links between them are something more.

B. Mesosystem ✓ KEY

The mesosystem is the set of connections between microsystems, such as home and school.

C. Exosystem

The exosystem holds settings the child is not in that affect him, such as a parent's workplace.

D. Macrosystem

The macrosystem is the broad culture, values and laws, not specific home-school links.

Rationale: Bronfenbrenner described nested systems: the microsystem of immediate settings, the mesosystem of connections among those settings, the exosystem of settings that affect the child indirectly, the macrosystem of cultural values and laws, and the chronosystem of change over time. Strong home-school partnerships are a classic mesosystem support.

References: Berk (Development Through the Lifespan) — History, theory and research strategies: Bronfenbrenner's ecological systems theory

☐ Approve ☐ Revise ☐ Retire Notes:

An 8-year-old explains that he shares his snack with a classmate "because then she'll share her cookies with me tomorrow." For him, right action is whatever serves his own interests through fair trades. In Kohlberg's theory, this reasoning BEST fits which stage orientation?

A. Punishment and obedience

Stage 1 reasoning centers on avoiding punishment from authority, not on making even trades.

B. Interpersonal conformity

Stage 3 reasoning seeks approval as a good person and to maintain relationships.

C. Instrumental exchange ✓ KEY

Stage 2 sees right action as meeting one's own needs through fair, tit-for-tat exchanges.

D. Social contract orientation

Stage 5 views laws as flexible agreements serving the greater good, far beyond this reasoning.

Rationale: Kohlberg's preconventional level has two stages: Stage 1, punishment and obedience, and Stage 2, the instrumental purpose orientation, in which right action is what satisfies one's own needs, often through equal exchange ("you scratch my back, I'll scratch yours"). Conventional reasoning (Stages 3 and 4) shifts toward approval and social order.

References: Berk (Development Through the Lifespan) — Moral development: Kohlberg's preconventional level (Stage 2, instrumental purpose orientation)

☐ Approve ☐ Revise ☐ Retire Notes:

A 5-year-old talks aloud to herself while working on a difficult puzzle ("No, the corner piece goes here first...") and talks more as the task gets harder. How would Vygotsky MOST likely interpret this behavior?

A. As self-guiding speech that helps her plan and that will later become silent inner speech ✓ KEY

Vygotsky saw private speech as a tool for self-regulation that rises with task difficulty and is later internalized.

B. As egocentric speech that shows she cannot yet take the perspective of a nearby listener

This is Piaget's view of such speech, which Vygotsky disputed.

C. As a sign of possible language delay that calls for a speech-language evaluation soon

Private speech is typical in the preschool years and is not a sign of delay.

D. As attention-seeking behavior that adults should ignore so that it decreases over time

The child is directing her own actions, not seeking attention, so ignoring it misreads its purpose.

Rationale: Piaget called children's self-talk egocentric speech, reflecting an inability to take others' perspectives. Vygotsky argued instead that this private speech is a tool for guiding thought and behavior, increases when tasks are challenging, and gradually becomes silent inner speech. Research has generally supported Vygotsky's view.

References: Berk (Development Through the Lifespan) — Early childhood cognition: Vygotsky's private speech versus Piaget's egocentric speech

☐ Approve ☐ Revise ☐ Retire Notes:

In the Strange Situation, a 12-month-old explores the toys with little attention to his mother, shows little distress when she leaves and, when she returns, looks away and does not seek contact. Which attachment classification BEST fits?

A. Secure

Secure infants use the parent as a base, may be upset at separation and seek comfort on reunion.

B. Resistant (ambivalent)

Resistant infants are highly distressed at separation and both seek and resist contact on reunion.

C. Disorganized

Disorganized infants show confused or contradictory behavior such as freezing or dazed looks.

D. Avoidant ✓ KEY

Avoidant infants show little separation distress and avoid or ignore the parent on reunion.

Rationale: Ainsworth's Strange Situation classifies infants mainly by reunion behavior. Avoidant infants seem unresponsive to the parent, are not distressed by separation and turn away on return; this pattern is linked to caregiving that rebuffs bids for comfort. Main and Solomon later added the disorganized category.

References: Berk (Development Through the Lifespan) — Emotional and social development in infancy: measuring attachment security (the Strange Situation)

☐ Approve ☐ Revise ☐ Retire Notes:

Ainsworth's home observations linked infants' later attachment classifications to how their caregivers had responded during the first year. Which caregiving pattern is MOST strongly associated with secure attachment?

A. Responding to each whimper at once so that the infant rarely experiences any distress at all

Sensitivity is about accurate, well-matched responses, not removing all distress.

B. Holding the infant as many hours as possible, whatever signals the infant may be giving

Contact that ignores the infant's cues is intrusive rather than sensitive.

C. Noticing the infant's signals, reading them accurately and responding promptly and fittingly ✓ KEY

Sensitive responsiveness, as Ainsworth defined it, is the caregiving feature most tied to secure attachment.

D. Giving consistent physical care on a fixed schedule while limiting emotional responsiveness

Routine care with little emotional responsiveness is more consistent with avoidant patterns.

Rationale: Ainsworth found that sensitive caregiving, meaning noticing the infant's signals, interpreting them accurately and responding promptly and appropriately, predicted secure attachment. Inconsistent care was linked to resistant attachment, and rejecting or emotionally unavailable care to avoidant attachment. Research since then shows a moderate link between sensitivity and security.

References: Berk (Development Through the Lifespan) — Emotional and social development in infancy: factors that affect attachment security (quality of caregiving)

☐ Approve ☐ Revise ☐ Retire Notes:

A client mentions that she skipped her sister's wedding. The counselor asks, "Why did you do that?" and the client becomes guarded. What is the MOST likely problem with the counselor's question?

A. "Why" questions can sound judgmental and lead the client to defend or justify herself ✓ KEY

"Why" questions often imply criticism and invite explanations and defensiveness rather than exploration.

B. It is a closed question, so it limits the client to a brief yes-or-no type of response

A "why" question is open, not closed; the problem is its judgmental tone.

C. It shifts the focus from the client's own experience to her sister's needs and feelings

The question stays on the client's actions; it does not shift focus to the sister.

D. It asks about the past, and counselors should limit questions to the present moment

Exploring past events is often useful; the timing of the event is not the issue.

Rationale: Questions beginning with "why" tend to put clients on the defensive because they can sound like a demand for justification. "What" and "how" questions, such as "What led to that decision?", invite description and exploration. Counselors also avoid strings of questions that make a session feel like an interrogation.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: questions and the problems with "why" questions

☐ Approve ☐ Revise ☐ Retire Notes:

Reviewing a recording, a supervisor notes that a trainee often changes the subject, jumping to topics that interest the trainee instead of staying with what the client just said. Which component of Ivey's attending behavior should the trainee practice?

A. Eye contact

Eye contact is a visual attending behavior; the problem described is in what the trainee says.

B. Vocal qualities

Vocal qualities concern tone, pace and volume rather than the topics the trainee follows.

C. Verbal tracking ✓ KEY

Verbal tracking means staying with the client's topic and not jumping to new subjects.

D. Body language

Posture and gestures matter, but they do not address topic jumping in the trainee's speech.

Rationale: Ivey describes attending behavior through visuals (eye contact), vocal qualities, verbal tracking and attentive body language. Verbal tracking means following the client's story without topic jumps, which shows the client that the counselor is listening. Counselors also notice which topics they selectively attend to.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: attending behavior and Ivey's microskills (verbal tracking)

☐ Approve ☐ Revise ☐ Retire Notes:

A 70-year-old client says, "Moving in with my daughter means I'll see my grandkids every day. But I'm giving up the house I lived in for forty years." Which response BEST reflects the client's feelings?

A. You will get to see your grandchildren every day once you move in with your daughter.

This paraphrases part of the content and leaves out both the feelings and the loss.

B. Have you thought about whether you could keep the house and visit the family more often?

This question moves to problem solving before the client's feelings have been heard.

C. Many people your age move in with family, and most adjust well within the first year.

Reassurance with a generalization minimizes the client's feelings and may sound ageist.

D. You're looking forward to being near the grandkids, and also grieving leaving your home. ✓ **KEY**

This captures both sides of the client's mixed feelings without choosing between them.

Rationale: Clients often express mixed or contradictory feelings about one event. An accurate reflection of feeling names both emotions and links them to their sources, often with "and" rather than "but" so that neither feeling cancels the other. Paraphrasing content, advising or reassuring misses the emotional message.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: reflection of feeling and mixed emotions

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor paraphrases, "So you're angry that your boss ignored your idea in the meeting." The client pauses and says, "Not angry, really. More like invisible." What is the BEST next response from the counselor?

A. Explain that anger is a common reaction to being ignored and is often hidden under hurt

Insisting on the counselor's label overrides the client's correction and weakens the alliance.

B. Accept the correction: "Invisible, as if your ideas don't register. Did I get that right?" ✓ **KEY**

Taking in the correction and checking accuracy keeps the reflection client-centered and builds trust.

C. Move on to problem solving so that the client does not feel stuck on the word choice itself

Skipping past the correction misses the client's more precise and meaningful feeling.

D. Ask the client to describe a time she felt angry at work so the anger can be explored more

This pursues the counselor's hypothesis rather than the feeling the client has just named.

Rationale: Paraphrases and reflections are tentative hypotheses that the client can confirm or correct. When a client corrects one, the counselor accepts the new wording, reflects it and checks for accuracy (a "check-out"). Clients often refine their own understanding through this exchange, so a corrected reflection is still useful.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: paraphrasing, accuracy and checking out

☐ Approve ☐ Revise ☐ Retire Notes:

After a counselor gently points out a discrepancy, a client says, "Okay, I see that I keep missing my kids' games, even though I say family comes first. I'm still not sure what to do about it." On Ivey's Client Change Scale, this response is BEST classified as:

A. Denial

Denial would mean not seeing or refusing to admit the discrepancy, which the client openly accepts.

B. Partial examination

Partial examination involves working on only part of the discrepancy; he acknowledges all of it.

C. Acceptance and recognition ✓ KEY

He fully accepts the discrepancy but has not yet generated a new way of acting.

D. Transcendence and new constructs

The top level involves new, larger ways of thinking and acting, which he has not reached.

Rationale: Ivey's Client Change Scale rates client responses to confrontation from denial (level 1), through partial examination (2), acceptance and recognition (3) and creation of a new solution (4), to transcendence or development of new, more inclusive constructs (5). A client who fully owns the discrepancy but has no plan yet is at level 3, and the counselor can now help generate new options.

References: Ivey et al. (Intentional Interviewing) — Confrontation: the Client Change Scale (level 3, acceptance and recognition) · Gladding (Counseling: A Comprehensive Profession) — Counseling skills: confrontation and evaluating the client's response

☐ Approve ☐ Revise ☐ Retire Notes:

Each session, a client asks her counselor what she should do and then thanks him warmly for "knowing best." The counselor notices he has begun giving advice he would not usually give. Which response BEST uses immediacy?

A. "I notice we've settled into a pattern where you ask and I answer. What's that like for you?" ✓ KEY

Naming the pattern in their relationship as it happens and inviting her view is relationship immediacy.

B. "You seem to rely on others for answers, and that likely began in your family growing up."

This is an interpretation about her past, not a here-and-now comment on their relationship.

C. "Let's set a rule that I will not answer any more direct questions about what you should do."

Imposing a rule changes behavior without exploring what is happening between them.

D. "You are capable of making good decisions, and I trust you to figure out what is best for you."

Reassurance may be kind, but it does not address the relational pattern the counselor noticed.

Rationale: Immediacy is the counselor's direct, present-tense comment on what is happening between counselor and client, often a pattern that mirrors the client's other relationships. Naming the pattern tentatively and inviting the client's experience opens it for exploration. It also helps the counselor step out of a role the client may be pulling him into.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: immediacy

☐ Approve ☐ Revise ☐ Retire Notes:

A client has met all of her counseling goals, but her counselor keeps suggesting "a few more sessions" because he enjoys their work and will miss her. What should the counselor do FIRST?

A. Keep meeting, since extra sessions are unlikely to harm a client who is doing well

Continuing counseling that no longer serves the client puts the counselor's needs ahead of hers.

B. Recognize his own reactions, consult a colleague and start planning a timely ending ✓ **KEY**

Naming his countertransference, seeking consultation and moving toward termination protects the client.

C. Ask the client if she would like to keep coming monthly for some ongoing social support

Offering ongoing sessions for social contact blurs the professional role and serves the counselor.

D. End counseling at the next session without discussion so his feelings do not interfere

An abrupt ending without preparation denies the client a planned, collaborative termination.

Rationale: The ACA Code of Ethics directs counselors to end a counseling relationship when the client no longer needs or benefits from it. A counselor's own sadness or reluctance about ending is a form of countertransference that can prolong counseling for the counselor's benefit. Recognizing it, consulting and planning a well-prepared termination keeps the work centered on the client.

References: ACA Code of Ethics — A.11.c Appropriate Termination · Gladding (Counseling: A Comprehensive Profession) — Termination: counselor resistance to ending

☐ Approve ☐ Revise ☐ Retire Notes:

A university counselor learns that several transgender students have been denied campus housing that matches their gender identity. She works with residence life administrators to revise the university's housing policy. In the MSJCC, this intervention targets which socioecological level?

A. Intrapersonal

Intrapersonal work addresses an individual's own thoughts, feelings and identity.

B. Interpersonal

Interpersonal work addresses relationships with family, friends and peers.

C. Public policy

Public policy refers to local, state and federal laws, not one university's internal rules.

D. Institutional ✓ **KEY**

Changing the policies of a social institution such as a university is institutional-level advocacy.

Rationale: The Multicultural and Social Justice Counseling Competencies place counseling and advocacy interventions at socioecological levels: intrapersonal, interpersonal, institutional, community, public policy and international or global. Institutions include schools, universities, churches and agencies, so changing a university housing policy is institutional work. Public policy work targets laws and government policies.

References: MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions: socioecological levels (institutional)

☐ Approve ☐ Revise ☐ Retire Notes:

A 13-year-old with a Black father and a White mother tells her school counselor that classmates keep asking "what she really is," and she feels she has to pick one racial group to belong to. In Poston's biracial identity development model, which stage BEST fits?

A. Personal identity

Personal identity is the early childhood stage, when self-concept rests on personal traits rather than group.

B. Choice of group categorization ✓ KEY

In this stage, social pressure pushes a biracial youth to choose one racial or ethnic identity.

C. Enmeshment and denial

Enmeshment/denial follows the choice and brings guilt or confusion about rejecting one heritage.

D. Integration of both racial heritages

Integration is the final stage of valuing all of one's heritages, which she has not reached.

Rationale: Poston's five-stage model of biracial identity development runs from personal identity, through choice of group categorization, enmeshment/denial and appreciation, to integration. In the choice stage, peers, family and society press the young person to identify with one group. Counselors can validate the pressure and support exploration of all heritages.

References: Poston (Biracial Identity) — Stage 2, choice of group categorization · Sue & Sue (Counseling the Culturally Diverse) — Racial/cultural identity development: biracial and multiracial identity (Poston's model)

☐ Approve ☐ Revise ☐ Retire Notes:

A White counseling student now rejects racism intellectually and seeks out classmates of color, often asking them to explain racism to her, while still judging their experiences by White norms. In Helms's White racial identity model, which status BEST fits?

A. Reintegration

Reintegration is a retreat to idealizing Whites and blaming people of color, which she has moved past.

B. Pseudo-independence ✓ KEY

An intellectual commitment to racial justice while leaning on people of color and White standards marks this status.

C. Immersion and emersion

In immersion/emersion, she would turn to other Whites to explore what it means to be White.

D. Disintegration

Disintegration involves guilt, confusion and conflicting loyalties, not a settled intellectual stance.

Rationale: Helms's model moves from contact, disintegration and reintegration (abandoning racism) to pseudo-independence, immersion/emersion and autonomy (building a nonracist White identity). In pseudo-independence, a person accepts racial equality intellectually but still relies on people of color to explain racism and views them through White frames. Immersion/emersion shifts the focus to understanding one's own Whiteness.

References: Sue & Sue (Counseling the Culturally Diverse) — White racial identity development: Helms's White racial identity statuses

☐ Approve ☐ Revise ☐ Retire Notes:

Core Counseling Attributes

16 items · NCE weight 8% (13 of 160 scored items)

Q185 of 200 · nce-s-cor-065 · Counseling and Helping Relationships · counselor self-awareness and bias · hard · In review

A counselor in long-term recovery through a 12-step program works at an outpatient clinic. Reviewing her notes, she sees that she steers nearly every client toward meetings and describes clients who prefer SMART Recovery or medication for opioid use disorder as "not ready." Which step BEST reflects counselor self-awareness?

A. Keep recommending 12-step meetings, since her own recovery shows how well that path works

This generalizes from one personal experience to all clients and leaves the bias in place.

B. Tell each client about her own recovery so they understand why she favors 12-step meetings

Self-disclosure here serves to justify the counselor's preference and adds pressure; it does not examine the bias.

C. Recognize that her own path is shaping her advice, and explore each client's preferred route ✓ **KEY**

Naming how her history colors her judgment, then centering each client's choice, is the core of self-awareness and avoids imposing values.

D. Stop mentioning any mutual-help groups so that she cannot influence clients' choices at all

Withholding useful options overcorrects; the aim is to offer choices without bias, not to remove information.

Rationale: Counselors' personal histories, including their own recovery, can quietly shape what they recommend and how they label clients. Self-awareness means noticing that pattern, examining it (often in supervision), and returning the choice to the client. ACA Standard A.4.b asks counselors to be aware of and avoid imposing their own values, attitudes and beliefs.

References: ACA Code of Ethics — A.4.b Personal Values · Corey (Theory & Practice) — The counselor as a person: personal needs, values and their influence on counseling

☐ Approve ☐ Revise ☐ Retire Notes:

Q186 of 200 · nce-s-cor-066 · Group Counseling and Group Work · counselor self-awareness and bias · medium · In review

A counselor leading a hospital support group for family caregivers reviews a session recording. She notices that she invites the two members who are nurses to speak more often and responds to them at length, while briefly thanking a member with little formal education. What should the leader do?

A. Reflect on the pattern in supervision and deliberately balance her attention among members ✓ **KEY**

Examining the bias and then consciously equalizing attention addresses both the leader's self-knowledge and the group climate.

B. Ask the two nurses to share less often so that the other members have more time to speak

This places the burden on members for a pattern the leader created and leaves her own bias unexamined.

C. Tell the whole group she has been favoring some members, and apologize to all of them at once

A group-wide confession without first understanding the pattern may embarrass members and centers the leader.

D. Continue as before, because members with clinical training contribute more useful information

This rationalizes the bias; every member's experience is valuable, and unequal attention signals unequal worth.

Rationale: Group leaders communicate members' worth through whom they call on and how fully they respond. Unequal attention based on education or status is a form of bias that can silence members and weaken cohesion. Best practice asks group workers to know their own values and blind spots and to attend to diversity within the group.

References: ASGW Best Practice Guidelines — Best practice in performing: self-knowledge and attention to diversity among members

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in training asks her supervisor how to become more aware of her own cultural biases. Which activity MOST directly builds this kind of self-awareness?

A. Reading research summaries that describe the values of many different cultural groups

This builds knowledge about others, a different competency from awareness of one's own biases.

B. Treating every client in exactly the same way so that cultural differences stop mattering

A "colorblind" stance ignores culture and leaves the counselor's own assumptions unexamined.

C. Limiting her caseload to clients whose backgrounds closely match her own life experience

Avoiding difference prevents growth and does nothing to surface her assumptions.

D. Examining how her upbringing shaped her values, through reflection and supervision ✓ **KEY**

Looking inward at how her own cultural history formed her values and reactions is the essence of counselor self-awareness.

Rationale: Self-awareness is the counselor's understanding of how their own cultural background, values and assumptions shape their perceptions of clients. It grows through deliberate self-reflection, such as journaling or a cultural genogram, and through supervision and feedback. Learning about other groups is valuable but builds knowledge rather than self-awareness.

References: Sue & Sue (Counseling the Culturally Diverse) — Counselor awareness of own assumptions, values and biases · MSJCC (Ratts et al., 2016) — Counselor self-awareness domain

☐ Approve ☐ Revise ☐ Retire Notes:

A perfectionistic 20-year-old engineering student has felt worthless since childhood. The counselor notices that she becomes animated and praises him when he reports top grades, but grows quiet when he describes setbacks. From a person-centered view, what is the MAIN concern?

A. Praising his achievements may slowly weaken his motivation to keep his grades high

Motivation is not the person-centered concern here; the issue is what her shifting warmth teaches him about his worth.

B. Her responses may repeat the conditions of worth that feed his sense of worthlessness ✓ **KEY**

Warmth that rises and falls with performance is conditional regard, echoing the conditions of worth that Rogers saw as the root of incongruence.

C. He may come to depend on the counselor's approval more than on support from his peers

Dependence could follow, but the central problem is that her regard has become conditional on achievement.

D. Talking about setbacks at length may reinforce a negative core belief about himself

This is a cognitive framing; person-centered theory sees exploring setbacks in an accepting climate as helpful.

Rationale: Rogers proposed that people internalize conditions of worth when they are valued only for meeting others' standards, producing incongruence and fragile self-regard. Unconditional positive regard offers acceptance that does not depend on performance. A counselor whose warmth tracks the client's achievements can unintentionally recreate the very conditions that brought him to counseling.

References: Corey (Theory & Practice) — Person-centered therapy: conditions of worth and unconditional positive regard

☐ Approve ☐ Revise ☐ Retire Notes:

Which counselor statement BEST illustrates unconditional positive regard?

A. I'll keep supporting you as long as you keep working on the goals we set together.

Support that depends on meeting goals is conditional regard.

B. I care about you as a person, whether or not we see this choice the same way. ✓ KEY

This separates acceptance of the person from agreement with a choice, which is the heart of unconditional positive regard.

C. You're a model client. You do everything I suggest for you between our sessions.

Praise tied to compliance signals that the client is valued for doing what the counselor wants.

D. I'm proud of you for making the right decision this week instead of the old one.

This evaluates the client's choice as right or wrong, implying regard depends on the "right" behavior.

Rationale: Unconditional positive regard is Rogers' term for accepting and caring for the client as a person without conditions. It does not require approving of every behavior, but the counselor's valuing of the client does not rise or fall with the client's choices. Praise tied to compliance or "right" decisions conveys conditional acceptance.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance

☐ Approve ☐ Revise ☐ Retire Notes:

In the fifth session, a counselor refers to a grieving client's late son as "Marcus." The client stiffens and says quietly, "His name was Martin." Which counselor response BEST demonstrates congruence?

A. Of course, Martin. Let's go back to what you were saying about the funeral service.

Glossing over the error hides the counselor's real experience and leaves the client's hurt unacknowledged.

B. I see many clients, and names sometimes get mixed up. I'm sorry that it happened.

The excuse is defensive and minimizes the mistake, shifting attention to the counselor's workload.

C. I'm fairly sure the name was written that way in my notes, so I'll check them later.

This deflects responsibility to the paperwork rather than owning the error.

D. I got his name wrong, and I'm sorry. Martin matters, and I want to get it right. ✓ KEY

The counselor openly owns the mistake and conveys real care, so words and inner experience match.

Rationale: Congruence means the counselor is genuine: what they express matches what they actually experience, without a professional facade. When a counselor makes a mistake, a congruent response owns it simply and honestly rather than hiding, excusing or deflecting it. Such moments, handled openly, often strengthen trust.

References: Corey (Theory & Practice) — Person-centered therapy: congruence, or genuineness

☐ Approve ☐ Revise ☐ Retire Notes:

A trainee tells her supervisor, "To be congruent, I should tell clients every reaction I have toward them as soon as I notice it." Which reply from the supervisor MOST accurately describes congruence in person-centered theory?

A. Congruence means your inner experience and outward expression match, not voicing every feeling ✓ **KEY**

Rogers described congruence as awareness and genuineness, with no facade; it does not require impulsively sharing every reaction.

B. Congruence is mainly about matching your body language to the client's posture and vocal tone

This describes mirroring, an attending technique, not genuineness.

C. Congruence requires sharing your personal history so the client sees you as an equal partner

Self-disclosure of history is a separate skill and is not what congruence requires.

D. Congruence is reached by keeping a calm, neutral manner no matter what you feel in a session

Hiding behind a neutral front is the professional facade that congruence rejects.

Rationale: In person-centered theory, congruence means the counselor's inner experience, awareness and outward expression are consistent, so there is no false front. It does not mean impulsively sharing every feeling. Counselors use judgment about expressing persistent reactions that are relevant to the relationship, and they do so in a way that serves the client.

References: Corey (Theory & Practice) — Person-centered therapy: congruence does not mean impulsive sharing of all reactions

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old client seeking help with heavy weekend drinking says his goal is to cut back to two drinks a night, not to quit. He has no history of withdrawal. The counselor believes abstinence would be safer. Which approach BEST strengthens the working alliance?

A. Explain that moderation goals rarely succeed, and make abstinence the first treatment goal

Imposing the counselor's goal undermines agreement on goals, a core component of the alliance.

B. Accept his goal without discussing it at all, so that he does not feel pressured or judged

Silently withholding a relevant concern is not genuine collaboration and skips mutual goal-setting.

C. Share her concern with permission, then agree with him on a goal and tasks he will try ✓ **KEY**

Offering her view respectfully while negotiating shared goals and tasks builds the agreement and bond the alliance depends on.

D. Put off any goal setting until he has kept a drinking diary for several weeks on his own

Monitoring can help, but delaying all goal discussion leaves the alliance without shared direction.

Rationale: Bordin described the working alliance as agreement on goals, agreement on tasks, and an emotional bond. When client and counselor see the goal differently, the counselor can share information with permission while respecting the client's autonomy and negotiating a goal both can work toward. Imposing a goal, or hiding a concern, weakens that collaboration.

References: Gladding (Counseling: A Comprehensive Profession) — The working alliance: goals, tasks and bond · Miller & Rollnick (MI) — Exchanging information with permission; supporting autonomy in goal choice

☐ Approve ☐ Revise ☐ Retire Notes:

At the first meeting of a 10-week court-mandated anger management group, several members say they are only there to get their paperwork signed. Which leader action is MOST likely to begin building a working alliance with the members?

A. Review the attendance rules and the consequences of missing sessions for each member

Rules need to be clear, but leading with consequences emphasizes control rather than shared purpose.

B. Remind them that their anger brought them here, so they should take the group seriously

Lecturing increases defensiveness and does nothing to create agreement on goals.

C. Show a video outlining the curriculum so that members know what the group expects of them

This presents tasks without the members' input, missing agreement on goals and the bond.

D. Acknowledge the mandate and invite each member to name a goal worth working on for them ✓ KEY

Validating the reality of the mandate while eliciting personal goals starts agreement on goals and builds the bond.

Rationale: With mandated members, the alliance begins by acknowledging their situation honestly and finding goals that matter to them within the group's requirements. Bordin's components of goals, tasks and bond apply in groups as in individual work. Leading with rules or lectures tends to deepen resistance rather than engagement.

References: Gladding (Counseling: A Comprehensive Profession) — Working alliance with reluctant and involuntary clients; goals, tasks and bond

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 34-year-old client who uses a power wheelchair arrives for an intake with a personal care attendant. Which counselor behavior BEST conveys respect?

A. Speaking directly to the client and asking how they would like the room arranged ✓ KEY

Addressing the client directly and inviting their preferences treats them as the expert on their own needs.

B. Directing the intake questions to the attendant, who likely knows the client's needs

Talking past the client to a companion is disrespectful and undermines the client's autonomy.

C. Moving the wheelchair closer to the desk so the client does not have to adjust it

A wheelchair is part of the client's personal space; moving it without asking is intrusive.

D. Telling the client how inspiring it is that they still get out and come to counseling

Calling routine activity "inspiring" is patronizing and frames disability as the client's defining trait.

Rationale: Respect means treating clients as capable adults and the experts on their own lives. With clients who have disabilities, that includes speaking to them directly rather than through companions, asking before offering help, and avoiding patronizing praise. These behaviors communicate that the client, not the disability, is the focus.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling individuals with disabilities: addressing the person directly and avoiding paternalism

☐ Approve ☐ Revise ☐ Retire Notes: _____

In a school counseling group for 14-year-olds coping with family changes, a usually quiet member describes crying the night her father moved out. Another member snickers. Which leader response BEST conveys warmth and respect while protecting the group?

A. Ignore the snicker so that the attention stays on the member who just took a risk

Ignoring the behavior leaves the sharer exposed and implies that mocking is acceptable in the group.

B. Calmly block the snicker, then thank the member for trusting the group with this ✓ **KEY**

Blocking protects the vulnerable member, and the warm acknowledgment honors her risk and reinforces safety norms.

C. Ask the member who snickered to leave the room until he is able to act respectfully

Removal is a heavy first response that shames the other teen and shifts focus away from the sharer.

D. Ask the group to vote on whether laughing at another member should be against the rules

Putting safety to a vote in the moment leaves the sharer unprotected and makes respect negotiable.

Rationale: Group leaders shape norms of safety and support, especially early and with adolescents. When a member takes a risk and is met with ridicule, the leader blocks the hurtful behavior promptly and then affirms the member who disclosed. Doing both conveys warmth to the individual and respect for the group as a safe place.

References: Yalom & Leszcz (Group Psychotherapy) — The therapist's basic tasks: culture building and shaping norms of safety and support

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old client tearfully says she has decided to move her mother, who has advanced dementia, into a memory care facility. The counselor grew up in a family where caring for aging parents at home was expected. Which response BEST reflects a nonjudgmental stance?

A. Did you look into all of the in-home care options before deciding on a facility for her?

The question implies the decision was not thought through and carries the counselor's own family values.

B. Many families do find a way to keep a parent at home if they get enough outside help.

This subtly argues for the counselor's preferred choice and adds to the client's guilt.

C. This decision is weighing on you. Tell me what went into it and how you're feeling. ✓ **KEY**

The counselor reflects the client's feeling and invites her story without evaluating the choice.

D. You made the right choice. Memory care is much better for her than staying at home.

Approval is still a judgment; it closes off the client's mixed feelings instead of exploring them.

Rationale: A nonjudgmental stance withholds both disapproval and approval so that the client can explore her own decision and feelings. Here the counselor's family norms create a pull to question the choice, which ACA Standard A.4.b cautions against. Reflecting the emotional weight and inviting exploration keeps the focus on the client's values.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor helping a client with job stress learns the client drinks six to eight beers most nights. Wanting to stay nonjudgmental, the counselor decides not to ask about the drinking because the client has not raised it as a concern. Which statement BEST evaluates this decision?

- A. It is sound, because exploring an issue the client has not raised imposes counselor values
Assessing a clinically relevant behavior is not value imposition; ignoring it may leave a real risk unaddressed.
- B. It is sound, because asking about substance use is likely to damage the working alliance
Respectful, curious inquiry seldom harms the alliance; avoiding the topic sacrifices sound care.
- C. It is unsound, because the counselor should first warn the client about the health risks
Leading with a warning is directive and likely to provoke defensiveness; assessment and permission come first.
- D. It is unsound, because being nonjudgmental withholds moral judgment, not clinical inquiry ✓ KEY
A nonjudgmental stance concerns how the counselor regards the person, not whether clinically important issues are explored.

Rationale: Being nonjudgmental means suspending moral evaluation of the client, not suspending clinical judgment. Heavy nightly drinking may bear directly on job stress, health and safety, so the counselor should raise it, ideally by asking permission and exploring with curiosity. Acceptance and assessment work together.

References: Miller & Rollnick (MI) — Acceptance in the spirit of MI; raising a concern with permission

☐ Approve ☐ Revise ☐ Retire Notes:

An intake form lists a 30-year-old client's background as "Puerto Rican and Irish." The client has come for help with work stress and uses they/them pronouns. Which approach BEST reflects cultural humility?

- A. Ask how the client describes their identity and which parts of it matter for this concern ✓ KEY
Inviting the client to define their identity and its relevance treats them as the expert on their own culture.
- B. Review literature on Puerto Rican and Irish family values before the next session begins
Group knowledge can inform hypotheses, but it risks substituting generalizations for the client's own account.
- C. Avoid raising culture at all, since the client came in about work stress and not identity
Assuming culture is irrelevant forecloses exploration; identity may shape how work stress is experienced.
- D. Assume the client identifies mostly as Puerto Rican, since that heritage is listed first
Inferring identity from the order of words on a form is an assumption, not humility.

Rationale: Cultural humility emphasizes openness and curiosity about how each client understands their own identities, rather than presuming. Multiracial and multiethnic clients in particular may identify in many ways, and those identities may matter more or less for a given concern. Asking, rather than assuming, keeps the client at the center.

References: Sue & Sue (Counseling the Culturally Diverse) — Multiracial individuals: self-identification and avoiding imposed labels · MSJCC (Ratts et al., 2016) — Client worldview: understanding clients' multiple identities

☐ Approve ☐ Revise ☐ Retire Notes:

A White counselor suggests that a 24-year-old Black client ease evening anxiety by taking long walks through his suburban neighborhood after dark. The client pauses and says, "That's not as simple for me as it might be for you." Which response BEST shows awareness of privilege?

A. Walking really helps with anxiety, so let's find some ways to make it work for you anyway.

This pushes past the client's concern and keeps the counselor's experience as the default.

B. You're right, I assumed. Help me understand what walking at night is like for you. ✓ **KEY**

The counselor owns that the suggestion reflected her own experience and invites the client's reality.

C. I didn't mean anything by it. I suggest walking to all of my clients who have anxiety.

Defending intent and claiming equal treatment overlooks that the same advice carries different risks.

D. Let's skip walking, then. Breathing exercises are something you can do safely at home.

Switching techniques avoids the issue the client raised and misses the chance to repair and learn.

Rationale: Privilege often shows up as assumptions about what is safe or easy, drawn from the counselor's own experience. When a client signals that a suggestion does not fit his reality, the counselor should acknowledge the assumption without defensiveness and explore the client's experience. This repairs trust and reduces the power imbalance in the relationship.

References: MSJCC (Ratts et al., 2016) — Counseling relationship: how privilege and oppression influence the counselor-client relationship · Sue & Sue (Counseling the Culturally Diverse) — White privilege and counselor assumptions

☐ Approve ☐ Revise ☐ Retire Notes:

Q200 of 200 · nce-s-cor-080 · Counseling and Helping Relationships · empathy and its levels · easy · In review

A client says, "I finished my degree at 52, and none of my kids came to the graduation. I guess it's no big deal." The counselor replies, "Congratulations! What are your plans now that you've graduated?" On Carkhuff's empathy scale, this response is BEST classified as

A. Interchangeable (Level 3)

An interchangeable response would reflect both the pride and the hurt the client expressed.

B. Additive (Level 4)

An additive response would go beyond the client's words to name deeper feelings, such as disappointment.

C. Subtractive (below Level 3) ✓ **KEY**

The reply ignores the hurt about the missing children and moves to a new topic, taking away from what the client expressed.

D. Additive (Level 5)

Level 5 responses add significantly to the client's feelings and meaning; this one omits the main feeling.

Rationale: On Carkhuff's five-level scale, Level 3 is interchangeable with the client's expressed feeling and meaning, Levels 4 and 5 are additive, and Levels 1 and 2 are subtractive. A response that attends to surface content while missing the core feeling, here the hurt behind "no big deal," falls below the minimally facilitative level.

References: Gladding (Counseling: A Comprehensive Profession) — Carkhuff's scale of empathic understanding: subtractive, interchangeable and additive responses

☐ Approve ☐ Revise ☐ Retire Notes: